

Jupiter Life Line (JLHL)

Conference call transcripts, oldest-first. 11 call(s). Generated 2026-08-02.

Call: Nov 2023

November 21, 2023

To,
National Stock Exchange of India Ltd.
Exchange Plaza, BKC
Bandra-Kurla Complex,
Bandra (East), Mumbai-400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: Q2 & H1 FY24 Earnings Conference Call – Transcript
Reference:- Intimation of Earnings Conference Call dated November 3, 2023 and Audio Link of
Analyst/ Investor Conference Call dated November 15, 2023

Dear Sir/ Madam,
Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q2 & H1 FY24 Results Conference Call held on Wednesday, November 15, 2023 at 10:00 AM (IST) for the quarter and half-year ended September 30, 2023.

This information is available on the website of the Company www.jupiterhospital.com
You are requested to kindly take the afore mentioned on record and oblige.

Thanking You,

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 11

"Jupiter Life Line Hospitals Limited
Q2 FY24 Earnings Conference Call "

November 15, 2023

Disclaimer: E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges — BSE Limited and National Stock Exchange of India Limited and the Company website on 15th November 2023 will prevail

MANAGEMENT : MR. ANKIT THAKKER – EXECUTIVE DIRECTOR &
CHIEF EXECUTIVE OFFICER , JUPITER LIFE LINE
HOSPITALS LIMITED
MR. ANAND APTE – CHIEF OF BUSINESS AND
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MR. PRASHANT PITALE– FINANCE , JUPITER LIFE LINE
HOSPITALS LIMITED
STRATEGIC GROWTH ADVISORS – INVESTOR
RELATIONS ADVISORS

MODERATOR : MR. JAINIL SHAH – JM FINANCIAL

Jupiter Life Line Hospitals Limited
November 15, 2023

Page 2 of 11

Moderator: Ladies and gentlemen, good day and welcome to the Q2 FY24 Earnings Conference Call of Jupiter Life Line Hospitals Limited hosted by JM Financial.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Jainil Shah from JM Financial. Thank you and over to you, Mr. Shah.

Jainil Shah : Good morning, everyone. On behalf of JM Financial, we welcome you all to Jupiter Life Line Hospitals Limited Q2 and H1 FY24 Earnings Conference Call.

We have Dr. Ankit Thakker, executive director and CEO, and the senior management team with us. The Company's "Financial Results and Investor Presentation" have been uploaded on the Company's website and stock exchanges. We will begin the call with opening remarks from the management, followed by Q&A.

Before we begin, I would like to point out that some of the statements made during today's call may be forward-looking. A disclaimer to that effect has been included in the Earnings Presentation.

I would like to hand over the call to Dr. Ankit for his opening remarks. Thank you and over to you.

Ankit Thakker : Thank you, Jainil. Good morning, everybody. Thank you for taking the time to join us on our call this Diwali week.

This is our first Earnings Call, and we are here to discuss the "Business Financial Performance" of Q2 and the First Half of FY24.

With me today are Mr. Anand Apte - Chief of Business and Strategy, and Mr. Anish Vyas and Mr. Prashant Pitale from our Finance team along with our IR Advisor SGATEAM.

So, being the first Earnings Call, I am going to take some liberties and address some of our philosophies and thought processes also, besides talking about numbers for the benefit of the wider audience who may not have had a chance to meet us during the IPO process.

But before we start, I would like to express my deep gratitude to all our shareholders, both in our previous private affair and now in the listed world as well for buying into our vision and work. I am also grateful to the bankers and the lawyers who helped us tell our story to the wider investor community. I am thankful to all our doctors, team members, partners, consultants, Jupiter Life Line Hospitals Limited

November 15, 2023

Page 3 of 11

vendors who helped us to create this story in the first place and most importantly, I am humbled by the trust that all our patients have had in us for over 16 years now. It is for you that we exist and your well-being and interest are foremost in our minds at all times. And it is because of this focus on patient centricity, we have developed our business model the way we have, and coincidentally, we found that the interests of patients and investors largely overlap; we have taken a conscious call to have all our hospitals as custom designed and built because we realized that a well-planned infrastructure is essential to delivering good care.

We also own all our assets by buying land and owning the building, plant and machinery, we are able to avoid the future stress of inflation-linked and perpetual drain, that of the lease on our cash flows. We are not in this for the short term and we are happy to make peace with the reality that healthcare is a capital intensive business and in order to build a business that has strong fundamentals and which is sustainable over the long term, we will have to make bigger investments in the early phases. We have also chosen not to follow the hub and spoke model because we believe that spokes are incomplete facilities and we would want

to focus our energies

on building full service centers of excellence, which can not only provide the best care to our patients but also achieve economies of scale and generate good returns for our investors. We hope to continue walking on this path and we hope to keep growing and creating value for the communities we serve and our investors.

A lot of people told me during the IPO process that once you are listed, there will be a lot of quarterly pressure, but those of you on the call who I have met during the road shows will be able to vouch for me when I say that I have always maintained that we are in this for the long haul. Of course, the financials need to be reported on periodic basis, which we will do, but so far we have not and going forward, we do not intend to make decisions or take actions purely

for short-term interest. In our private avatar, we were blessed to have shareholders who bought into the vision of long-term value creation and wholeheartedly supported the guiding principle of always doing the right thing. I am sure even in the listed world, we will find similar support. To give you a brief introduction about the Group, for those of you who may not have a background about us, Jupiter started its first hospital in Thane in the Mumbai Metropolitan Region that is MMR in 2007, the second one was in Pune in 2017 and the third in Indore in late 2020. This is a 96 plus percent subsidiary. Thane and Pune hospitals are both around 375 beds each and were Greenfield projects, while the Indore Hospital was an acquisition that is planned for about 430 beds, but we are currently operating around 231. All our hospitals are full service independent hub hospitals, where we provide all services from childbirth and newborn care to cancer and organ transplantation services. As we speak, we are constructing a 500 bed hospital in Dombivli, which is also in MMR. The land is purchased, all permissions for construction received, the excavation is now complete and we have begun constructing upwards. The project is likely to be operational anywhere between 2 to 3 years from now. We have a strategic focus on the healthcare market in Western India where we have a strong understanding of the regional nuances and believe there is a significant and growing demand for Jupiter Life Line Hospitals Limited
November 15, 2023

Page 4 of 11

high quality health services. In fiscal 22, the penetration of health insurance in India was around 38%, but the average for the West Indian states that is Maharashtra, Gujarat, Goa and MP was about 78%. We believe that by developing our business model around the insured population, we will be able to address the affordability question in healthcare and generate strong demand for our facilities. Even now, we have over half of our revenues from the insured patients, with the government schemes only contributing around 1% of our revenue. Our aim for the next few years is to continue growing in Western India region and build more hub hospitals of between 300 to 500 beds, taking our groups strength to about 2500 beds. I realize I have gone on too long on an earnings call without talking about specific updates and numbers. The presentation has been uploaded and I am sure you would have gone through it, but let me enumerate some key developments.

In the current quarter, the Company has become fully debt-free by repaying all our loans with the fresh proceeds from the IPO. We are also left with a net cash of about Rs. 320 crores on our book, which we plan to use for future CAPEX and growth. Our hospital in Indore has now turned EBITDA positive; soon, we expect it to be positive even on PAT levels. We have completed empanelment with the insurance companies and going forward we believe this should improve the demand and the occupancy levels in Indore. In Pune, now we have signed rate revision contracts with the insurance

companies and going forward, we believe these will help improve our ARPOBs and the hospital's margins. Eventually, the Company's financial performance should improve due to the insurance developments of Indore and Pune going forward.

Coming to numbers:

In FY24 Q2, the total income stood at Rs. 264.2 crores, an 18.4% growth year-on-year. EBITDA for the quarter stood at Rs. 62 crores, representing a margin of 23.5%. EBITDA has increased 8.8%. PAT for the quarter stood at Rs. 33.7% representing a margin of 12.8%. PAT increased by 24.4% corresponding quarter year-on-year.

Summary for the first half:

H124 revenue was Rs. 508.4 crores, showing an increase of 19.8% compared to H1 year-on-year. EBITDA for the half year stood at Rs. 116.1 crores, representing an increase of 8.4% compared to H1 year-on-year. EBITDA margin 22.8%, PAT for the half year stood at Rs. 87.7 crores. The average occupancy was 62.3% in H1 FY24 compared to 58.6% in H1 FY23. The ARPOB for the first half was Rs. 53,075 at the console level, and the ALOS stood at 3.89 days in the first half of this year.

So, with this summary, I am happy to talk to everyone, take questions, and look forward to talking to you. Thank you.

Moderator: Thank you very much, sir. We will now begin the question-and-answer session. The first question is from the line of Abdul kader Puranwala from ICICI Securities. Please go ahead.

Jupiter Life Line Hospitals Limited
November 15, 2023

Page 5 of 11

Abdulkader Puranwala: Sir, my first question is basically on Thane and Pune, so with the insurance rate revision, which has happened, what is the positive impact on EBITDA that you are already seeing because of Thane and with the rate revision in Pune, how do you see the margins panning out for the near-term 3 to 4 quarters from here on?

Ankit Thakker: The rate revision in Pune was signed early this month and probably last week and will come into

impact from this week onwards. So, we believe that the Pune rate revision of insurance companies should show you a positive impact for the last 4 to 5 months of this year. The rate revision should be in high single digits, I think, and that should give rise to ARPOB and revenue increase on a blended level because 100% is not insurance, on a blended level, the ARPOB and revenue increase should be around 4% or so.

Abdulkader Puranwala : Sir, second question is on Dombivli where you have constructed 500 beds, once Dombivli comes into or becomes operational, so how do you see Thane operations getting impacted, will there be any impact, first of all? And for Dombivli per se, are there any similar metrics like what you have in Thane achievable at this facility as well?

Ankit Thakker : As I was saying in the opening remarks that Dombivli is in MMR region and our business model is predominantly in all the units, we have high insurance segment patient. So, the insurance reimbursements in MMR region everywhere should be similar. So, we have a feeling that of course first year or two, you do more of primary and secondary care work, the complexity of work includes only after year 2 onwards, but once it settles down then Dombivli and Thane should show similar metrics what we believe and as far as impact on Thane is concerned, I think it would have no impact because the population of that region itself is already very high and growing very rapidly, so I don't think that would have any ripple effect on Thane. The local demand itself is quite high to take care of Dombivli Hospital.

Abdulkader Puranwala : What I was asking basically is, are you providing any particular guidance in terms of your FY24-25 revenue and EBITDA margins or just to have some bit of a flavor since Q2 has seen some bit of an improvement in your overall margin, so where do you see ending it up for the year ended or for next year as well?

Ankit Thakker : No, honestly, we are not

providing guidance as a thought process, and we will provide updates on what has already happened in each quarter. You could make assumptions based on what you have seen in the first half of this year, but we do not want to give future guidance.

Moderator : Thank you. We will take the next question from the line of Tushar Manudhane from Motilal Oswal Financial Services. Please go ahead.

Tushar Manudhane : Sir, just on this Pune Hospital, is the insurance and the self pay mix similar to the Company level or is it different?

Jupiter Life Line Hospitals Limited
November 15, 2023

Page 6 of 11

Ankit Thakker : So, Pune and Thane both have a higher level of insurance. Roughly two-thirds of the patients are insured in Thane and Pune. Indore had a much lower number, and that is why at the console level I think you see some 52%-53% of insurance that was because Indore empanelment was not fully complete, and Indore also generally historically had lower penetration, which is now growing, but for those two reasons, Indore was dragging down the insurance as a payer segment on a console level. Pune and Thane have about two-thirds of insured patients, 65%.

Tushar Manudhane : Sir, secondly on ARPOB, while in first half FY24, the growth rate has been 4% when I compare with FY23 and the first half ARPOB and with this rate revision coming into picture, so any further other levers like case mix optimization which can drive ARPOB from here on?

Ankit Thakker : So, case mix optimization potential is there in Indore. Thane and Pune have a fairly matured case mix. I don't think there can be dramatic changes in case mix in Thane and Pune. So, as an ARPOB driver, inflation is the ARPOB driver for Thane and Pune.

Moderator : Thank you. The next question is from the line of Harsh Bhatia from Bandhan AMC. Please go ahead.

Harsh Bhatia : Just one or two clarifications, in terms of Thane asset, can you help us understand the OP/IP volume growth, we have given the combined H1 to H1 OP/IP volume growth, which is mid to high teens on an H1 to H1 basis, but purely from the Thane asset, if you can help us understand?

Ankit Thakker : Thane, volume growth you want find out, I don't know if I have it immediately, but let me just try and look. Follow up question, so maybe we could answer that till someone looks it up.

Harsh Bhatia : And secondly, just to understand the cost structure from the quarter-to-quarter perspective, on a year-on-year basis, could you help us understand like what is the movement in the overall cost structure because Indore has now become EBITDA positive I think, in second quarter, but despite that there is a slight dilution at the margins purely from the cost perspective, gross margins are more or less stable, so where does the incremental cost coming from?

Ankit Thakker : Some doctor costs because of some new hires, which we have done in some of the hospital, is an increase. The second contributor for increase in some cost is in Pune, because we have added beds in Pune just a few months back and the fixed cost associated with those beds has already come in, but obviously, they don't get occupied the day you commission them. So, for a couple of quarters, whenever we commission new beds, we see a little bit of increased cost and the revenue lags by a couple of quarters to fill up those beds. So, these are two factors. The third factor is Q2 and you are from Mumbai, so you know Q2 in this part of the world is infection and fevers are quite high. So, of your total occupancy, you have a larger percentage of medical

management and for them , the ARPOB will be lower if you have a higher medical management.
So, these are a few factors which have played some role.
Jupiter Life Line Hospitals Limited
November 15, 2023

Page 7 of 11

Anand Apte : This is Anand Apte . With regards to your previous question, the Thane revenue gre

w by 14%,

so OP volume was up by 3% and IP volume was up in terms of numbers by 11%. (The said statement was mentioned erroneously, it should be read as OP volume was up by 11% and IP volume was up by 3%).

Harsh Bhatia : So, IP 11% and OP 3% and right now the occupancies come around 71% -72%?

Anand Apte : That is right. (The said statement was mentioned erroneously, OP volume was up by 11% and IP volume was up by 3%)

Moderator : Thank you. The next question is from the line of Anuj Suleja from Trulife . Please go ahead.

Anuj Suleja : So, I just wanted to understand what is the seasonality like for Jupiter because we have a unique mix of say Western and also that is Indore, so can you give a sense on how H2 look, how it looks in front of H1 and if you could also provide some amount of guidance in terms of how should we be looking at occupancies and ARPOBs for the second -half of the year ?

Ankit Thakker : So, Q1 and Q3 are typically slightly lower because of lifestyle -related issues that is Q1 being summer vacations in this part of the world, there is both , at any given point, some of the doctors are always on vacation. Similarly, elective surgery is also the patients would like to push or pull a little bit for reasons which can be postponed, they choose to postpone it and again Q3, festivals and weddings, similar reasons where elective work kind of goes down a little bit, so Q1 and Q3 are slightly lower . Q2 is slightly higher because of a little bit of overflow from the Q1 elective procedures and also the infections which I was just talking about being much higher in Q2, so Q2 is higher in terms of occupancies and volumes and Q4 is kind of somewhere in the middle.

Anuj Suleja : And in Q1, there was some amount of one-off because you had hired some new talent, right? Is that understanding correct?

Ankit Thakker : Yes.

Anuj Suleja : So, on a steady state, the numbers that have come this quarter and the numbers that have come last quarter somewhere in the middle should be the ideal EBITDA margin for the business ?

Ankit Thakker : There should be something like that. So, honestly, I know you have asked a numerical quantitative question, but I want to give you a qualitative answer to that. How do we look at the business and how do we look at our approach to management . So, as far as we are concerned or as far as the revenues, if you look at it is a collection of each individual encounters between a doctor and a patient, right? So, whatever needs to be done for each patient needs to be done for that patient. You can't do more and you should not do less. This quarter, for example, you had more medical management patients , and the trend was that on average, medical management patients did not require as much of an ICU stay or had as many critical problems as they did last year. Now , how do you plan that forecast that or guide for that , you can't and if the patients don't
Jupiter Life Line Hospitals Limited
November 15, 2023

Page 8 of 11

require it, what do you do about it? You don't do anything , you don't give them that critical care .

As a result of that, will your ARPOB or margin get compressed by a few points? Yes, it will , do we get troubled or worried about it? No, we don't. Similarly, next year, if maybe there is more critical illness and there is a higher ARPOB associated with that, will we pat our backs and chest thump? No, we will not. So, we don't think that too much microscopic analysis in healthcare is doable, and even before the listing event, we had never looked at business in that fashion.

However, do we need to be attentive of which way things are going? Of course we need to be.

So, when we look at the overall utilizations, the occupancies, the number of procedures that we are doing , those kinds of things are more of a guidance for us, whether things are going in the right direction or not , and so far , I think

they are going in the right direction.

Moderator : Thank you. The next question is from the line of Sachin Shah from SS Securities . Please go ahead.

Sachin Shah : I have couple of questions , what are your views on ownership versus leasing model? And will you be considering leasing opportunity to reach 2500 beds ?

Ankit Thakker : So, we will consider everything at all times. We are not closed -minded or rigid in our attitude or approach. So, as far as consideration goes, we will always consider all opportunities that present itself , if they make sense, we will take them on , but broadly, what is our thought? Broadly, our thought is that ownership is a better long -term story. Suppose you are going to lease an asset to me. In that case, the lease will always be marked to market with the current inflation and

economic conditions. The lease is going to escalate year-on-year, the lease is never going to go away. So, far pre-listing, we were debt funded, we funded all our assets with debt, the debt goes away in 8 to 10 years and then the asset is free on board. So, we believe that after the first 3, 4, 5 years, the lease model does not play out well, and the ownership model plays out much better year 5 onwards. So, for that reason, we have chosen to own the assets; however, if some interesting structure comes up or the very good opportunity comes up that is not available on ownership, and we think we can justify it, we will look at the lease option, but as a default parameter we prefer to work.

Sachin Shah : How do you think of expanding post your Dombivli facility?

Ankit Thakker : We are currently looking for more land in Western India. We would like to buy two pieces of land which is our thought for now, and build up two more hospitals of this size; reaching that 300 to 500 beds is a sweet spot in terms of efficiency and of operations and finances both. That is what we are trying to do that identical two parcels of land which we can buy and build upon.

Moderator : Thank you. The next question is from the line of Bhagwan Chaudhary from Sunidhi Securities.

Please go ahead.

Bhagwan Chaudhary : Just in a broader sense, I have 2-3 questions, and this is regarding every hospital, so how should we look at it? The first question is from the Indore side, currently, it has 50% kind of occupancy,

Jupiter Life Line Hospitals Limited

November 15, 2023

Page 9 of 11

so going forward, what can be the maximum occupancy the second thing, how do we look in terms of the revenue mix, will that be more accretive towards the margin, some that patient mix change, or it will be the same which is going on now?

Ankit Thakker : Whenever we design a new facility, our thought is that at the peak or the matured state, it should reach mid 70% occupancy, if we think mid 70% of what and that is how we will size the facility, whatever we think can be at 70% plus because if you think that you can't achieve that then better to have a smaller size, no point having a huge facility which you can't fill up, so eventually at a mature state, whatever we build should reach 70 plus percent in occupancy. That is the first factor. Second factor is specifics that currently, Indore is at 50% and we are updating 231 beds. So, once the occupancy crosses 60%, 65%, we will add some more beds and then ramp those up and then add some more beds and ramp those up till we are saturating the entire capacity. So, that is the thought process of occupancy ramp up for Indore. On the ARPOB and margin level, last year was the second full year of operations in Indore, and we were almost at a breakeven level, with a very small few lakhs of negative EBITDA on Rs. 100 crore revenue. So, this year, we have turned EBITDA positive. Going forward in the next couple of years, EBITDA margins of Indore should also become high teens and then go forward, but at all times, we think that

Indore margins might be 1% or 2% lower than those in the Mumbai region, because of the cost structure of Indore is lower and that leads us to believe that even though Indore can reach north of 20%, but it should be a little lower than that of Thane.

Bhagwan Chaudhary : Will this margin expansion in Indore come from the occupancy, or will there be some contribution from the ARPOB as well?

Ankit Thakker : Both case mix optimization -driven ARPOB growth and occupancy.

Bhagwan Chaudhary : Secondly, in the same in the same line, if I look at the Thane hospital where your occupancy is 72%, what can we expect from here onwards in Thane, any improvement in the occupancy or will it be driven by ARPOB now onwards?

Ankit Thakker : All plateauing lingering kinds of growth can be possible. So, realistically, I don't think any hospital can consistently operate above 80% occupancy. So, the mid to high 70s is what it can reach, but is there in my control any magic to make it 72 to 77 tomorrow, no, there is not, but yes, that slow little bit of lingering upward trend in occupancy can happen and the inflation-linked ARPOB growth will happen.

Bhagwan Chaudhary : And the same comment on the Pune side as well because?

Ankit Thakker : But has a lower percentage occupancy, so Pune occupancy will grow faster, and Pune has got 60% now. So, that has a 10%-15% ARPOB growth as well as occupancy growth. So, Pune will see both ARPOB growth as well as occupancy growth.

Jupiter Life Line Hospitals Limited

November 15, 2023

Page 10 of 11

Bhagwan Chaudhary : And my last question on this Dombivli unit, by what time do you expect it to come be operational, in 26 or 27?

Ankit Thakker : Yes, FY26, it should be operationalizing, end of 26.

Moderator : Thank you. The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Please go ahead.

Abdulkader Puranwala : Sir, at Indore, so now that the insurance empanelment has happened, how should we see the

occupancy ramp up from here on and then some qualitative indicator if you could provide? And secondly, you talked about the high teen margins for the near term for Indore, so once there are close to 200 beds that you to operationalize here, so how soon you would be operationalizing them and post that, will there be any immediate impact on the margins of Indore as well?

Ankit Thakker : Indore, how soon we will ramp up the occupancy ; currently it is 50% , so once it crosses 60% , we will start planning for increasing the beds, and we generally , as a thumb rule, would like to keep playing between the 50 % and 65% occupancy range. So, as a broad indicator, once we reach 65% occupancy , add enough beds to come down to 50% occupancy and then again ramp them up a gain and keep doing these 2x or 3x until you exhaust everything. So, that is how we plan to ramp up Indore and margins, yes, once the bed capacity goes up and those start getting occupied , then the high teen and eventually 20% margin is what we are expecting there.

Abdulkader Puranwala : And sir, last one on your further expansion beyond Dombivli , sir, if you could provide any flavor, have you already initiated docs for buying land or doing some M&A of already an established hospital chain , and what is the kind of ticket size you would be typically looking out for when you try and search for a potential acquisition candidate?

Ankit Thakker : So, nothing concrete on the M &A or acquisition side , we have not found anything that looks very promising or aligned with our thought process and business model yet. So, I have nothing to say there . On the land side , yes, land being land , everyone in India understands that it is one of the most challenging things to acquire in the country's land and at the right location at the right value and free of litigation. So, that is where the challenge lies. So, we

are currently

engaged in several conversations about land . Still, till the time you don't sign anything, I don't think it is wise to engage in speculations on land discussion, but yes, we are talking to a few people for land , and as soon as we have anything to announce, we will make that announcement to everybody.

Moderator : Thank you. The next question is from the line of Jigar Shah from AK Securities. Please go ahead.

Jigar Shah : I have just a couple of operational questions, how do you go about selecting location for building a hospital and what are the key criteria ? The second one would be, what are the key therapies that you focus on at your hospital?

Jupiter Life Line Hospitals Limited
November 15, 2023

Page 11 of 11

Ankit Thakker : The second question is very easy. I will do that first. We don't focus on any one or a few therapies . We choose to do everything in all our hospitals because we think healthcare and medical branches are interlinked with each other , and in order to excel in any branch of healthcare, you will have to excel in all of them. So, all the facilities are full service and independent, and we try to do everything everywhere and do it well. So, that is the second part. The first part is the location selection. So, in Western India , where we operate, I think it is one of the more underserved markets as far as Healthcare is concerned, North and South have more penetration in general. So, plenty of locations are densely populated in prominent Tier 1 locations and don't have health facilities . So, we believe that instead of going and sitting next to an already established hospital in a more glamorous location, we would rather go to some such location where there are a lot of residential populations , million plus people and who don't have access to quality health care and as I was saying earlier, with the rising penetration of insurance, the affordability question is now slowly getting out of the equation. Lot of people, especially post COVID, are choosing to get themselves insured even if their employer does not provide them an insurance , a lot of employers also now are realizing that health insurance, like in the West is becoming kind of minimum prerequisite as a part of their employment benefit program. So, in combination of all this , high insurance , making affordability easy, high population density and lack of other facilities wherever we find these kind of locations and plenty of them, we think that is a good area to set up a hospital.

Jigar Shah : And lastly, sir, our Dombivli hospital, once it gets operationalized, we would be the largest in the area, right?

Ankit Thakker : Yes.

Moderator : Thank you. Ladies and gentlemen, as there are no further questions, I would now like to hand the conference over to the management for closing comments. Over to you , sir.

Ankit Thakker : So, thank you all for taking time and attend the call and listening to us. I hope that the answers were clear and satisfactory. If there are any further afterthoughts or any further questions, which you may have at a later date , you could reach out to the SGA team. They are managing our IR activity and will happily put us in touch with you . Again, on behalf of all of us, Happy Diwali, have a great year ahead and talk to you again next quarter. Thank you.

Moderator : Thank you very much, sir. Thank you, members of the management . Ladies and gentlemen, on behalf of JM Financial, that concludes this conference. We thank you for joining us and you may now disconnect your lines. Thank you.

Call: Feb 2024

February 16 , 2024

To,
National Stock Exchange of India Ltd.
Exchange Plaza, BKC
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: - Q3 & 9M FY24 Earnings Conference Call – Transcript

Reference : - Intimation of Earnings Conference Call dated February 1, 2024 and Audio Link of Analyst/ Investor Conference Call dated February 12, 2024

Dear Sir/Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q3 & 9M FY24 Results Conference Call held on Monday , February 12, 2024 at 09:30 AM (IST) for the quarter and nine month ended on December 31, 2023.

The same will be available on the website of the Company at www.jupiterhospital.com

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking You,

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 10

“Jupiter Life Line Hospitals Limited Q3 & 9M FY24
Conference Call ”

February 12, 2024

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MANAGEMENT : DR. ANKIT THAKKER – EXECUTIVE DIRECTOR & CEO
MR. ANAND APTE – CHIEF STRATEGY OFFICER
MS. FALGUNI SHAH – BUSINESS CONTROLLER
MS. SUMA UPPARATTI – COMPLIANCE OFFICER &
COMPANY SECRETARY
MODERATOR : MR. ABDUL KADER PURANWALA – ICICI SECURITIES
LIMITED

Page 2 of 10

Moderator: Ladies and gentlemen, good day and welcome to Jupiter Life Line Hospitals Limited Conference Call Hosted by ICICI Securities Limited .

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” and then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Abdul Kader Puranwala. Thank you and over to you, sir.

Abdul Kader Puranwala : Good morning, everyone. And on behalf of ICICI Securities, welcome you all to Jupiter Life Line Hospital s Limited Q3 and nine-month FY24 Earnings Conference C all.

Today on the call, we have with us Dr. Ankit Thakker – Executive D irector and CEO, Mr. Anand Apte – Chief S trategy Officer and other members of the Senior Management Team.

We will begin the call with opening remarks from Dr. Ankit and post that We will have a Q&A session. So, now I hand over the call to Dr. Ankit for his opening remarks. Thank you.

Dr. Ankit Thakker: Thank you, Abdul. Good morning, everybody. I thank you all for joining us on our Earnings Call to discuss the “Business and Financial Performance ” of 3rd Quarter and Nine Months of FY24. I hope you had a chance to see the “Financial Results ” in the “Presentation ” on the website and the Stock Exchanges.

With me on the call today is Mr. Anand Apte – our Chief of Business & Strategy. I am also accompanied by Falguni Shah – our Business Controller and Suma Upparatti - Compliance Officer and Company Secretary and of course our IR team represented by SGA.

Throughout our journey, we prioritized a patient -centric approach and we've always aimed to deliver high quality medical ser vices and a hospitable experience to the patients and their families. This commitment has garnered greater trust from our patients, our doctors and the entire team, which has primarily contributed to our year -on-year growth in performance. We continue to r emain optimistic about the tremendous scope of expansion for quality health services in India and especially in Western Indian region. With the growing aspiration of the average Indian consumer, the rising middle class and the increased penetration of the health insurance, more and more people are demanding a higher quality and a better experience in their health care. Jupiter's brand and offering has therefore become more relevant these times. We are committed to our Greenfield hub focused growth strategy in Western India but shall also continue to evaluate suitable growth opportunities for the company as they present themselves from time to time.

As you know from our previous communications, we are in the process of establishing a new 500 bed Quaternary Ca re Hospital in Dombivli in the Mumbai metropolitan region, which will

Jupiter Life Line Hospitals Limited

February 12, 2024

Page 3 of 10

be constructed over 600,000 square feet. The construction is currently in full swing and working as per our scheduled time. As the stated objective of the IPO, we mentioned last time, we have repaid all our debt obligations and now got annualized finance saving s of over Rs. 40 crores. In the prev ious quarter, we also completed e mpanelment with the insurance companies in Indore , and consequently , our occupancy has increased from 51.2% in t he last quarter to 56.2% in Q3 FY24. The contract renegotiations with insurance companies at the Pune Hospital are also concluded and it has been one of the factors that has helped to increase the ARPOBs from ~49,000 in Q2 24 to 53,400 in Q3 F Y24.

Specific numbers for the current period :

In Q3, our total income stood at Rs. 273.6 crores an increase of 19.8 % Y-o-Y. The EBITDA for the

quarter stood at Rs. 62.9 crores that is an increase of 39.2 % Y-o-Y, the margin representing 23%. The PA T for the quarter was Rs. 43.7 crore representing a PAT margin of 16%. For the nine months period, the total income has been Rs. 782.1 crore, 19.8% growth. The EBITDA for nine months is Rs. 178.9 crores, that is a 17.6% increase Y-o-Y. And the EBITDA margin is 22.9%. The PAT for nine months is Rs 131.3 crores . The average occupancy for these 9 months has been 63.2% compared to 60.6% in the previous year same period. ARPO B for 9 months FY24 is Rs 53,585 and the ALOS is 3.92 for these 9 months .

So, that is a quick summary. You ha ve probably seen this. We can use this time better to talk and answer your questions, so the floor is open.

Moderator: Thank you very much. We will now begin the question -and-answer session. We have our first question from the line of Ashay Jain from Jain Capital. Please go ahead.

Ashay Jain: I have a couple of questions. So, firstly, we've always focused on the latest technology and the instruments or equipment that have very high AMC packages. So, how much do we spend yearly as a whole new instrumen t, or you may say what the maintenance CAPEX would be ?

Dr. Ankit Thakker: So, yes, we are focused on buying the appropriate technology for helping us provide the quality of care we would like. It is hard to answer in terms of annual CAPEX because we are in the stage of various new developments from time to time. So, in the year of a new hospital, it has been in the year of launching a new department within a hospital it comes up. Suddenly, nothing is happening in the year and just the AMC, then the number does not look big. AMC for different equipment would range between 5% and 10% of the capital cost of procurement.

Moderator: The next question is from the line of Sagar Deha. Please go ahead.

Sagar Deha: So, I have two questions. Have you identified any other expansion opportunities apart from the Dombivli Hospital?

Jupiter Life Line Hospitals Limited
February 12, 2024

Page 4 of 10

Dr. Ankit Thakker: Hi Sagar, so we are evaluating a few opportunities in Maharashtra currently. Couple are in serious stages of discussion, but I don't have anything concrete to announce today which means nothing is signed, sealed and delivered but we are looking at some opportunities in Maharashtra as we speak.

Sagar Deha: Okay, understood. Thank you. And also, I wanted to understand how much the bed capacity can be expanded in Pune and Indore hospitals. And by what timeline can we achieve this?

Dr. Ankit Thakker: So, Pune, there is just the CAPEX for expansion is over. We just have to activate the last 30 beds, which we can do in Pune. Currently, as probably if you look at this quarter, the Pune occupancy is something like mid 60%. So, maybe sometime in the coming financial year, we will activate Pune's last balance capacity. And that would be the end of new bed capacity for the Pune hospital. For Indore, we have close to 200 bed growth opportunities. This quarter, we have shown an occupancy of, again, mid 50% and 56.2%, I think is the number for this quarter. In previous communications, I said that we usually think of adding new beds when we are between 60% and 65%. So, I anticipate that milestone to be achieved quite soon and we should be thinking of adding new beds in Indore.

Moderator: Thank you. The next question is from the line of Rohit Mehra from SK Securities. Please go ahead.

Rohit Mehra: So, my first question is, what are the key therapies you focus on at your hospitals?

Dr. Ankit Thakker: So, there is no key therapy that we focus on. We have always maintained that we would like a comprehensive service offering, which means we focus on everything from childbirth to newborn care. We do oncology, orthopedics, cardiology, organ transplants, all of it. So, there is no key therapy area because we believe

that healthcare has to be treated as a whole and we need to excel in all the branches of medicine to do justice to our patients. So, we don't have a key focus area. Our decision is that we should have a comprehensive offering.

Rohit Mehra: Yes, that's good. And my second question is, is cash realization different compared to the insurance? And why is government payment such a low number?

Dr. Ankit Thakker: So, government such a low number is by choice. We don't have empanelment with government schemes. There is significantly high insurance penetration in the communities where we are present. So, most of our patients come with health insurance, either one they bought on their own or provided by their employer. So, that is what forms a bulk of our payer mix and the balance pay out of pocket. So, because we don't have any government obligations, that is we have not taken any free land or any subsidy or any undue favors from the government, for that reason we are not even compelled to do those schemes.

Moderator: Thank you. The next question is from the line of Ashay Jain from Jain Capital. Please go ahead.

Jupiter Life Line Hospitals Limited
February 12, 2024

Page 5 of 10

Ashay Jain: I had one more question that our Thane Hospital is quite mature one. So, we can only expect inflation or some price hike led growth; however, in the Pune and Indore location, the hospitals can grow faster in order to have better growth. So, do we see adding bed capacity at a rapid pace over there?

Dr. Ankit Thakker: Yes, so you already answered a part of my question in your question. So, your analysis is correct and I agree with that. In Pune, I just answered the previous question where I said, we just have another 30 beds left to be commissioned, which are in a semi-ready state. Hopefully in the next year, we should start those, which is the end of capacity expansion for Pune. Indore, we also think we should be adding beds in the next year because we believe we will reach that 60 plus percent occupancy. So, that is where we are on bed expansion.

Ashay Jain: Okay, understood and apart from that, what is the overall expansion plan say within 5 years or five years down the line?

Dr. Ankit Thakker: Our stated objective is to go to 2500 odd beds in the next 5 years with a greenfield strategy that is a key focus and the direction to which we are working. Meanwhile, if some interesting

opportunity does present itself on the M&A side, we will evaluate it as it comes.

Moderator: Thank you. The next question is from the line of Nishi Shah from Aditya Birla Capital. Please go ahead.

Nishi Shah: I have a couple of questions. First is building a new hospital and ramping up them up requires a good amount of time. So, why don't we aggressively plan for inorganic acquisition?

Dr. Ankit Thakker: So, we prefer organic acquisition for a few reasons. One is control that means it helps us decide exactly where we want to be located, it helps us decide the size, scale, scope and design of the infrastructure, it helps us decide what kind of technology to buy, it allows us to set our own culture from the word go rather than inheriting something. So, the degree of autonomy in a green field is much higher and we don't look at this business or this hospitals as short horizon quick turn around, quick exit kind of a story. We think of living in those buildings for life and for that reason we believe that the first 2 or 3 years invested in doing it right help us reap the reward in the future for a very long time to come, so that is a key reason. Then also on the capital side, a great asset available for acquisition with everything that you like including financial metrics and locations and assets generally does not come at the cost at which you will be able to build yourself. It generally comes with a huge premium associated to it. In our assessment, those

kind of premiums are not something we are looking to pay, which is why I previously said that we evaluate opportunities from time to time. We found Indore a great opportunity for acquisition and we did that. So, if again some good opportunity does present itself, we are absolutely for it. But in our assessment, there are few and far between. So, that is why I don't call it my strategy, but we are open to it.

Jupiter Life Line Hospitals Limited
February 12, 2024

Page 6 of 10

Nishi Shah: Okay, that answers my question and I have one more question. What would be the key drivers for EBITDA growth in FY25?

Dr. Ankit Thakker: So, EBITDA growth, Thane again, as we discussed earlier, will be predominantly pricing, inflation etc. related growth. Pune, there will be a combination of pricing and occupancy led growth and Indore will have the third factor as well which is pricing, occupancy and improved case mix because it is just about getting matured, but I wouldn't say it is a fully matured hospital. So, in terms of case mix complexity, it has a little catching up to do. So, that is the third driver for Indore.

Moderator: Thank you. The next question is from the line of Ankit Shah from Canara Robeco AMC. Please go ahead.

Ankit Shah: My first question is on the employee cost. So, if I look at the employee costs have risen like 17% Y-o-Y and even on a Q-o-Q basis, it is higher. So, any particular reason for this, and what would be the sustainable number going forward?

Dr. Ankit Thakker: Hi, so employee costs in this quarter are higher. Traditionally, it has been higher in Q3 because in 3rd Quarter, we generally award the annual bonuses, and the 3rd Quarter number is higher on account of those bonuses. So, if you remove that bonus, the employee cost generally does not vary too much from quarter to quarter. It is generally flat throughout the year. The professional fee component sometimes does vary because that is dependent on the volume of work and the kind of work that the doctors do and it may sometimes vary with that. But employee cost is largely flat. Q3 is higher on account of bonuses.

Ankit Shah: Got it. So, this is expected to normalize going forward?

Dr. Ankit Thakker: Yes, so whatever you saw in Q2, I think, again, you should see in Q4 logically.

Ankit Shah: Got it. And my second question is related to Dombivli expansion. So, here, how much CAPEX has been incurred so far? And for FY25, what is the number you're expecting?

Dr. Ankit Thakker: So, far we have finished the excavation and just started constructing. So, in terms of number, it is not a big number. In FY 25, we would probably spend about Rs. 100 crores on the Dombivli project.

Ankit Shah: Got it. And thirdly, last quarter you mentioned that Indore hospital had achieved EBITDA breakeven but PAT breakeven is still not achieved. So, can you give some sense when would Indore achieve optimal levels of EBITDA margins because you highlighted some key levers for overall improvement over there. So, when would that optimal EBITDA margin be achieved in your view?

Jupiter Life Line Hospitals Limited
February 12, 2024

Page 7 of 10

Dr. Ankit Thakker: So, I believe that hospitals' greenfield in nature takes place in the years 4 to 6. In that 3-year period, they should sometimes reach optimal EBITDA margins. So, currently, Indore is probably third year. So, in the next year or two, we believe it should be in a comfortable position on the EBITDA margin side.

Moderator: Thank you. The next question is from the line of Ashay Jain from Jain Capital. Please go ahead.

Ashay Jain: So, a couple of questions. So, probably in the tier two cities like Indore and Pune like hunting for a good talent is an issue or it is not? And how do you safeguard or maintain the employee retention levels at your end?

Dr. Ankit Thakker: So, both Indore and Pune are quite prominent

cities today. Pune of course would probably rank higher than Indore in terms of its nature in the country. So, Pune has absolutely zero challenge. Indore also for I would say 90% of the problems, there are great doctors available locally for some super specialized kind of work. Indore, because it did not have great infrastructure on the healthcare side, was not able to attract doctors. The problem was not with the city, the problem was with the offerings of hospitals. Now, whenever we have tried and successfully attracted talent to the Indore hospital from other parts of the country, I don't see that as a challenge for both hospitals.

Ashay Jain: Okay understood, one last bit. So, do you give out any ESOPs or is there a structured policy or something for the same?

Dr. Ankit Thakker: We do not have ESOP program currently.

Moderator: Thank you. The next question is from the line of Abdul Kader Puranwala from ICICI Securities Limited. Please go ahead.

Abdul Kader Puranwala: So, my first question is on Thane Hospital. So, if you could explain how the ramp-up happened in this particular quarter. And while you're seeing some margin improvement in Indore and Pune, how has the margin profile of Thane been vis-à-vis it was last year?

Dr. Ankit Thakker: So, Thane is kind of a steady, nearing peak kind of a situation. So, nothing much has changed in Thane. The occupancies are improving in Pune and Indore, both being growth phases. In Pune, we added beds last year. Indore is getting established in the community and for those reasons we are seeing increased occupancy in Indore and Pune. Again, as I said, Pune also had a rate revision due from the insurance companies we negotiated, which has also helped to improve the revenues in ARPOB profiles. So, those are some factors for the three hospitals.

Abdul Kader Puranwala: And so the balance 200 beds, what you may add in Indore maybe next year. So, out of that, could you help us understand the spread in terms of the number of ICU beds which could get added or tertiary care beds over there?

Jupiter Life Line Hospitals Limited

February 12, 2024

Page 8 of 10

Dr. Ankit Thakker: So, we still have to plan which way what all we will add and how to do the offerings. We have flexible space available. So, it will be a mix of ICU and ward beds of different kinds, maybe some operation theaters. So, it will be a combination of all, but today I don't have an answer of exactly what that breakup will be.

Abdul Kader Puranwala: So, at the 60%-63% mark, it was kind of an occupancy. What will Indore reach? I assume you will reach the optimum level of margins as well. So, I mean, directionally, if you have to see in terms of the ARPOB or what would be the growth drivers from there on? I mean, would it be only the new bed situation or are there certain specialties where you're not seeing a ramp up right now and that you are currently working on?

Dr. Ankit Thakker: Yes, Indore will see both, occupancy and specialty drivers both. So, it is not that we are not focusing on specialty, but it is just a part of maturity of a hospital in a community, as we have noticed earlier than our understanding of the space, that you may have, for example, a neurology department, but you will not, you know, if someone gets a stroke in the middle of the night in the first year, there may not be either a top of mind recall or a confidence of the community in a new hospital. So, establishing of medical teams, establishing a route in the community, slowly growing volumes of all the branches, slowly attracting more and more tertiary care and quaternary care referrals because some of these things are a function of time, not necessarily the function of focus. Of course, in some other places, you like adding specialties, you don't start with a heart transplant in the first 6 months of operation. You start and establish your cardiology

or cardiac surgery or critical care and once all of that is established, then you start venturing into those things. So, some specialties get added over time and some are already there, but they take a little time to mature.

Abdul Kader Puranwala: Got it sir. And final one if I may. Sir, you've spoken about evaluating certain acquisitions in the western region. So, where are we on that? Have we had some initial talks for evaluating any of those assets what you have liked?

Dr. Ankit Thakker: Nothing really worth reporting right now, nothing that is very exciting and on the table. But of course, whenever there is, I will reach out to everyone to inform you. But currently nothing was informed.

Moderator: Thank you. The next question is from the line of Nishi Shah from Aditya Birla Capital. Please go ahead.

Nishi Shah: I believe we acquired hospitals; we have 96%. So, do we intend to take balance as well?

Dr. Ankit Thakker: So, the balance, small 2%-3% is with the local doctor there who is part of the group from the beginning and currently has a desire to hold on to the equity. So, that is where it stands.

Moderator: Thank you. The next question is from the line of Chintan Shah from JM Financial. Please go ahead.

Jupiter Life Line Hospitals Limited

February 12, 2024

Page 9 of 10

Chintan Shah: So, I just have one question. So, recently there have been news flows, I'm not sure whether the regulations are applicable, but the news flow is that the healthcare insurance customers they can encash or use the cash to facilitate any of the hospitals. Since we have such a huge share of insurance in our payer mix, so just wanted to understand whether this impacts us from a medium to long term perspective in any way. Just wanted to know your thoughts on this. Thank you.

Dr. Ankit Thakker: Hi Chintan. So, I don't see how it affects us in the short to medium term at all because we already have an agreement with all the insurance companies. So, nothing much should change there. In the long term, I think it is good because it makes insurance as a product more attractive to the community and I think if this does get implemented eventually and reduces the hurdles for claiming your insurance, then I think more and more people would choose to get insured. So, in the long term, it is favorable. Short to medium term, it does not make any difference in my view.

Chintan Shah: Understood. So, from an industrial perspective, it's good, but now say for hospitals, when we have this partnership in place and we have a certain payout structure, so how does that get impacted from a long-term perspective or is there no impact? How should we see this?

Dr. Ankit Thakker: I did not understand your question. Payout structure meaning?

Chintan Shah: No, so basically we have partners that we would have partnered with a lot of insurance companies. So, now when customers have the option etc. to go to any sort of hospital, so how does this partnership basically get impacted?

Dr. Ankit Thakker: No, so I don't think people are coming to us because the hospital of their choice is not accepting a health insurance. So, I think that premise is probably slightly flawed because generally, any hospital, even of medium, forget large size, should have some kind of insurance tie-ups. It is generally the smaller ones which don't have tie-up. So, I don't think that now because they are allowed to go to some of those smaller hospitals as well that is why they will choose to go there and they'll stop using Jupiter as their provider. So, I don't think that has any role to play.

Moderator: Thank you. The next question is from the line of Sanketa Kohale from Prabhudas Liladher. Please go ahead.

Sanketa Kohale: Sir, my question is regarding ARPOB across the units going forward?

Dr. Ankit Thakker: I just said a couple of minutes back that ARPOB driver in Thane will be inflation

in Pune will

be inflation and occupancy and in Indore will be inflation, occupancy and case mix.

Moderator: Thank you. The next question is from the line of Jainil Shah from JM Financials. Please go ahead.

Jupiter Life Line Hospitals Limited

February 12, 2024

Page 10 of 10

Jainil Shah: Just a bookkeeping question. Our employee costs have risen in almost 16% quarter-on-quarter. So, is this largely related to Indore Hospital or Pune bed additions, and have we completed our doctor hires or employee additions?

Dr. Ankit Thakker: Yes, so this is related to the annual bonuses which we pay in 3rd Quarter that is the added component on this quarter's numbers for employee cost. In the next quarter again, it will be similar to what you saw in Q2.

Moderator: Thank you. As there are no further questions from the participants, I would now like to hand the conference over to the management for closing comments. Thank you and over to you sir.

Dr. Ankit Thakker: Thank you Viren. Thank you everyone to join us for this call today. I hope the answers were satisfactory. If you need any more questions, feel free to reach out to us or the SGA team; they will contact us for appropriate responses. Thank you everyone. Have a great day and a great week ahead.

Moderator: On behalf of ICI CI Securities, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: May 2024

May 17, 2024

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: - Q4 & FY24 Earning Conference Call - Transcript

Reference : - Intimation of Earnings Conference Call dated May 06, 2024 and Audio Link of Analyst/ Investor Conference Call dated May 13, 2024

Dear Sir/Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q4 & FY24 Results Conference Call held on Monday, May 13, 2024 at 10:00 AM (IST) for the quarter and financial year ended on March 31, 2024.

The same will be available on the website of the Company at www.jupiterhospital.com.

You are requested to kindly take the afore-mentioned on record and oblige.

Thanking You,

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 13

“Jupiter Life Line Hospitals Limited Q4FY24 Earnings
Conference Call of ”

May 13, 2024

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MANAGEMENT : DR. ANKIT THAKKER – EXECUTIVE DIRECTOR &
CHIEF EXECUTIVE OFFICER , JUPITER LIFE LINE
HOSPITALS LIMITED
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MR. NITIN PATODI – HEAD OF FINANCE , JUPITER LIFE
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MS. FALGUNI SHAH – BUSINESS CONTROLLER ,
JUPITER LIFE LINE HOSPITALS LIMITED
SGA – INVESTOR RELATIONS ADVISORS
MODERATOR : MR. PARAM DESAI – PRABHUDAS LILLADHER

Jupiter Life Line Hospitals Limited
May 13, 2024

Page 2 of 13

Moderator: Ladies and gentlemen, good day and welcome to Q4 FY24 Earnings Conference Call of Jupiter Life Line Hospitals Limited Hosted by Prabhudas Lilladher Private Limited .

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing * then 0 on your touchtone phone. Please note that this conference is being recorded. I will now hand over the conference to Mr. Param Desai from Prabhudas Lilladher . Thank you, and over to you, sir.

Param Desai: Thanks, Muskan . Good morning, everyone.

On behalf of Prabhudas Lilladher , we welcome you all to Jupiter Life Line Hospitals Limited 's Q4 and FY24 earnings conference call. Today , we have Dr. Ankit Thakker, the executive director and CEO; Mr. Anand Apte, the chief strategy officer; and other members of the senior management team on the call .

I will hand over the call to Dr. Ankit for his "Opening Remarks ," and post that we will have the Q&A session. Over to you, Dr. Ankit.

Dr. Ankit Thakker: Thank you, Param. Good morning, everyone. I thank you for joining us on our earnings call to discuss the Business and Financial Performance of Q4 and the Financial Year '24 . I hope everyone had a chance to view our financial results and the presentation that was posted on the website and on the stock exchanges. I am accompanied by our Head of Finance , Mr. Nitin Patodi , our Company Secretary and Compliance Officer , Ms. Suma Upparatti and Ms. Falguni Shah, our Business Controller , and SGA, our Investor Relations Advisors , on this call.

As we end Financial Year '24 and begin a new one, one thing we can all agree upon is the fact that India is growing. India is growing in all spheres and has attained the mass and momentum that will be hard to stop. All segments of the society are moving higher up on the socio -economic ladder. Millions are pulled out of poverty , and a large new middle class has emerged in the country. The aspiration of an average Indian is increasing every year. With the democratization of mobile phones, data, and social media, pretty much each and every Indian today has visibility of what can be from the palm of their hands. A smartphone , air conditioner, and even air travel are not just for the elites anymore. An average Indian today aspires to better experiences and more comfort in life. The same is true for health care. Rural and Tier 3 India today is struggling for access. It aspires to healthcare facilities closer to their homes. However, urban India does have access. Their aspiration is for higher quality and a better experience of care. Jupiter is addressing the urban aspiration. We are committed to building and operating world -class hospitals and bringing care back into healthcare. We are focused on large, prominent urban centers in Western India and want to provide all health care services in each of our facilities to the dense residential micro markets where we are located. We approach the business with a relationship lens and have established strong ties with the community and doctors alike. . We don't distinguish between high and low margin specialties, rather we endeavor to cater to all the Jupiter Life Line Hospitals Limited

May 13, 2024

Page 3 of 13

needs of our patients. Each of our hospitals will provide all services, from community and primary care up to quaternary care , such as transplants and cancer treatments.

We also consider insurance providers as our partners in growth and have associations and tie -ups with virtually all of them. We realize that quality healthcare can only be affordable for most Indians with the help of insurance. The growth and penetration of corporate hospitals have increased

the demand for insurance ; similarly, higher insurance penetration improves demand for high quality corporate hospitals. We believe that this mutually reinforcing growth has also reached a critical mark and going forward , both the sectors will see healthy growth in tandem. Therefore, we affirm our optimism for the tremendous opportunity for quality healthcare providers in India for the foreseeable future.

Let me give you some key highlights :

We had decided to take the company public to accelerate its growth trajectory. We stated an objective to double our size to 2500 beds in the next five years. In pursuing this milestone, I am

pleased to inform you that we have signed the new land for a second hospital in Pune. This is in South Pune and is about a one hour drive from our current hospital in the west of the city. We will build 500 beds at this location. It is almost 3 acres of land currently structured as a 10-year flat non-escalating lease, and we have a right to buy the land any time after 3 years. This enables us to defer the CAPEX for a few years, protect the cash flows, and secure the certainty of ownership for the company soon. In Indore, our occupancy is nearing 60% of the installed capacity, and we think this is a good time to plan for capacity enhancement. We have secured permission to build two additional floors in Indore. We will be able to build about 75 beds or so in the new floors, taking our capacity north of 300 beds. This should cost around Rs 20 lakhs a bed, that is approximately Rs. 15 crores. In Pune, we will also commission 22 new suite rooms to enhance our operational capacity. Here, the cost will be less, about Rs 10 lakhs per bed, as the physical structure is ready, and we only need to finish some small interior-related activities. In Dombivli, the construction is progressing as per expectations, and the project is slated to be completed by Q4 FY26. In past calls, I informed you that we have become a debt-free company post IPO. This year, we have realized annualized cost savings of about Rs. 40 crores on that account. In Indore, all insurance empanelments are done now. The contracts have been renegotiated with insurance companies in Thane and Pune this year. And in February, March, we also took a small price hike for self-payers, which has partially contributed to improved ARB OPs in the last quarter.

Regarding financial performance, Q4 highlights that the total operational income for Q4 stood at Rs. 291.4 crores, representing an increase of 20.3% year-on-year. The EBITDA was Rs. 63.2 crores, an increase of 22.7% year-on-year, with margins of 21.7%. PAT for the quarter was 45.3%, representing a margin of 15.5%. The total operating income for the entire financial year was Rs. 1,073.4 crores, an increase of 19.9% year-on-year. EBITDA for the whole year was Rs. 242.1 crore, an increase of 18.9% year-on-year. The annual EBITDA margin is 22.6%. PAT was

Jupiter Life Line Hospitals Limited
May 13, 2024

Page 4 of 13

Rs. 176.6 crores representing a margin of 16.5%. The average occupancy for the financial year was 63.8% compared to 62.6% last year. The ARB OP is Rs 54,871 on a blended basis for all the units and the ALOS is 3.93 days. So these are some highlights, and I am happy to take questions and talk to all of you. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. First question is from the line of Amey from JM Financial. Please go ahead.

Amey Chalke: I have first question on the quarter. I don't know whether this was explained in the opening remarks, but there is an absolute jump quarter-on-quarter in employee costs and other costs. Can you provide the reason for the same and are there any one-offs?

Dr. Ankit Thakker: Yes, so in this quarter, we have, on the employee benefits side, gratuity and leave encashment provisions, which we do with actu

arial valuations at the end of the year. So the entire year's impact is in this quarter. And on the other expenses also, we have the annual whatever provisions and write-offs done in this quarter. For the annualized level, our provisions and write-offs stand about a quarter to 0.3% of annual revenue, that has an effect in this quarter. In this quarter, we also had a large marathon event to promote awareness of children's congenital heart diseases. So, there are some one-time expenses this quarter that have an impact.

Amey Chalke: So, Ankit, should we assume that, if not all but some of the items are a bit seasonal in nature, that will continue next year as well like the gratuity or the kind of marketing spend which we account for during the last quarter?

Dr. Ankit Thakker: Yes, it could be.

Amey Chalke: The second question I have is post Pune and Dombivli over the next two years, we will still have a good amount of FCF generation. Any plans for FCF, free cash generation after spending for Pune and Dombivli? So how do you plan to spend that free cash over the next two years? Any thoughts on the same?

Dr. Ankit Thakker: Yes, so like I said, our current short-term target is to get to 2500 beds. With Dombivli and Pune, we have a thousand bed visibility that takes us from 1200 to 2200 ballpark, which means that we want to get one more hospital on our horizon. Sometime this year we hope to secure one more land on which we can build another hospital, which will take care of deploying the cash that we generate.

Amey Chalke: Sure, and I assume that this would be in the nearby region, the Western region, which is the kind of strategy we have been employing so far.

Dr. Ankit Thakker: Definitely Western India.

Jupiter Life Line Hospitals Limited
May 13, 2024

Page 5 of 13

Amey Chalke: The last question if I can squeeze in. In Pune, the bed -per population is generally on the higher side if you compare across cities, but it seems that private chains may be on the lower side in terms of presence. So what metrics do you see when you choose the city to enter?

Dr. Ankit Thakker: So Pune, what is on the higher side? Your voice was slightly...

Amey Chalke: The bed per population is on the higher side.

Dr. Ankit Thakker: So two things need to be accounted for. One is, and I briefly referenced that in the opening remarks. One is that, the micro market. Pune today has a population of close to between 6 and 7 million, which is large, right? Traveling from one part to the other in Pune is becoming as difficult as traveling between Eastern Suburbs and Western Suburbs or Bombay, for example. So you will need healthcare solutions in all micro markets. You will not be able to centralize healthcare in large cities, and you expect that the whole city will gravitate toward one point to get its healthcare. So where the beds are located is an analysis which needs to be done on a granular level. The other thing which needs to be done is analyze the quality of beds. Urban India as I was saying today is not satisfied with any bed. More and more patients today are moving away from unorganized and poorly built hospitals to the organized health providers. So simple count of beds does not tell the whole story. You need to analyze the quality of beds and you need to analyze where they are located. So when we look at both these criteria, we realize that there are significant white spaces in Pune where the aspiration for good quality healthcare is not being met.

Moderator: Thank you. The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah: My first question is about the quantum of price that we've taken in the self-care category.

Dr. Ankit Thakker: About 5%.

Jainil Shah: And the insurance renegotiation in Thane has been done again in 4th quarter or was it done in 3rd quarter?

Dr. Ankit Thakker: Thane

and Pune third quarter.

Jainil Shah: Both of them were done in the third quarter. What's the overall CAPEX spending that we have planned for FY25 and FY26?

Dr. Ankit Thakker: So in FY25 and 26, Dombivli should be fully over. So we would be spending about Rs 300 odd crores for Dombivli and we would be spending maybe around Rs. 200-250 crores for the new Pune hospital. So that gives you to Rs. 550 crores and maybe as a maintenance or upgradation CAPEX of Rs. 40-50 crores in the current three hospitals, so around Rs. 600 crores CAPEX in the next year is on the cards.

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 6 of 13

Jainil Shah: So when do we expect the Pune Hospital to start the operations?

Dr. Ankit Thakker: So we just secured the land a month or so back. We are in the process of drawing up plans and getting environmental clearances, municipal approvals, etc. So once all those are ready, we'll announce the commencement of construction date.

Jainil Shah: I missed on this last one on tax rate. What's the tax rate guidance?

Dr. Ankit Thakker: 25% I think is the tax rate we are working with now.

Moderator: Thank you. The next question is from the line of Anjana Shah from Shah Investments. Please go ahead.

Anjana Shah: As you mentioned in your opening remark, we signed up for a new location in Pune. So would you just be able to help me understand what the CAPEX that we intend to spend and also in the first phase, how many beds would be included?

Dr. Ankit Thakker: So CAPEX should be Rs. 500 crores for the project and in phase one, we should commission something like 250 out of 500 beds.

Anjana Shah: Okay. Sir, also, could you explain the reason behind the lease and purchase options for land?

Dr. Ankit Thakker: Yes, to elaborate a little more on this structure, we have a definitive right to buy this land anytime after three years in our lease document. So the challenge we generally have with leased options is the uncertainty associated with it and permanently inflating lease expenditure. In our case, we have a flat lease which will not escalate for 10 years. And at any point after three years, we have a definitive right to buy. So this takes care of two of our concerns. Along with that, we get an opportunity to defer the CAPEX by 3 years at least and achieve some cash flow efficiency. So for that reason, we have negotiated this deal with the sellers.

Anjana Shah: That was helpful, sir. So just another question with regards to the Pune Hospital. So would this affect the performance of the existing hospital? And also by when do we expect to start the construction and commencement of the Pune Hospital?

Dr. Ankit Thakker: Yes, so as again to reiterate, our current Pune Hospital is in Baner, which is west of Pune. The new one we have is in the Ganga Dham area of Bibwewadi which is in the south of Pune. If you want to drive between the two facilities on a regular day, it would take about an hour. Pretty much both the hospitals have an entirely non-overlapping catchment. So we think it will only complement each other rather than competing with each other or cannibalizing each other's

performance because they are quite far from each other. As far as commencement goes, as I just told Ja nil that we are in the process of regulatory approvals, as soon as those get over, we will be in a position of greater certainty to announce commencement o f construction and from then on commencement of operations.

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 7 of 13

Moderator: Thank you. The next question is from the line of Nishi Shah from RH Investme nt Management.

Please go ahead.

Nishi Shah: So Pune is currently at 62% occupancy with an ARBOP of Rs. 53,878 with peak or maximum occupancy? How much ARBOP can we generate?

Dr. Ankit Thakker: So. I wouldn't think that occupancy should have a bearing on ARPOB

because ARPOB

essentially takes occupancy out of the equation, however, I will answer both the questions independently. I think on occupancy front, Pune, the current unit from 62 can easily grow 10 percentage points and go to mid -70% in terms of occupancy and ARPOB, we think that for the given time, it is at a good level and going forward it should see inflation -linked growth in the ARPOB.

Nishi Shah: And I have one more question. We plan to add 75 beds to Indore Hospital. So, how many more can we add?

Dr. Ankit Thakker: So, we are currently at 231, we can add 200 more. We have excess land, also in Indore we have 5 acres of land. Currently we are adding 75. Once these get occupied to a reasonable level, we'll plan the next phase of work.

Moderator: Thank you. The next question is from the line of Kunal Shah from Anova Capital. Please go ahead.

Kunal Shah: Sir, I have two questions; one pertains to our Pune hospital's CAPEX plans . Just to continuing with the previous -to-previous participant. Just wanted to check that have we incurred any CAPEX in FY24 with respect to Pune Hospital?

Dr. Ankit Thakker: No, we have not incurred CAPEX. We have paid some deposits for the lease, but we have not incurred any CAPEX yet. The CAPEX will only start after the approvals are in place.

Kunal Shah: And any anticipated ballpark number, would you like to give for FY25 CAPEX in the Pune hospital and how is the construction progressing?

Dr. Ankit Thakker: No. So, Pune is just at the approval stage, so construction has not commenced. Once the approvals are in place, depending on how many months they take ,, we will be able to say how much of the year remains and how much money we can spend in the year. But typically, in the first maybe. 3 -4 months it's excavation and site mobilization. You don't spend a lot of Money.

So, I don't expect very heavy CAPEX on the Pun e hospital in the current financial year.

Kunal Shah: And my second question is pertaining to our Dombi vli Hospital . So just wanted to broadly understand the demographic there and what is the competitive landscape ?

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 8 of 13

Dr. Ankit Thakker: So, Domb ivli is still part of Mumbai Metropolitan Region. It is on the real estate side, especially residents, but even commercial to a great extent , one of the fastest growing markets in real estate development in the MMR region. So , it has a huge new population coming up there. Besides the new population, which is coming, it already has an old residential population of about more than 2 million people in a 30 minutes driving distance in neighboring suburbs of Domb ivli, Kalyan, Ulhas Nagar , Ambarnath , Badlapur, etc. and between this population, old and new which is coming, there is no organized corporate health provider. So , we think significant latent demand is not being addressed currently.

Moderator: The next question is from the line of Pr olin from Edelweiss Alternat ives Public Equity.

Prolin: The first question is , in terms of this negotiations with insurance companies . Can you help us understand what kind of hike you have in both hospitals on average ?

Dr. Ankit Thakker: It is in the high single digits between 8% and 10% in Q three we did it. But yes , it is in the between 8% and 10% .

Prolin: For both the hospitals and Thane .

Dr. Ankit Thakker: Yes.

Prolin: And I mean in terms of A RPOB, y ou answered that in Pune and Thane , we will see an inflation led ARPOB kind of a growth. Also, in your starting comment , you mentioned that you don't differentiate between primary, secondary, and tertiary. But is there a mix change that will also help ARPOB going forward ? What are the challenges for that to not happe n because there ideally, we would assume that more secondary, tertiary will help in terms of ARPOB? But is that not the case , can you share some color there?

Dr. Ankit T

hakker: Case mix optimization as a driver of ARPOB is a definite factor in our view first 3 to 5 years of a new hospital because that is a time where various departments are still getting set up and established. That is a time where the hospital is developing relationships and roots in the community. New doctors are joining, new departments are being started. So, year-on-year case mix optimization will have some goal in the first up to 5 years in our view. After 5 years, we believe that most of the services you want to offer are already on offer; most of the doctors you want on your panel are already on your panel. After that, you will have limited levers to drive the case mix. The way we do things we don't try to push one branch at the expense of the other. We say that we are large healthcare providers in the region where we operate for that micro market of 30 -40-50 minutes driving distance and we will take care of everything that you need. We don't want to tell our patient that if you need a low-cost dialysis then you manage your problems. But if you need a transplant come to me or if traditionally people said Pediatrics was low margin kind of work, if your child is in trouble, go somewhere else but if you are in trouble, I will take care of you. So, those are not the body language or the communication we want to have with our community. We say that we will take care of anything that you need and provide.

Jupiter Life Line Hospitals Limited
May 13, 2024

Page 9 of 13

So, it is a little bit of a relationship business, it is not a fully transactional business. So, I think it needs to be done in the right way.

Prolin : No, that's very clear, Dr. Ankit, thank you for that. And my one more question that I wanted to ask you was in terms of Dombivli right, you mentioned that that micro market has an existing base of 2 million people then there is 30 -minute driving distance. So, is it fair to say that in terms of trajectory, occupancy, and ARPOB, this will be very much like what happened in your Thane hospital? Is there a fair assumption to make?

Dr. Ankit Thakker: Yes, that could be a fair assumption to make. We broke even Thane and were positive in the first year. Pune and Indore have achieved that milestone in the second year, but that is a fair assumption. But, however, to be conservative if you want to peg it in line of Pune and Indore and put it as tier 2 I wouldn't object to that.

Prolin : Sure. So, I mean more than break-even I wanted to understand the ARPOB and the occupancy trajectory.

Dr. Ankit Thakker: But occupancy and ARPOB will eventually determine the break-even metric you prefer.

Prolin : But in three years, it should be more nearer to Thane than Pune. Am I correct in that assumption?

Dr. Ankit Thakker: It should.

Prolin : And lastly on this new land for the new hospital in Pune, while you have a long term lease without any escalation, but there is no price which you have already agreed for the purchase, right? Am I correct there in my assumption?

Dr. Ankit Thakker: You are correct.

Moderator: Thank you. The next question is from the line of Harshal Patil from Mirae Asset Capital. Please go ahead.

Harshal Patil: I just need one clarification. So, this was again relatively capital expansion plans especially with the Indore facility where we said that we will be adding 75 beds and for the Pune thing will be putting up about 22 beds so. You just alluded to about Rs 10 lakhs per bed for Pune hospital. So, So, what would it be for Indore like for the 75 beds expansion plan?

Dr. Ankit Thakker: Rs 20 lakhs for Indore so around Rs 15 crores for Indore and Pune Rs 2-3 crores.

Harshal Patil: This pretty much is part of our Rs 600 crore CAPEX plan, right?

Dr. Ankit Thakker: Yes.

Jupiter Life Line Hospitals Limited
May 13, 2024

Page 10 of 13

Harshal Patil: Any ballpark number, when we look at the balance capacity at Indore

re to be put up, let's say once

these 75 beds are operationalized maybe two years down the line or something like that.

Dr. Ankit Thakker: Hopefully, these beds should be ready by the end of this year. So, next year we'll get to operate them. So, the general thought process remains the same: you should think of enhancement between 60% and 65% occupancy when you reach. So, when we reach that milestone, we'll think about capacity enhancement.

Harshal Patil: Just one last on Indore. In Indore, how is the overall competitive landscape panning up considering things like we got some big names out there Medanta and all. I just wanted to have some qualitative inputs from you on Indore especially and the way you see the city panning up, especially with respect to the healthcare that will be my last question Sir. Thank you.

Dr. Ankit Thakker: So, Indore again is a huge city and probably the most prominent city in Madhya Pradesh and the neighboring areas. The city itself has a population of about 4 million and very, very large drainage for healthcare. So, in terms of demand, there is a huge demand for healthcare and quality healthcare again. I think in terms of quality bed to population ratio it is not nearing

saturation yet. Also, as I was saying for Pune, you need to also look at the micro market so in the part of the city where we are located in that half of the city, we are probably the only high-quality health provider. So, I think Indore looks very promising, combining the micro market of where we are located and the overall size of the larger market.

Moderator: Thank you. The next question is from the line of Amey from JM Financial. Please go ahead.

Amey Chalke: I just had one or two questions related to the ARPOB and ALOS. So, in Indore Hospital particularly is there any absence of Power Specialty which you would like to add or something which could improve the ARPOB or do you think these ARPOB are by nature representing the market and they are largely saturated. And second question is, is there any way to increase ALOS going ahead which are closer to 4 currently some of the hospitals are operating it at peak. So, how do you see that improving in the future.

Dr. Ankit Thakker: So, Indore has both the drivers. It has not reached that full maturity in terms of specialties. So, Specialty mix as a driver we still have probably some more time to reach that maturity. So, I think within the next year or two, we should see a little bit of ARPOB improvement even on the account of include Specialty. But it has made significant progress in the last couple of years; for example, we are now doing complex cancer work. We recently did a skin cancer radiation. We did a single patient liver and kidney transplant in the same patient. So, on the case mix side, it is heading in the right direction, and we expect a little improvement there in the times to come. And what was your second question Amey?

Amey Chalke: It's related to a ALOS, we are at around 4.

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 11 of 13

Dr. Ankit Thakker: ALOS, we don't look at it as an organizational target. ALOS is a function of what treatment you are doing in your specialty mixes. So, and I just spoke about these transplants and liver and kidney and this patient what you do is going to stay for 15 days or more. So, if you do more and more of this, your ALOS will be higher, so you do more of medical critical work the ALOS is higher. You do more of routine surgical work the ALOS is shorter. So, ALOS cannot be compared on a hospital -to-hospital level, even within our group, leaving us alone to some other hospital chain. So, that is not a number which we look at.

Amey Chalke: From our point of view, like when we see the ALOS reducing, we get to know that the hospital can now occupy more patients basically and have the same capacity and generate more revenue.

So, that's why I was asking

directionally is there a way to gauge whether it will go down or you think it would remain same kind of work you are doing or kind of specialties you're adding or kind of transplants or the way they are ramping up basically?

Dr. Ankit Thakker: So, I'm sorry, but I disagree with that analysis. The conclusion is that reducing ALOS may not improve your financial performance or efficiencies because the inputs are different, but that might be a longish discussion. I don't know if this is the right platform for that.

Moderator: Thank you. The next question is from the line of Riya Jain from NM Capital Limited. Please go ahead.

Riya Jain: I have two questions. The 1st is how much price increase advantage can we expect from ARPOB in Q1 FY25 and also which hospitals have seen pricing increases?

Dr. Ankit Thakker: So, annually, as I was saying, except for Indore, which has a little bit of case mix component still left to be achieved. Inflation-linked ARPOB growth is all that we are hoping for and guiding for whatever is the inflation number you want to plug in so that is the ARPOB growth.

Riya Jain: My second question is like in terms of hiring, so generally at what stage of construction you start hiring in terms of support staff, nursing staff and doctors for any new hospital?

Dr. Ankit Thakker: Last quarter.

Riya Jain: And will the cost of this new hires be similar to Thane Hospital or materially different?

Dr. Ankit Thakker: No, it should be similar.

Moderator: Thank you. The next question is from Sanketa's line from Prabhudas Liladher. Please go ahead.

Sanketa Save Kohale: As you mentioned, the price hike has been taken across the self-care category. So, if you can quantify this number across unit wise, Thane, Pune Indore and typically, how should one see this number going forward?

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 12 of 13

Dr. Ankit Thakker: So, around 5% is the price rise for self-payers. This could happen on an annual basis, depending on local market conditions, but that is the thought.

Sanketa Save Kohale: Regarding 75 bed expansion in Indore and 22 bed expansion planning in Pune, do we need to hire more technical talent for this, and will we also be catering to specific therapies?

Dr. Ankit Thakker: They will be the general beds, they will cater to all the therapies, the new hires which we will need will only be the local operational in nature. So, for example floor doctors, nurses,

housekeeping, etc., but no senior or major hires for this new facility.

Sanketa Save Kohale: I have another question regarding the margins for the FY24, which is currently at 22.9% which is flat approximately. So, how should we see this number taking into account new additions across Pune and Indore?

Dr. Ankit Thakker: So, Pune, it is not a huge capacity addition. I don't think the margins can get impacted significantly for Pune. But Indore, in the first maybe three to six months of the new facility, you might see a higher fixed cost and not fully utilized so there could be a small impact on the Indore margins for one or two quarters, but then they should stabilize and then start improving going forward on account of operating leverages.

Moderator: Thank you. The next question is from the line of Harsh Bhatia from Bandhan AMC. Please go ahead.

Harsh Bhatia: Just one quick question. I joined the call late, but in terms of that, Dombivli asset the last time we spoke is that at your end, any Greenfield asset might take up to around 2 years to break even, which might be at a point of 40% to 45% occupancy. Please correct me if I'm wrong, but just for the first 12 months just for the Dombivli project, is there any understanding that what could be the incremental losses stemming from the project? I'm not sure whether we have disclosed that information or that level of detail, but anything would help.

Dr. Ankit Thakker: No, we have no

t gone into that level of detail, but maybe if you want to model, we can say about

Rs 2 crores a month or something of that sort Rs 25 crores could be.....

Harsh Bhatia: That would be assuming a 20% to 30% occupancy level. So, a Rs 2 crores to Rs 3 crores per month loss is what we should be expecting for the first 12 months?

Dr. Ankit Thakker: You could do that.

Harsh Bhatia: Sure. And at that point of time if I may ask what would be the number of beds that would have been operationalized, I think the overall project is 400-500 beds capacity.

Dr. Ankit Thakker: 500 beds we plan to start around 250 in Phase -1.

Jupiter Life Line Hospitals Limited

May 13, 2024

Page 13 of 13

Harsh Bhatia: So, that base you would be expecting a Rs 2 crores to Rs 3 crores per month EBITDA loss, you would say?

Dr. Ankit Thakker: You could factor in whatever you feel like around Rs 2 crores.

Moderator: Thank you. That was the last question for the day. I now hand the conference over to Dr. Ankit for from Jupiter Life Line for closing comments. Over to you Sir.

Dr. Ankit Thakker: So, thank you everyone for joining us this Monday morning. I hope that the questions were answered satisfactorily. If there is anything further you could definitely reach out to the SGA Team and they will put you in touch with us to address some follow on. All the best and have a good week ahead.

Moderator: Thank you. On behalf of Prabhudas Liladher Private Limited that concludes this conference. Thank you for joining us and you may now disconnect your lines. Thank you.

Call: Aug 2024

August 14, 2024

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: Q1 FY25 Earning Conference Call – Transcript

Reference : Intimation of Earnings Conference Call dated August 05 , 2024 and Audio Link of Analyst/ Investor Conference Call dated August 12 , 2024

Dear Sir/Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q1 FY25 Results Conference Call held on Monday, August 12, 2024 at 10:30 AM (IST) for the quarter ended on June 30, 2024.

The same will be available on the website of the Company at www.jupiterhospitals.com .

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking You,

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 9

"Jupiter Life Line Hospitals Limited
Q1 FY25 Earnings Conference Call"
August 12, 2024

"E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 12th August 2024 will prevail."

MANAGEMENT : DR. ANKIT THAKKER – EXECUTIVE DIRECTOR AND
CHIEF EXECUTIVE OFFICER
MR. ANAND APTE – CHIEF OF BUSINESS AND
STRATEGY
MR. NITIN PATODI – HEAD OF FINANCE
MRS. SUMA UPPARATTI – COMPANY SECRETARY AND
COMPLIANCE OFFICER
MS. FALGUNI SHAH – BUSINESS CONTROLLER
SGA - INVESTOR RELATIONS ADVISORS

MODERATOR : MR. HIMANSHU BINANI – ANAND RATHI SHARE AND
STOCK BROKERS

Jupiter Life Line Hospitals Limited
August 12, 2024

Moderator: Ladies and gentlemen, good day and welcome to the Jupiter Life Line Hospitals Q1 FY25 Earnings Conference Call hosted by Anand Rath Share and Stock Brokers . As a reminder, all participant line will be in listen -only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star t hen zero on your touch -tone phone.

Please note that this conference is being recorded. I now hand the conference over to Mr.

Himanshu Bi nani from Anand Rath Share and Stock Brokers . Thank you and over to you, sir.

Himanshu Binani: Thank you. Sejal. Good day everyone and on behalf of Anand Rath Institutional Equities, I welcome all the participants on the call today. We have with us the management of Jupiter Life Line Hospitals to be represented by Dr. Ankit Thakker, ED and CEO of the company along with the senior management team. So without any further ado, I would like to hand over the call to the management for their opening remarks, post which we'll take the floor open for Q&A. Thank you and over to you, sir.

Ankit Thakker: Thank you, Himanshu. Good morning, everyone. Thank you for joining us on our earnings call to discuss the business and financial performance of Q1 FY25. I hope everyone had a chance to view our financial results and the presentation which were uploaded on our website and the stock exchanges.

I'm accompanied by our Chief of Business and Strategy, Mr. Anand Apte , our Head of Finance, Mr. Nitin Patodi, our Company Secretary and Compliance Officer, Mrs. Suma Upparatti and Falguni Shah, our Business Controller along with SGA, our Investor Relations Advisors on this call. We've had a good beginning to FY25. All three of our hospitals have been awarded the gold tier for the NABH Digital Accreditation.

All three of our hospitals are now also offering da Vinci robotic surgeries in addition to the robotic orthopedic surgeries that they were offering earlier. The capex for expansion is going as per plan. We have commissioned an additional 22 beds in Pune on 1st July'24. The construction activity for our expansion in Indore has also commenced. We will be adding about 75 beds there. They are expected to be ready by the end of this financial year and commence operations in early Q1 FY26.

The construction in Dombivli is also progressing as per plan. Our second hospital in Pune has now begun application for regulatory permissions. We hope to secure all the necessary clearances in the next one or two quarters and then begin with the construction there as well. We shall keep you apprised of the progress on that front in our future calls. Coming to this quarterly financial performance, the total income for Q1 FY'25 stood at INR 288.6 crores, an increase of 18.2% year -on-year. EBITDA for the quarter stood at INR 65.3 crores, an increase of 20.9% year -on-year. The margin was 22.6%. PAT for the quarter, 44.6% (Please note 44.6% was mentioned erroneously, it should be read as INR 44.6 crores) , representing a margin of Jupiter Life Line Hospitals Limited

August 12, 2024

15.5%. The average occupancy was 63.9% in Q1 FY25, compared to 57.2% Q1 FY24. The ARPOB for this quarter was INR59,700 and the ALOS was 3.76 days. So, with this, I open the floor for questions and answers and I am happy to discuss what you would like. Thank you.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Aman Vishwakarma from Phillip Capital. Please go ahead.

Aman Vishwakarma: Thank you for taking my question. I just had a question on your robotic surgery. So, have you seen any sort of traction there? Because I don't believe a lot of insurance companies also cover those.

So, I mean, if you could give us some insight on that. Thank you.

Ankit Thakker: Hi, Aman. So, robotic surgeries, as I said, the orthopedic robotic surgeries we have had for a couple of years and they have been doing well. They are covered by quite a few of the insurance companies. The da Vinci robotics have just been installed maybe around a month back. So, it's too early to talk about traction. But we are in dialogue with insurance companies and quite a few of them have come on board to that as well. But as we have, you know, used it for more time, I'll be happy to update you of how it's going.

Aman Vishwakarma: Okay. And just quickly on the ARPOB . So, how are our ARPOBs in the robotic surgeries or the da Vinci robot surgeries different from standard ARPOBs, if you could just give me a broad number?

Ankit Thakker: So, we don't track surgery or procedure -wise ARPOBs or department -wise ARPOBs very closely. But broadly speaking, the consumables and the, what do you say, the cost of using the robot would probably push up the cost of surgery maybe between a lakh and two.

Aman Vishwakarma: Okay. Got it. Excellent. Thank you. That's all from me. Thank you.

Moderator: Thank you. The next question is from the line of Karan Mehra from Mehta Investments. Please go ahead.

Karan Mehra: Hello, sir. Thank you for the opportunity. A couple of questions. Sir, on a quarter -on-quarter basis, our revenue has been muted. So, if you can please highlight the reason for the same.

Ankit Thakker: Hi, Karan. So, over the past calls and whenever we have spoken, it has been discussed that the seasonality as far as our business goes is to the tune of maybe around 10% or so, with Q1 and Q3 being relatively weaker quarters. Q1 has traditionally been a weak quarter because a lot of elective work gets, pushed out of the quarter because of vacations and things like that in this part of the world.

So doctors want to travel, patients want to avoid what they can. And similarly Q3 with the weddings and festivals and things like that the lifestyle -related procedures tend to get avoided in Q1 and Q3. So for those reasons, these two are traditionally weaker quarters. So this is as per expectation.

Jupiter Life Line Hospitals Limited

August 12, 2024

Page 4 of 9

Karan Mehra: Okay. And sir I believe so the occupancy at Thane Hospital was at around 69% in Q1. So this was on the lower side compared to previous quarters. So I believe the seasonality which you spoke this is related to the same?

Ankit Thakker: Correct.

Karan Mehra: Okay sir. That answers my question. Thank you.

Moderator: Thank you. The next question is from the line of Nishi Shah from RH Investment . Please go ahead.

Nishi Shah : Hello, sir. Thank you for the opportunity. I have a couple of questions. So are we planning any price hikes and self -payers in any of the hospitals in FY25? And if so when are we planning to hike and in which hospital?

Ankit Thakker: So, I do not think there is a plan of self -payer price hike currently in this year. We may do it at the end of the year or beginning of next year as far as the self -payers are concerned.

Nishi Shah : Okay, sir. And we added 22 beds in Pune Hospital. How much capex did we do and further is there any scope of adding few more beds in Pune Hospital or have we reached the maximum capacity?

Ankit Thakker: We just have a semi -finished ICU for 12 beds which has to be commissioned in Pune. Apart from that, there is nothing more which we can do in Pune. So some of the capex for these beds was already incurred in the previous period where we had again finished the raw shell and part interior, etc . In the current quarter or two we have probably spent about INR 10 lakhs, INR 15 lakhs per bed for the finishing of these beds.

Nishi Shah : Okay, sir. That answers my question. Thank you sir and all the best.

Moderator: Thank you. The next

question is from the line of Ameya from JM Financials. Please go ahead.

Ameya : Yes thank you for taking my question and congrats on this set of numbers. Ankit the first question I have is on the expansion side. Last quarter if I remember correctly we had given an indication of adding one more hospital maybe second half of this year. So how things are panning out on that front?

Ankit Thakker: So yes you are right. We have a desire to add one more hospital in this financial year. We have a couple of negotiations underway, but Greenfield projects mean that you have to first buy land and in India that comes with its own set of timelines. So today I can't give you a definitive answer about where it is and when it will come up. But we are quite optimistic that in this year, we should be able to do one more.

Ameya : Sure so far we have the Dombivli which is coming in FY27 or maybe a few end of FY26 then there will be another Pune hospital which is coming up , but for next one and a half year how the growth would pan out and from where it will come from?

Jupiter Life Line Hospitals Limited

August 12, 2024

Page 5 of 9

Ankit Thakker: Yes, so Dombivli will come, then Pune will come and then the year after that we hope the third one will - we should be able to launch. So that is a longer term growth pipeline. The shorter term growth pipeline will predominantly be from Pune and Indore. With Pune still being at 53% of occupancy last year in Q1 has grown to 62% in this quarter. So it still has about a 10% headroom for growth in terms of occupancy. So Pune should contribute to growth over the next year and a half.

Indore similarly from last year around 46%, 47% has grown to 57% in this quarter in terms of occupancy. So Indore has a headroom for growth and plus in the next year as I said we'll also

add 75 more beds. So, on an expanded base we will have a still bigger headroom for growth in Indore. So these are the two principal areas of growth. Thane, of course, is around 70% odd of occupancy. So its job is mainly to continue doing the performance that it is and continue generating the cash that it is in order to fund our future growth.

Ameya : Sure. So assuming all these three hospitals get matured over, let's say, next three years potentially is it fair to say that we could reach 25% plus margin?

Ankit Thakker: Yes. So around 25% give or take something is a respectable margin according to us and that can be expected.

Ameya : Sure. Thank you so much. I will join back.

Ankit Thakker: Thank you.

Moderator: Thank you. The next question is from the line of Raj Mehta from Wisdom Advisors. Please go ahead.

Raj Mehta: Yes. Hello. Good morning, sir and sorry for joining late. So I have a question on the robotic surgery part. So we have started robotic surgery across our hospitals. So can you please highlight which of the major divisions will get a benefit and also how will it improve our overall revenue?

Ankit Thakker: Hi, Raj. So, we did talk a little about this, but robotic surgeries will predominantly be for abdominal surgeries, for Urology, for Gynaecology, for general surgeries, for cancer, for transplants. Predominant indications are these for the robotic surgery. And there are certain clinical advantages associated with it. So, those will be passed on to the patient.

In terms of revenue increase, I don't really think we, at a blended, consolidated level, adding one or two these kind of branches or services will have a huge or a meaningful impact for you to notice from the outside. So, it is more of keeping in line with the medical advancements and offering the patients the best choices to seek their treatment.

Raj Mehta: Got it, sir. And on the location front, so apart from New Pune, which other locations are we looking for? So, is there anything in the final negotiations?

Ankit Thakker: Yes. So, again, as I said, we prefer to remain

restricted to Western India for this phase of growth.

And we are looking at certain locations across Maharashtra and Gujarat. So, yes, I think I really

Jupiter Life Line Hospitals Limited

August 12, 2024

Page 6 of 9

can't get into too much detail about the location of the land because I don't want to engage in speculation. But as soon as we have something concrete to announce, I will definitely do this. We are optimistic to close something in this year.

Raj Mehta: Got it, sir. That's it from my side and thank you and all the best.

Ankit Thakker: Thank you, Raj.

Moderator: Thank you. The next question is from the line of Harshal Patel from Mirae Asset Capital Market. Please go ahead.

Harshal Patel: Thank you, sir. And thanks for the opportunity. So, I just have two questions to ask. One is for the Indore addition of 75 beds, typically what would be the capex per bed that we'd be targeting considering that there is some bit of construction already being done?

Ankit Thakker: Yes. Hi, Harshal. So, for Indore, we'll probably incur INR 20, 25 lakhs sort of capex for the new beds being constructed.

Harshal Patel: Okay. So, that's it. And so, just one point to understand. On the NABH acquisitions GoI standard that we've got for all the three hospitals, from an understanding perspective, sir, does this by any way help us in pricing renegotiations with the insurance companies or what can be a benefit that we could see out of that?

Ankit Thakker: No. So, this is not done with a pricing angle in mind. It is more of continual quality improvement. So, we have always been NABH accredited for the healthcare services. Recently, there is also an initiative to improve the digital spine of the hospital. And this is a new kind of accreditation initiated for the digital framework in the hospital. So, we volunteered and got ourselves accredited on that front as well. So, it is just a commitment to quality. It is not really done with a pricing angle.

Harshal Patel: Okay. That's it, sir. Thank you.

Ankit Thakker: All right. Thank you.

Moderator: Thank you. The next question is from the line of Aman Vishwakarma from Phillip Capital PCG. Please go ahead.

Aman Vishwakarma: My question is on the case mix that you operate in. So, I don't think you give out that data. So, if you could give us a broad number as to what cases predominantly are something that you cater to?

Ankit Thakker: Yes. So, we are truly a full service community hospital. So, we do everything from childbirth going up to transplant, oncology, everything. We don't really have a specific focus area in any of our hospitals. There is not a single branch which probably would go significantly above 15% or maybe not even 15%. So, it is well diversified and it is spread out quite evenly.

Aman Vishwakarma: And just I think, excuse my ignorance, but this has been answered previously. I mean, so how are we seeing that ARPO B growth coming through going forward? I mean, if not the case mix, do we expect any other driver to drive the growth in ARPO B?

Ankit Thakker: Yes. So, Thane and Pune, ARPO B will predominantly be driven only by inflation now. In dore case mix will have some role to play. So, when I say case mix, it is, complexity and the frequency of complex procedures. So, in a, let's say when you start a new hospital on day one, you may not be doing everything right. On day one of starting a new hospital, for example, you may not have a liver transplant team.

But over the years, once your gastroenterology, pathology get established, then you start doing transplant. But the day you, so that is an introduction of a new complexity in case mix, which is at the higher end of the spectrum and which can improve your ARPOB. The second driver now, which is happening in Indore for case mix, is that you have a transplan

t team, but the day you

have a team, they don't start becoming fully busy or occupied.

They take a couple of months or years to fully settle down and get fully utilized. So, that is a second driver of case mix. That is a frequency of use. So, currently, Indore is in that second phase. The first phase of introduction of new specialties is pretty much over. The second phase of, getting matured into doing more tertiary and quaternary work is underway. So, maybe for another year or two, Indore will see this case mix driver. After that, Indore should also slide into this inflation-linked growth orbit.

Aman Vishwakarma: Okay, got it. And just lastly on our doctors. So, I mean, are we doing anything different to attract more talent to Jupiter? I mean, giving out there, I mean, so what is the structure like to hiring and, retaining them broadly?

Ankit Thakker: Yes, I don't know if I can give you anything concrete on that front. It is there are a lot of things which are cultural in nature in terms of manpower engagement of all kinds doctor included. So, yes, I don't have a short answer to this question. But we do have a strong team of doctors in all our units. And we have a very, very low attrition rate in doctors, maybe very low single digits.

So, I don't know if that helps you.

Aman Vishwakarma: Okay, got it. Okay, that's all for my end. Thank you.

Moderator: Thank you. The next question is from the line of Aditi Sawant from ADM Advisors. Please go ahead.

Aditi Sawant: Yes, hi, thank you so much for the opportunity. I have a few questions on a Dombivli hospital. I just wanted to understand that as of Q1 FY '25, how much capex we have incurred for that hospital including land?

Ankit Thakker: So, we have incurred about INR 95 crores on Dombivli so far.

Jupiter Life Line Hospitals Limited
August 12, 2024

Aditi Sawant: Okay, and have we started tying up with doctors for the same hospital like when that process usually starts? I just wanted to get a sense on that. And what steps will you need to take to attract footfall initially?

Ankit Thakker: So, no, we haven't started engaging with doctors yet. It is too early for that. We'll probably start one quarter before we are ready to operationalize. So, that should be a good enough of a time to start team building. As far as attracting footfall, so, I don't know if you're aware of the geography of this region but Dombivli is kind of an extended training area or north of Thane. So, both in terms of doctors and the residential community Jupiter is quite well known and respected as a high-quality provider. So, we will definitely enjoy that brand recognition once we start there.

Aditi Sawant: Okay. Thanks. Thanks for that and all the best for upcoming quarter.

Moderator: Thank you. The next question is from the line of Ruchi Gupta from Value Capital. Please go ahead.

Ruchi Gupta: Yes. Good morning, sir. I just, I have two questions basically and I want to ask you the experience in an Indore hospital. Do you think that we will be able to add one more in that location?

Ankit Thakker: So, Ruchi, currently, there is no plan to add one more in Indore but the experience is very encouraging and we are getting extremely strong feedback and response from the local community and the doctors both. But currently there is no plan to add one more Indore.

Ruchi Gupta: Okay. All right. One more question, sir. Can you please share demographic of both the locations which are there at Pune?

Ankit Thakker: So, you mean geographical distance between them? What do you mean?

Ruchi Gupta: A demographic of both the locations.

Ankit Thakker: So, first one that we are operating now is in Baner, which is west of Pune. That area is relatively newer developed area of Pune, which is predominantly driven by the IT growth, which happened. So, with Hinjewadi on the far west and then Baner, Balewadi, Sus Pashan, Bavdh

an

these parts of Pune subsequently have developed very, very strongly.

So, this is a relatively younger and newer part of Pune with a significantly dense new population.

As against that the new location which we are doing is in the south of Pune, which is a very, very dense residential population not much commercial activity happening there but very dense residential population, which is not very new but a traditional part of Pune. And both the locations are probably maybe around an hour away as a driving distance course.

Ruchi Gupta: Okay. That is very helpful. Thank you, sir.

Moderator: Thank you. The next question is from the line of Rohan from Anand Rathi. Please go ahead.

Jupiter Life Line Hospitals Limited

August 12, 2024

Page 9 of 9

Rohan: Thank you for the opportunity and congratulations on a good set of numbers. So, I just had one question. So, post 1Q, what is the net cash available with the company? Thanks.

Ankit Thakker: So, net cash at the end of this quarter should be something between INR250 crores to INR300 crores.

Moderator: Mr. Rohan, does that answer your question?

Rohan: Yes. Thank you.

Moderator: Thank you. As there are no further questions, I would now like to hand the conference over to the management for closing comments.

Ankit Thakker: So, thank you very much for joining us on this call. I hope that we have been able to answer all the questions satisfactorily. But if you need any more questions or clarifications, please do not hesitate to contact our IR team and they should be able to connect you and answer the questions.

Thank you so much. Good day.

Moderator: On behalf of Anand Rathi Shares and Stock Brokers that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Nov 2024

November 14, 2024

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980

Subject: Q2 & H1 FY25 Earnings Conference Call – Transcript

Reference: Intimation of Earnings Conference Call dated October 29, 2024 and Audio Link of Analyst/ Investor Conference Call dated October 11, 2024

Dear Sir/ Madam,

Pursuant to Regulation 30 of the SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q2 & H1 FY25 Results Conference Call held on Monday, November 11, 2024 at 10:30 AM (IST) for the quarter and half year ended on September 30, 2024.

The same will be available on the website of the Company at www.jupiterhospital.com.

You are requested to kindly take the afore-mentioned on record and oblige.

Thanking you.

For JUPITER LIFE LINE HOSPITALS LIMITED ,

Suma Upparatti
Company Secretary & Compliance Officer

Encl.: As stated above.

Page 1 of 7

“Jupiter Life Line Hospitals Q2 & H1 FY25 Earnings
Conference Call ”

November 11, 2024

“E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange on 11th November 2024 will prevail.”

MANAGEMENT : DR. ANKIT THAKKER – ED & CEO, JUPITER LIFE
LINE HOSPITALS LIMITED
MR. ANAND APTE – CHIEF OF BUSINESS & STRATEGY ,
JUPITER LIFE LINE HOSPITALS LIMITED
MR. NITIN PATODI – HEAD OF FINANCE , JUPITER LIFE
LINE HOSPITALS LIMITED
MRS. SUMA UPPARATTI – COMPANY SECRETARY &
COMPLIANCE OFFICER , JUPITER LIFE LINE
HOSPITALS LIMITED
SGA – INVESTOR RELATIONS ADVISORS

MODERATOR : MR. HARSHAL PATIL – MIRAE ASSET CAPITAL MARKETS

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 2 of 7

Moderator: Ladies and gentlemen, good day, and welcome to Q2 & H1 FY25 Earnings Conference Call of Jupiter Life Line Hospitals hosted by Mirae Asset Capital Markets .

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Harshal Patil from Mirae Asset Capital Markets. Thank you, and over to you, sir.

Harshal Patil: Thank you, Sejal. Good morning, all. On behalf of Mirae Asset Capital Markets, I welcome you all to Q2 and H1 FY25 Post-Results Conference Call of Jupiter Life Line Hospitals .

From the management , we have Dr. Ankit Thakker – the ED and CEO, along with other Senior Management Team members. Dr. Ankit, over to you for the initial comments.

Ankit Thakker: Thank you, Harshal . Good morning, everyone. Thank you for joining us on our earnings call to discuss the business and Financial Performance of Q2 and H1 FY25. I hope all of you had a chance to go through the “Financial Results ” and the “Presentation ” posted on the website and the exchanges.

I am accompanied by Mr. Anand Apte, our Chief of Business & Strategy ; Mr. Nitin Patodi – our Head of Finance, and Suma Upparatti – our Company Secretary & Compliance Officer, along with our Investor Relations Advisors, SGA.

So, we got listed in the 2nd Quarter of last year. Today marks a year of us meeting over these calls. After a year of doing this, realization dawned upon me. All of you have signed up for our Earnings Call , not for my Mann Ki Baat . So, out of respect for all of you and your time, beginning today, we will do away with generic opening remarks. Of course, whenever there is something substantial to announce, we will talk about it .

Let us move straight to the numbers and then dive into the Q&A :

For the quarter, the income stood at Rs. 324.2 crores, representing an increase of 22.7% year -on-year. The EBITDA was Rs. 76.6 crores, representing 23.5% growth. The EBITDA margin was 23.6%. The PAT at Rs. 51.5 crores, an increase of 52.9% YoY . The PAT margin is 15.9% for the quarter. The half yearly numbers, the revenue was Rs. 612.8 crores, a 20.5% increase YoY . EBITDA Rs. 141.9 crores, 22.3% increase YoY . The margin is 23.2% of EBITDA. The PAT is Rs. 96.1 crore in the first half, representing a 9.5% increase. And the PAT margin is 15.7% . The ARPOB for the first half is Rs. 57,700 as against Rs. 53,100 for the same period last year. ALOS is 3.85 days compared to 3.89 days last year.

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 3 of 7

The occupancy for the period this year was 67.2% as against 62.3% last year. The overall volume has been 486,000 in the first half compared to 435,700 last year. The payer mix insurers contributed 55.4% of the revenue, self -payers 53.5% and the government schemes at 1.1%. So, these are the headline numbers , and I am happy to answer your questions.

Moderator: Thank you very much. We will now begin the question and answer session. The first question is from the line of Dr. Sandeep, who is an individual investor. Please go ahead.

Dr. Sandeep: If I can be indulged, I have two small questions. The first one is on having this doubt on the auction process in Andheri to Medanta. Do we have any plans now to have any inorganic growth opportunities in our planning process? This also becomes important from the fact that KIMS also is coming up very soon in Hiranandani estate in Thane.

Ankit Thakker: Yes, thank you Dr. Sandeep. So, we keep on exploring growth opportunities from time to time. Currently , we have two new Greenfield

projects underway. One in Dombivli which we have started constructing for 500 beds. The second one in Pune is under regulatory clearances. We expect approvals in the next quarter and plan to start construction soon after. So, the thought process is that we must expand some more. Current geography in mind is Western India. While we will keep looking at opportunities, we also are mindful of the value we need to pay for to pursue that growth. So, wherever we have that opportunity and value intersection , we will go ahead and do that.

Dr. Sandeep: That is true. And if you please do not mind, just a quick one on that. We do know that Thane Hospital is the star hospital. Invariably, this has gotten a very decent occupancy level. So,

would there be an idea to take over the Fortune Hotel property and convert entirely into the hospital?

Ankit Thakker: So, yes, Thane is the first one which we started. Our thought process is that there should not be one star in the group. All the hospitals are designed to deliver at a similar level. They are built similarly, they are sized and scaled similarly. So, as all of them achieve maturity, which Pune has already pretty much achieved, you will eventually find there are no star hospitals. To answer your specific question about Fortune, even today as we speak, Fortune occupies 1 think 2% or 3% of the built-up space, less than 4% in any case. And it is not a huge footprint. It is more of an amenity that helps us deliver better care. It is a source of comfort for the patients and the doctors and the community and which is why it exists, it will not make too much of a difference.

Moderator: Thank you. The next question is from the line of Rohit from SK Securities. Please go ahead.

Rohit: I have a couple of questions. And the first one is in H1 FY25, our overall volume and EBITDA increased by 12 % and 9 % on YoY basis, respectively, right. And can you please highlight for Q2 FY25 as well?

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 4 of 7

Ankit Thakker: So, it is part of the growth process generally, you know, there is no magic that we can spin quarter to quarter. As this year against last year, we have had a little bit of increase in bed occupancy, I mean bed capacity availability and Indore being in its growth phase now, you are seeing that growth in terms of numbers how to extrapolate it. You guys are more adept at it than I am. In terms of business guidance, what I can tell you is that the Pune 22 beds which were just added in this quarter will continue to get more occupied in the second half of the year. In Indore, we are adding 75 to 80 odd beds, which are expected to begin operations in the next quarter so they should start contributing a little bit. And Thane is currently at good occupancy, so it should continue performing the way it is.

Rohit: My second question is a little bit of broader level that how do you see the healthcare landscape evolving over the next few years and how is Jupiter positioning itself to maintain its leadership role in the western region?

Ankit Thakker: So, healthcare landscape has a huge headroom to grow because if you look at both the metrics because what we strongly believe is that you need to analyze the quantity and quality both. The easier thing is to analyze quantity if you look at the number of beds currently in the areas where we operate in western India versus what the desired numbers, you will see that there is a huge gap. But if you do a secondary analysis and say that, okay, from the available beds, how many are of the quality that I desire, then you will see that the gap is still bigger. Even in a city like Mumbai, a large share of healthcare is provided by the unorganized sector, which is small nursing homes and not large, organized healthcare providers. So, over the next decade, I think both the factors will play in a) new capacity will get added and b)

the old unorganized capacity

will eventually have to get replaced by new organized facility. So, this is on the supply side.

On the demand side, we believe now that the insurance sector in healthcare has reached a critical mass and from here forward it has only one way to go, and that is north. So, with increased insurance penetration, the affordability for healthcare of a common man will drastically increase. And once people are insured, which means they don't have to pay out of pocket, then their desire for a higher quality consumption goes up disproportionately because if I had to pay out of pocket, I am willing to make some compromises, but if my insurance provides for my care, then who wants to go to those dimly lit unorganized places. Everyone wants to go to a place where they can fully trust the quality, so I think are two key factors which will play out over the next decade.

Moderator: Thank you. The next question is from the line of Shivam Agarwal from Mirae Asset Capital Market. Please go ahead.

Shivam Agarwal: So, I have a couple of questions. So, firstly on the Indore side, Indore facility. So, can you provide for the occupancy for the current quarter?

Ankit Thakker: So, it was around 61% in this quarter, the Indore occupancy.

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 5 of 7

Shivam Agarwal: And for the first half, we have reported 59% of the occupancy. So, how can we see ramp up over the next 1 to 2 years post like 75-bed addition that we have spent?

Ankit Thakker: Yes, so in percentage terms, next quarter you will see a dip because the base will go become bigger. But in absolute terms, you will see that the occupancy will be higher. Typically, when we achieve a +60% occupancy, we want to expand more beds. So, between 60 % and 65%, we always try to add more beds if it is possible. So, now we have crossed that 60% mark in the

next quarter, we add these new beds. So, in absolute terms, they will keep growing, but in percentage terms, you will see a little drop. Once in the expanded base, we again cross 60 %-65%. We will try and explore the possibility of adding some more beds in it.

Shivam Agarwal: Okay, so like I guess like 200 odd beds can be added in the Indore facility that I think I remember correctly. So, out of the 75 like 125 beds, more can be added in the facility. Am I correct?

Ankit Thakker: Yes, something like that.

Shivam Agarwal: Okay, and second one is on the CAPEX front. So, like we have few Greenfield opportunities in Dombivli and Pune facility. So, like any update on both the facilities?

Ankit Thakker: Yes, Dombivli is progressing as per schedule. We should expect to start operations in the calendar year 26. Pune is in the regulatory phase. We have cleared up a few steps. Currently, because of elections in Maharashtra, the government is not really on for too much of clearances, but it has made progress. I think in the next quarter we should have all approvals and then thereafter we should be able to begin construction in Pune as well.

Shivam Agarwal: what is the CAPEX plan for FY 26 like CAPEX number if you can just provide it?

Ankit Thakker: For Pune?

Shivam Agarwal: Like overall CAPEX for the whole company as a whole?

Ankit Thakker: So, overall company in the next year I think the main CAPEX will only be for Dombivli and Pune unless of course we get new land, and we start a new project. So, I am not talking about that now because there's nothing to announce. But from the existing what we have, Thane, Pune and Indore, all three are pretty much set in terms of their CAPEX needs for the time being. Dombivli will need Rs. 150 odd crore I think in the next year and Pune should also need maybe Rs. 50 plus crore in the next year.

Moderator: Thank you. The next question is from the line of Sakshi Pratap from Worth Investment & Trading Limited. Please go ahead.

J

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 6 of 7

Sakshi Pratap: I just have one question. So, the second Pune Hospital is in advanced stages of regulatory work. When can we tentatively start the construction process, if you could highlight it?

Ankit Thakker: So, we expect the approval Sakshi in the next quarter, which is Q4 of this financial year. So, towards the end of this financial year, beginning of the next financial year, sometime in that vicinity, we should begin construction.

Sakshi Pratap: Also, how many more can we add after adding 75 beds in the Indore hospital?

Ankit Thakker: So, we have a large land area, +5 acres of land in Indore. So, there is a lot of empty land available where we can build. We have one semi-finished building which can take 100 plus beds. So, what we have committed currently is that we can do about 200 beds. But it depends on the need and depends on the growth, we can take that up.

Moderator: Thank you. The next question is from the line of Anisha Shah, who is an individual investor. Please go ahead.

Anisha Shah: So, I just had two questions. First is, we added 22 beds at the Pune Hospital. So, post this addition, is there any further scope of addition or will it reach maximum capacity?

Ankit Thakker: So, we have 10 to 12 beds which we can add in Pune. After that we are at maximum capacity.

Anisha Shah: And second question was that Indore occupancy for H1 stood at 57% and ARPOB was Rs. 46,100. So, can you please share for quarter 2 as well?

Ankit Thakker: So, for quarter 2, we had 61% was the occupancy and Rs. 43,500 was the ARPOB.

Moderator: Thank you. The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah: My question is on EBITDA margins. So, we have delivered almost 23% revenue growth, but our margins are still flat. So, is it fair to assume that most of it is being dragged by our Indore expansion or is there something else also?

Ankit Thakker: Yes, so Indore expansion definitely is a factor. I think the margin for this quarter is what, 23.6%. The new beds in Pune would not have full capacity utilization currently and Indore would not be at peak occupancy currently, so those are two slight drags. But having said that, our aspiration for EBITDA margin as we have said is 25% give or take something so we are in that zip code and that is where we are operating.

Jainil Shah: So, when do we think we will reach the 25% mark because we will be adding more beds in 4Q in Indore and then probably next year or so we will also have Dombivli expenses coming in.

Jupiter Life Line Hospitals Limited
November 11, 2024

Page 7 of 7

Ankit Thakker: So, on an individual level, the properties may reach. On a group level, I hope we don't because that would mean that there is always a growth pipeline, and something is still in a phase of

maturity. So, we would always want to have something new coming up which is dragging us down. I like to be dragged down rather than be flat. So, yes, on a mature unit level, I think we are probably there in Thane and Pune and by the time Indore reaches there, Dombivli should create a little bit of drag and then Pune should create a little bit of drag and then something else. Yes, that is a process.

Jainil Shah: Sure, and just one more question, have we seen any attrition in Thane so far? And I mean do you expect something to go ahead there?

Ankit Thakker: No, we have not seen any attrition in any of our hospitals, unusual attrition let us put it that way and when I am talking about it, I am talking about senior level which means doctors and senior management. I am not talking about nurses because that is a perennial problem, I must admit. Apart from that on the senior level, we have not seen any attrition.

Moderator: Thank you. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Ankit Thakker: So, thank you everyone for joining us on this call. I hope the answers were satisfactory. If there is any further clarification needed, please feel free to call our IR team. Thank you and have a good day.

Moderator: On behalf of Mirae Asset Capital Markets, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Feb 2025

February 11, 2025

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P. J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980

Subject : Q3 & 9M FY 25 Earnings Conference Call – Transcript

Reference: Intimation of Earnings Conference Call dated February 04, 2025 and rescheduled dated February 05, 2025 and Audio Link of Analyst/ Investor Conference Call dated February 07, 2025

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q3 & 9M FY 25 Results Conference Call held on Friday, February 07, 2025 at 6:30 PM (IST) for the quarter and nine months ended on December 31, 2024.

The same will be available on the website of the Company at www.jupiterhospital.com .

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking you.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 9

“Jupiter Life Line Hospitals Limited Q3 and 9 M FY25
Earnings Conference Call”

February 07, 202 5

“E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchange s and the Company website on 7th February 2025 will prevail.”

MANAGEMENT : DR. ANKIT THAKKER – ED & CEO

MR. ANAND APTE - CHIEF OF BUSINESS AND STRATEGY
MS. FALGUNI SHAH - BUSINESS CONTROLLER
MS. SUMA UPPARATTI - COMPLIANCE OFFICER &
COMPANY SECRETARY
SGA – INVESTOR RELATIONS ADVISORS

Jupiter Life Line Hospitals Limited
February 07, 2025

Page 2 of 9

Moderator: Ladies and gentlemen, good day and welcome to the Jupiter Life Line Hospitals Limited Q3 and 9 Months FY25 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference, please signal the operator by pressing '*' and then '0' on your touch -tone phone. Please note that this conference is being recorded.

This conference may contain certain forward -looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call.

These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

I now hand the conference over to Dr. Ankit Thakker, ED and CEO of Jupiter Life Line Hospitals Limited. Thank you and over to you, sir.

Ankit Thakker: Thank you. Good evening to everybody. I thank you for joining us on our Earnings Call late Friday evening to discuss our 'Business and Financial Performance' for this Quarter and Nine Months of FY25. I hope you could look at our 'Financial Results' and 'Presentations', which have just been posted on the website and stock exchanges.

I am accompanied by Mr. Anand Apte – our Chief of Business and Strategy ; Ms. Falguni Shah – our Business Controller ; and Ms. Suma Up paratti – our Compliance Officer & Company Secretary.

We move to the 'Updates of this Quarter':

As a part of our IPO objectives, we had an aim to establish new hospitals to take the total group tally to about 2,500 beds in Western India. We have already spoken about our Dombivli and the second Pune hospital, which had given you visibility of 2,200 beds.

We are now pleased to announce the acquisition of an over 2-acre plot of land in Mira Road. This will mark our sixth hospital and the third in the MMR region. This land was procured from the open market through a local landlord financed entirely through internal accruals and equity reserves and has a cost of approximately Rs. 75 crores. Our plan is as always to establish a full-service multi -speciality quaternary care hospital, which will have a capacity of around 300 beds. The project's estimated CAPEX will be about Rs. 400 crores, which will include the land. The financing of the remaining CAPEX will be subject to the planning approvals, board approvals etc. which will be communicated to you from time to time. Mira Bhayandar region is a densely populated residential locality with a significant unmet demand for quality health services. Currently, the area lacks comprehensive high -end health facilities, compelling the residents to rely either on local secondary care centers or they need to travel to Jupiter Life Line Hospitals Limited
February 07, 2025

Page 3 of 9

Thane or Mumbai for advanced medical treatment. The new hospital we propose there will cater primarily to the micro markets of Dahisar, Mira Bhayandar and the Vasai Virar regions and will address a critical healthcare gap in that area.

The second hospital of Pune in Bibvewadi has now received the regulatory approvals and from next month onwards that is March of 25, we hope to start construction for that hospital as well. The first phase with a capacity of 200 odd beds should be operational in calendar year 28. The Indore hospital has recently added 78 new beds on 1st of January 25. This brings the total bed capacity in Indore to 309 beds. We have executed this one quarter before schedule and the CAPEX for this development was a little under Rs. 25 crores that translates to roughly Rs. 30 lakhs per bed for the new phase of construction. The Dombivli project is advancing as per plan and is expected to be operational around Q1 of FY27.

The 'Numbers for t

his Quarter':

Income for this quarter stood at Rs. 322 crores which is a 17.7% increase Yo Y. EBITDA Rs. 76.4 crores, which is a 21.5% increase Yo Y. The EBITDA margin this quarter is 23.7%. The PAT is Rs. 52.5 crores which is a 20.1% increase Yo Y. The PAT margin is 16.3%. For the nine-month consolidated numbers, the revenue was Rs. 934.8 crores, a 19.5% increase year -on-year. The EBITDA for nine months so far is Rs. 218.3 crores, again a 22% increase. The

EBITDA margin is 23.4 %. The PAT is Rs. 1 48.6 crores , an increase of 13.2 % year-on-year and the PAT margin so far is 15.9 %. The ARPOB for 9 month of this year has been Rs 59,100 as compared to Rs 53,600 in the same period last year . The ALOS for 9 months is now 3.88 days as compared to 3.92 days. The occupancy for these 9 months was 66.7% compared to 63.2% last year. The overall volume has increased from 6,51,500 to 7,27,500. The payer mix is insurance represents 55.4% of the revenue. 43.5% comes from self -payers and government schemes at 1.1%. So, these are the big highlights.

With this, I open the floor for Q&A. Thank you.

Moderator : Thank you very much. We will now begin the question and answer session. Anyone who wishes to ask a question may press '*' and '1' on their touchtone telephone. If you wish to withdraw yourself from the question queue, you may press '*' and '2' . Participants are requested to please use handsets while asking a question. Ladies and gentlemen, we will now wait for a moment while the question queue assembles. The first question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: Thank you for taking my question and congrats to the management on good numbers. So, the first question I have, Ankit you saw the margin side, what levers do we have considering like Pune is also occupied up to 60% to 65%, Thane is also more or less optimally occupied. So, what levers do we have to improve margins going forward?

Jupiter Life Line Hospitals Limited

February 07, 2025

Page 4 of 9

Ankit Thakker: Hi Amey, so I lost your voice a little bit , but from what I could gather you were asking for levers for margin expansion in all the three units, is that correct?

Amey Chalke: Yes.

Ankit Thakker: So, in Thane , there is just one lever that is an inflation -linked price hike. In Pune , besides inflation -linked price hike s, there is a second lever also for occupancy growth which you could see that in this quarter is around I think 65%. And in Indore , besides price hike and occupancy , the third lever is case mix optimization which I believe should last for another year, year and half. So, these are the three levers in different units.

Amey Chalke: So, our margin for Thane unit will be more than 25%, right?

Ankit Thakker: It should be something like that, yes.

Amey Chalke: Got it. So, should we assume that these margins are typically the optimal margins for the locality we are taking for doctors, etc.?

Ankit Thakker: Correct.

Amey Chalke: Sure. And the second question I have is, in Thane particularly considering there was only one Jupiter corporate driven hospital, now two more are coming up. At least they have announced their plan. So, in terms of talent etc ., you are comfortable, or do you think that inflation should be there on account of this competition on the doctors and the nurses' talent etc .?

Ankit Thakker: So, I think Thane with a population of close to 30 lakhs or 3 million now has enough number of doctor representation out of those 30 lakh people who live here , so the native medical talent pool in Thane is pretty high and even today , there are to be honest very good doctors in Thane who don't work with Jupiter right because there is only as many who can be affiliated to a 300-400 bed hospital. So, I don't think either from the demand side or from the talent availability side, there should be any challenge

either for us or for the newcomers.

Ankit Thakker: For the last couple of quarters, we have said that besides one, even if there are a few more in Thane, the demand is sufficient to neither bother us nor bother the new entrant. So, yes, that is, we continue with our view.

Amey Chalke: And the second hospital which is coming up is closer to the Ghodbundar Road or where is the exact location?

Ankit Thakker: Which one?

Amey Chalke: The second hospital, the land which we acquired, is it closer to Ghodbundar Road or is it ?

Ankit Thakker: The Jupiter's land you are saying?

Jupiter Life Line Hospitals Limited

February 07, 2025

Page 5 of 9

Amey Chalke: Yes.

Ankit Thakker: Yes, so you must have read Ghodbundar . So, Ghodbundar is not Ghodbundar Road. It is the name of the area in Mira Road, so it is in Mira Road. It is not on Ghodbundar Road, it is an area called Ghodbundar in Mira Road.

Amey Chalke: Sir, got it. Thank you, sir.

Moderator: Thank you. Participants, to ask a question, you may press '*' and '1' . We have the next question from the line of Pranav Chawla from Ambit. Please go ahead.

Pranav Chawla: Hi, sir. So, can you just briefly highlight what are the hospitals that are there in our pipeline

and what will be their commencement pipeline and in every phase how many beds are we planning to get operational?

Ankit Thakker: So, I will definitely walk through it but we have included one slide in the presentation for this if you want to have a clearer picture at any date. Currently, we have three hospitals; Thane, Pune and Indore. Thane capacity is 377 beds, Pune 386 and Indore is planned for 431. Currently operating 309, so it has a scope to add 122 more. The new hospitals that are coming up Dombivli is planned for 500 beds, Pune 2, in Bibvewadi is planned for 500 beds and Mira Road is planned for 300 beds, so if you add all of them up it goes to 2494 which is close to 2500 for all practical purposes. So, that is the pipeline.

Pranav Chawla: And which year do these hospitals come in?

Ankit Thakker: Dombivli Q1 FY26 -27, Pune sometime Calendar Year '28, Mira road sometime Calendar Year '29.

Pranav Chawla: Sir, and what would be our case mix? What would be our largest therapies, top 3 therapies?

Ankit Thakker: So, we are, what do you say, full service multi-specialty hospital where you don't have any focused therapy area in any of the hospitals and all the branches are well-diversified.

Pranav Chawla: Okay, got it. I will get back in the queue.

Ankit Thakker: Thank you, Pranav.

Moderator: Thank you. To ask a question, ladies and gentlemen, you may press '*' and '1'. The next question is from the line of Abdulkader Puranwala from ICI CI Securities. Please go ahead.

Abdulkader Puranwala: Hi, sir. Thank you for the opportunity. So, my first question is with regards to your expansion plans. So, while we look at your history, you have been adding hospitals say one hospital in each of say 2 to 3 years span, while going ahead, what we are talking about is having one new hospital almost every year. So, what I would like to know is basically what could be the P&L
Jupiter Life Line Hospitals Limited
February 07, 2025

Page 6 of 9

impact of all the three hospitals coming in? And at the same time with the kind of CAPEX what you have lined up, which is nearly say Rs. 1400 crores for these three hospitals, how do you plan to fund the entire CAPEX?

Ankit Thakker: Thank you, Abdul. So, for the CAPEX question, currently we are sitting on something like Rs. 250 plus odd crores of cash on our books. And I told you that the nine month EBITDA this year is Rs. 218 crores with whatever simple extrapolations you want to do over a 5-year period, you will see that the current cash and the internal accruals which we will generate over a 5-year period will add up to the required CAPEX number. So, as far as we are doi

ng these

three hospitals because they are also greenfield which means the money has to be invested in a staggered way, not upfront. We will be able to deploy cash as we generate it through the running hospitals. So, we will be able to maintain the debt free status and fund these three hospitals. At future date if there is something interesting in terms of opportunity, then the board will evaluate it from time to time whether we need to fund more opportunity through debt equity or whichever way. So, that is on the CAPEX question. The P&L question is that I think industry average currently is that most new greenfield hospitals in the country are breaking even maybe sometime in the second year with small EBITDA losses or some EBITDA losses in the first year. So, from the current EBITDA numbers of the three hospitals, you can project some blips in the first year of operation of each hospital and stabilizing in the second year and becoming positive in the third year. So, with that, I think you will be able to build some kind of model.

Abdulkader Puranwala: Got it. That is well understood. Second question is with regards to your Indore. So, now that, you have been able to add beds quarter ahead. When should we expect the balance of 111 beds to be added at this facility?

Ankit Thakker: So, we have just added 78. We might add one more ICU about 10-11 beds soon. But after that, there is no immediate plan to add more beds. So, we were around 60% odd percent occupancy before these beds got added. With the addition of these beds, we would have slipped into 50% kind of occupancy. Till we don't come back to 60%-65%, we will not be discussing new bed addition. Once we reach that 60%-65% occupancy, we'll think about new beds.

Abdulkader Puranwala: Got it, sir. And just the final one, if I may. So, with regards to your Dombivli, now that it will get commercialized in Q1 of fiscal 27. So, by what time would the doctor hiring and other nursing staff will start?

Ankit Thakker: So, I think one quarter before, 3 to 6 months before you start talking and in the quarter before you start, you should start getting people on board. So, that is a typical timeline for all new Greenfield.

Abdulkader Puranwala: Thank you and wish you all the best.

Jupiter Life Line Hospitals Limited
February 07, 2025

Page 7 of 9

Moderator: Thank you. Participants who wish to ask questions, may please press '*' and '1'. We have the next question from the line of Anjana Shah from Sha h Investments. Please go ahead.

Anjana Shah: Thank you for this opportunity, sir. So, sir, I wanted to understand that, a competitor's planning to enter the Thane market as our neighbor. So, how do you perceive this competition and what strategies do we maintain to have our market position intact?

Ankit Thakker: Hi, Anjana. So, as I answered in the earlier question that I don't think there's any great strategy to be honest. I think the inherent demand itself is quite high in this region. And as the time passes, there is a definite replacement of unorganized and secondary care nursing home beds with organized and corporate multi-specialty hospital beds. So, a combination of the replacement of some beds and the unmet demand will mean that neither us nor them will have to have any extraordinary strategies. It is just you are here to serve the needs of the community and the community has a lot of unmet need. So, yes, I think it is much simpler than you think.

Anjana Shah: So, also we are working towards setting up three new hospitals. So, with respect to funding, how do we plan to finance these? Would we bring on some debt on the book? Would we raise further equity? Or would it be like a mix of both?

Ankit Thakker: So, as I just answered Abdul, for these three hospitals, we will not need more debt. Internal accruals and cash reserves will be sufficient to fund them. If we do take on additional

projects,

then we may need debt equity or a combination of it. But when those opportunities arise, we will evaluate them at that point.

Anjana Shah: Sure, perfect. Thank you. Thank you so much, sir. That was helpful.

Moderator: Thank you. Ladies and gentlemen, you may press '*' and '1' to ask a question. The next question is from the line of Anya Desai from Brig ht Securities. Please go ahead.

Anya Desai: Hi, sir. Thank you for the opportunity. I just had one question. Can you please share the ARPOB and occupancy rate for Q3 FY25?

Ankit Thakker: So, we have 65.7% of occupancy. The ARPOB is Rs 61,750. And what was the third question? Just these two?

Anya Desai: No, that's it. I just wanted to know for Q3 FY25.

Ankit Thakker: 65.7% and Rs 61,700.

Anya Desai: Alright. Thank you so much.

Moderator: Thank you. Ladies and gentlemen, to ask a question, you may press '*' and '1' on your touch-tone telephones. We have the next question from the line of Jigar Shah from Elevate Research.

Jupiter Life Line Hospitals Limited

February 07, 2025

Page 8 of 9

Please go ahead. Jigar Shah, your line has been unmuted. You may proceed with your question.

Jigar Shah: Good evening, sir. Sir, I have a couple of questions. In nine months, FY25, what was our total CAPEX excluding the recent land acquisition?

Ankit Thakker: Nine months FY25 what was our total CAPEX excluding the new land you are saying?

Ankit Thakker: Should be around Rs. 100 odd crores I think towards Dombiv li and Indore I think. I don't have the exact number on me now but it should be around Rs. 100 crores.

Jigar Shah: Okay, that helps. Secondly with the Dombiv li Hospital yet to commence operations in F Y25, what level of EBITDA loss do we anticipate during the initial couple of years?

Ankit Thakker: So, we don't have formal guidance on EBITDA loss currently, but we do think that the first year should be EBITDA negative and the second year should be EBITDA breakeven. That is what we have for now.

Moderator: Thank you. Participants are requested to please press '*' and '1' to join the question queue. We have the next question from the line of Karan Mehra from Me hta Investments. Please go ahead. Karan, your line has been unmuted.

Karan Mehra: Yes. Good evening, sir. Thank you for the opportunity. Sir, most of my questions are answered, so I only have one question. If you can provide some insights into the demographics of the Mira Bhayandar locality and how it influences our hospital's growth potential, if you can throw some light here, it would be helpful.

Ankit Thakker: Sure, so Mira Bhayandar, I don't know, are you from Mumbai?

Karan Mehra: Yes, I am.

Ankit Thakker: So, you would understand some geography if I talk about. So, you are familiar with Thane, I guess. So, just like Thane is to the eastern suburbs of Mumbai or the central line of Mumbai, Mira Bhayandar is to the western part of Mumbai. It is a dense residential neighborhood which essentially houses people who go to work in western Mumbai. It is not a commercial hub, it is mainly a residential hub. It is a very dense community and does not have healthcare facilities of great stature today. Mira Bhayandar, as a municipal corporation should have, I think a population of somewhere in the zip code of 15 lakhs, then it is surrounded by two other dense areas. One is Dahisar on the south and the corporation of Vasai Virar, Nalasopara, Naigaon on

the North. So, with all of this together with in say 30 to 60-minute driving distance, you would again have a population of 2 -3 million people who currently are essentially Mumbaiites in their mind , but don't have the facilities of healthcare in the place they live. So, this is the problem that we are trying to solve.

Jupiter Life Line Hospitals Limited

February 07, 2025

Page 9 of 9

Karan Mehra: Understood. Sir, thank you for sharing this idea. Thank you a

nd all the very best.

Moderator: Thank you. Participants who wish to ask questions may please press '*' and '1' at this time.

We have no further questions, ladies and gentlemen. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Ankit Thakker: So, thank you, everyone. I hope the questions were answered satisfactorily. If there are any more questions, you are most welcome to reach out to us or the SGA team, our IR Advisors. I wish all of you a very happy weekend ahead. Thank you.

Moderator: Thank you. On behalf of Jupiter Life Line Hospitals Limited, that concludes this conference. Thank you all for joining us. You may now disconnect your lines.

Call: May 2025

May 15 , 2025

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P. J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980

Subject : Q4 & FY 25 Earnings Conference Call – Transcript

Reference: Intimation of Earnings Conference Call dated May 05, 2025 and Audio Link of Analyst/ Investor Conference Call dated May 12 , 2025

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q4 & FY 25 Results Conference Call held on Monday , May 12 , 2025 at 10.00 A M (IST) for the quarter and Financial year ended on March 31, 2025.

The same will be available on the website of the Company at www.jupiterhospital.com .

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking you.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 10

"Jupiter Life Line Hospitals Limited
Q4 and FY '25 Earnings Conference Call"
May 12, 2025

"E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges and the Company website on 12th May 2025 will prevail."

MANAGEMENT : DR. ANKIT THAKKER – EXECUTIVE DIRECTOR AND
CHIEF EXECUTIVE OFFICER
MR. ANAND APTE – CHIEF OF BUSINESS AND
STRATEGY
MR. NITIN PATODI – HEAD OF FINANCE
MRS. SUMA UPPARATTI – COMPANY SECRETARY AND
COMPLIANCE OFFICER
SGA - INVESTOR RELATIONS ADVISORS

Moderator: Ladies and gentlemen, good day, and welcome to Jupiter Life Line Hospitals Limited Q4 and FY '25 Earnings Conference Call. As a reminder, all participant lines will be in listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not a guarantee of future performance and involve risks and uncertainties that are difficult to predict.

I now hand the conference over to Dr. Ankit Thakker, ED and CEO of Jupiter Life Line Hospitals Limited. Thank you, and over to you, sir.

Ankit Thakker: Thank you. Good morning, everybody. Thank you for joining us on our earnings call to discuss the business and financial performance of the last quarter and FY '25. I hope you all had a chance to view the financial results and investor presentation that were uploaded on the website and the stock exchanges.

I'm accompanied by Mr. Anand Apte, our Chief of Business and Strategy; Mr. Nitin Patodi, our Head of Finance; and Ms. Suma Upparatti, our Company Secretary and Compliance Officer. I'm also joined on the call with our Investor Relations Advisors, SGA.

So we have consistently communicated that our post-listing initial objective was to establish 2,500 beds and expand our networks from 3 to 6 hospitals. We believe that we could achieve this milestone by leveraging our existing cash reserves and internal accruals. I'm pleased to report that this first target of 2,500 beds in the Western India region is now within sight.

Over the next 4, 5 years, as these hospitals become operational, we are confident that the operating cash flows generated should be sufficient to fund this capex and their ongoing operations. However, while 6 hospitals represented our initial target, they are certainly not the final destination.

We are exploring some opportunities in addition to the above 6, which could be greenfield or in the form of an acquisition. With this in mind, we have raised additional debt to fund these new projects. The Board has carefully considered this and is comfortable with a prudent and responsible level of leverage to optimize our growth in both scale and speed. The broad contours of this debt structure are detailed in the investor presentation, but I will reiterate some key aspects.

Let me begin with our subsidiary, JHPPL, which operates our Indore hospital. Following our listing and primary equity raise, we had extended a loan to our Indore subsidiary to facilitate the repayment of their bank obligations and also to fund some of their capex initiatives. We have now raised a ₹250 crores debt within our Indore subsidiary to repay our intercompany loan.

Jupiter Life Line Hospitals Limited
May 12, 2025

Importantly, this Indore subsidiary now possesses sufficient cash flows to service this debt and the debt repayment benefits the listed entity directly because this amount represents equity in our books. Consequently, this money becomes available to pursue either a land purchase or a strategic acquisition, depending on the opportunities that may materialize.

Turning to the listed entity. We have secured a sanctioned amount of ₹350 crores loan, which is earmarked for capex. Of this sanctioned amount, we have drawn ₹75 crores. We have the option to utilize the remaining ₹275 crores from this debt facility to fund our capex plans, which could be anywhere, Dombivli, Pune or something else.

This strategic move means that our surplus internal accruals,

which are also reflected as equity,

will be available for future expansion initiatives. The cost of carrying this loan is expected to be less than 1%. We have proactively availed this loan so that we are well positioned to act swiftly should a compelling opportunity present itself without the need for last-minute fundraising.

As you see on our balance sheet today, our cash position stands at about ₹600 crores, comprising of ₹325 crores from the new debt and ₹275 crores from our existing cash reserves. In this financial year, we have also undertaken several significant brownfield capex initiatives.

Let me provide you an update on some of these, starting with our Indore facility. We executed a substantial expansion project, adding 2 additional floors to the existing hospital building, resulting in 78 new beds. These beds were commissioned on 1st January 2025. Furthermore, we have constructed an 11-bed ICU, which is now ready for commissioning, but will be

operationalized based on occupancy demand.

We have also pursued multiple expansion projects in our Thane facility this year. To begin with, we have received approval from the Municipal Corporation to develop the building in accordance with the new UDCPR regulations. This could potentially entitle us to increase FSI, allowing us to add an additional floor to the Thane Hospital.

However, this is contingent upon obtaining environmental clearances and some other necessary statutory permissions. We will keep you updated about the potential size and scope of future expansions at the Thane facility as and when these approvals are received.

In the interim, we have undertaken some capex activities to cater to the current clinical needs.

We decommissioned our 22 -bed economy ward and strategically reconfigured certain areas to create 2 new operating theaters, a second MRI machine, a second cardiac cath lab, and expansion of our Day care and chemo suites from 14 to 22 beds and a new OPD cluster.

We have also undertaken a comprehensive overhaul of some of our ageing engineering infrastructure, including chillers and cooling towers. The Dombivli Hospital construction is progressing as per plan with an expected commissioning within a year in Q1 of FY '27. The new Pune hospital is at the excavation stage after receiving all the environmental clearances. The construction is expected to begin post monsoon.

Jupiter Life Line Hospitals Limited

May 12, 2025

Page 4 of 10

In Mira Road, the land has been acquired, and we are at the post -purchase regulatory stage of 7/12 updation, etc., and we will start working on the architectural drawings for that project shortly.

Coming to the numbers, Q4 of this year versus last year. The total income this quarter stood at INR326.7 crores, an increase of 12.1% year -on-year. The EBITDA was INR78.3 crores for this quarter. The EBITDA margin was 23.9%. The PAT stood at INR44.9 crores in this quarter, a decrease of 0.9% year -on-year on account of increased depreciation in this quarter. The PAT margin stood at 13.7%.

For the whole year, the income is INR1,261.5 crores, an increase of 17.5%. The EBITDA for the year is INR296.6 crores, an increase of 22.5% year -on-year, and the EBITDA margin is 23.5% for the financial year '25. The PAT for the year is INR193.5 crores, representing a margin of 15.3%. The ARPOB has increased by 10.4% to INR60,600 in this year. The ALOS is at 3.89 days. The average occupancy for the year is 65.3%, an improvement of 150 points year -on-year. The breakup is 72.1% occupancy for Thane, 65.5% occupancy for Pune and 54.9% occupancy for the Indore hospital. The overall volume has increased 11.2% to 9.8 lakhs in the financial year. The payer mix split, insurance stood at 55.8%, self -payers 42.9% and government schemes at 1.3%.

So with this, I open the floor for questions.

Moderator: Thank you very much. We will now begin the question and answer

session. The first question is

from the line of Amey from JM Financial.

Amey: Congrats to Ankit and the management for the good set of numbers. Ankit, first question I have, although you mentioned some details at the back end of your speech regarding ARPOB. But for the quarter, is it possible to give ARPOB and occupancy how it has been and how it has performed year -on-year?

Ankit Thakker: You want the ARPOB for the quarter?

Amey: Yes.

Ankit Thakker: So ARPOB for the quarter is INR65,453.

Amey: Okay. And the year -on-year growth is around 10%

Ankit Thakker: Yes, around 10%, I think.

Amey: Yes. So do you expect this 10% year -on-year ARPOB growth to continue? And if yes, then what would be the levers for next year?

Ankit Thakker: No, I don't expect 10% to continue. As I was saying that the projection for ARPOB growth should be inflation -linked. Thane and Pune, both are matured hospitals. So besides inflation,

Jupiter Life Line Hospitals Limited

May 12, 2025

Page 5 of 10

there is no other lever for ARPOB growth. Indore may be slightly higher than inflation because for maybe one more year, it has a second lever that is of case mix optimization that is more tertiary work. So Indore may have an extra lever for maybe another year or so. But apart from that, I think if you want to extrapolate ARPOB, it is the safest to extrapolate it in line with inflation.

Amey: Sure. Then the volume growth has been around 10% as well for us on the IPD, OPD side. So that has resulted into something like 17%, 18% year -on-year growth on the revenue side. So how does that will shape up next year considering the ARPOB is going to fall a bit and the volumes for the existing hospital might not remain at around 10% level, right?

Ankit Thakker: So ARPOB is not going to fall. It will rise in line with inflation. The volume in Thane should be stable. In Pune and Indore, we could expect some growth in volumes .

Amey: Sure. The second question I have on the Thane, you said that you will get permission to add one more floor in Thane. So the current restructuring you have done where you have removed the economy ward, so is it keeping that thing in mind

Ankit Thakker: Yes. So now what is happening is that as you are seeing across the board, the length of stay for specific procedures and diseases is going down year -on-year with advancement of technology and medical science. So the supporting infrastructure is becoming more important and necessary. So we thought that we were in need of more operating rooms. We were falling short of our cath lab space.

So we added one more there. We were also struggling on the imaging side with MRs, so we added one there. Doctors' offices was becoming a challenge, which necessitated a need for another OPD cluster. So these supporting services were becoming more necessary because just adding beds is not going to solve all the problems.

Amey: Right. And once this added floor, whenever it gets completed, it's functional, do you expect there is a little scope to improve your margins on Thane Hospital because the incremental cost would be lower, right, for the incremental beds?

Ankit Thakker: Yes, ideally, if and when this floor does come up, we definitely think that it should contribute on a unit level a little more to the EBITDA margins because most of the fixed costs are taken care of and the added capacity in terms of manpower, power, etc., does not need as much fixed cost now. So yes, we think it should contribute well to the bottom line.

Moderator: The next question is from the line of Moksh Ranka from Aurum Capital.

Moksh Ranka: We are doing a greenfield capex in Pune, right? Could you please provide some color on the capex amount which will be investing in it? And it's actually we are doing a greenfield capex...

Ankit Thakker: Greenfield capex, yes, in Pune.

Jupiter Life Line Hospitals Limited

May 12, 2025

Page

6 of 10

Moksh Ranka: Yes. And could you give me some details about the amount you would be investing in it and the time line for this? And it's actually a very good location, I would say, because I just live nearby, so I would just like your comments?

Ankit Thakker: Thank you, Moksh. So the new Pune hospital is supposed to be 500 beds in size. We think that the capex incurred for that project should be somewhere in the tune of INR500 crores excluding land.

Moksh Ranka: And for the land, we have got some favorable terms. Is my understanding correct?

Ankit Thakker: So the contours of the land deal are that currently, it is on a lease for 10 years without any escalation for the whole tenure. And after the third year is completed, we have a right to purchase the land. So those are the terms of the land.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Sir, firstly, on Thane, you spoke about an addition of new floor. So sir, any time line you would like to provide on the quantum of bed you will add at this facility?

Ankit Thakker: Abdul, so no, I don't have a time line, honestly, at this stage. It will depend on the regulatory framework. Currently, the environmental committee for the state and for this region is not very active. So once those committees start getting active and Thane starts coming back on the fold of the permission stage, we will move the applications. And then whenever those applications come through, we will keep you updated. So today, I don't have an exact time line to share with you.

Moderator: Ladies and gentlemen, we seem to have lost the participant. The next question is from the line of Bino Pathiparampil from Elara Capital.

Bino Pathiparampil: Ankit, just in your speech, you had mentioned about the occupancy rates separately for each hospitals. One, could you please repeat that once? And was that for Q4 or the full year? And if it is full year, do you mind giving the Q4 numbers because you used to give it in the presentation earlier?

Ankit Thakker: So this is for the whole year, Thane 72.1%, Pune 65.5% and Indore 54.9%. And for the quarter, we have Thane 71.1%, Pune 61.1% and Indore 42.1%.

Bino Pathiparampil: Okay. Got it. And any reason why it's lower in Indore for the quarter?

Ankit Thakker: It's an expanded base. So we added 78 more beds. So in percentage terms, there is a dilution effect.

Moderator: The next question is from the line of Prisha Rathie from NMK Securities.

Prisha Rathi: So I have only one question. Could you please provide an update on the restructuring or the rationale behind Medulla Healthcare scheme?

Jupiter Life Line Hospitals Limited

May 12, 2025

Page 7 of 10

Ankit Thakker: So Medulla is a wholly owned subsidiary. It was activated during the COVID period where we were trying to create a separate entity to build and run our IT services. It was at that time, IT engineers, etc., were in a different mode. And they were not very keen to work with hospitals and some of them who were maintaining our unit at that time were wanting to commercialize health care software.

So with that view, they wanted to spin it off and create a new entity. This activity lasted for probably a year or so, I'm not sure, but we never really ended up going down that route of commercialization. We have some team restructuring and realignment of thoughts, and we said that, that is not the road we want to go down under.

So now this Medulla is not a functional entity. We are just carrying it on the books. So we thought that we should merge it and stop carrying one more entity. So that is the reason Medulla is being merged.

Moderator: The next question is from the line of Abdulkader Puranwala from ICICI Securities.

Abdulkader Puranwala: Yes. So sir, I mean, when we talk about going inorganically, are we trying to look for certain brownfield expansion? An

d in terms of the kind of asset would we look at, would it be the same

500-bed kind of a facility setup you have been adding to your network so far?

Ankit Thakker: Yes. So there is no change in thought process on any front. A, to begin with, Western India remains the focus. B, full service flagship hub model continues to remain the focus. C, greenfield continues to remain a preferred choice. D, on the acquisition and brownfield front, like we have always said, if a good opportunity at a good value does present itself, we are not averse to it, but we will insist that the quality of asset and the value at which we are able to acquire it seems justifiable in our mind. If that does happen, we will look at an acquisition.

Abdulkader Puranwala: But sir, any target multiple in terms of the acquisition value or the kind of amount you would be ready to spend? Any color on that would be also very helpful.

Ankit Thakker: No, there is no target as such. It's dynamic, right? I mean are willing to spend different things for -- I mean, different value for different things. So I don't want to close my mind and just give you some number. The only thing I want to tell you as long as it makes sense in our view, and we will do it, we will not do it just in a mindless race to add numbers and add speed. If it makes sense, we will do it.

Moderator: The next question is from the line of Himanshu Binani from Anand Rathi.

Himanshu Binani: So sir, if you can help me with the ARPOB numbers for Q4 for Thane, Pune and Indore?

Ankit Thakker: So ARPOB number for Q4 consolidated is INR65,453. We are not giving specific ARPOBs.

Himanshu Binani: So unit-wise, ARPOB you have not given.

Jupiter Life Line Hospitals Limited

May 12, 2025

Page 8 of 10

Ankit Thakker: No

Himanshu Binani: Got it, sir. And sir, secondly, can you please help me explain the Second Phase of this Indore expansion of 111 beds, by when this would be like coming?

Ankit Thakker: See, like I just answered some time back, if you look at the occupancy for this quarter in Indore is 40%, 45%, right? Once this number reaches 60%, 65%, we will start thinking about expansion.

Moderator: The next question is from the line of Aashita Jain from Nuvama Institutional Equities.

Aashita Jain: I just only have one question on Thane Hospital. So your competitors start its IT services soon and even Max has announced the new Thane hospital. So have you seen any impact on your senior doctors in this facility so far? And also with Thane attracting more hospitals in future, how do we plan to minimize such impact in the future?

Ankit Thakker: Aashita, so yes, so far, not a single one of our senior doctors has moved out and stopped association with us. So we continue to report 0 attrition at the senior consultant level. Going forward also, I don't perceive that to be a major risk factor. As I have said, we have fortunately developed strong and sticky teams in all the locations where we are operating. So attrition at the doctor side has not been a big problem.

However, with answering your question completely with new hospitals coming in, the supply of doctors in Thane is also extremely high as is the population of Thane, which is extremely high and growing at 3 million people. So I don't think that even if there is zero attrition from our side,

the new hospitals will struggle in building teams. They will be able to build teams also. And with the population, I think on the demand side, there should not be challenge for any of the 3 hospitals.

Aashita Jain: It's helpful. And sir, if possible, can you provide ARPOBs on an annualized basis for the 3 units? I know you're not sharing for the quarter, but for annual basis...

Ankit Thakker: No, unit-wise, we are not sharing. Total, if you want me to repeat, I can, INR60,600 is annualized and INR65,000 is for the quarter.

Moderator: The next question is from the line of

Rohit Mehra from SK Securities.

Rohit Mehra: My first question is regarding our Thane hospital that with the restructuring of the hospital in place, what is the expected increase in revenue per occupied bed?

Ankit Thakker: Yes, Rohit, so I have not done that math, and I have not done this restructuring purely from the lens of ARPOBs in revenue. It is predominantly from the occupancy and clinical needs. So it should have theoretically some impact. What impact it will have, maybe we'll see over the next couple of quarters. But I have not done the math. I don't have an answer to that question, unfortunately.

Jupiter Life Line Hospitals Limited
May 12, 2025

Page 9 of 10

Rohit Mehra: Okay. No problem. Okay. My second question is considering our cash balance of approximately INR600 crores, are we planning any acquisition or any such opportunities under consideration?

Ankit Thakker: Yes. So as I said, we are open to both greenfield opportunities and an acquisition as long as the broad criteria that we have is met, Western India flagship and a logical capex plan.

Moderator: The next question is from the line of Harshal Patil from Mirae Asset Markets.

Harshal Patil: Just most of my questions are answered. Just one thing I wanted to know, you've been quite alluding to even pursuing inorganic opportunities and targets for expansion. And at the same time, we've been quite focused on the western part of India as of now.

So any thoughts just purely qualitative comments from your side, maybe you would be looking to go beyond the western part of India or is there any targets that you are kind of looking at evaluating or something? So any qualitative comments just from an understanding perspective?

Ankit Thakker: Yes, Harshal, currently, there is absolute clarity that we will not go beyond western part of India for now. And on the acquisition side, again, it is just to complete the whole story that greenfield remains the first choice. But if there is a sensible acquisition opportunity, we are not averse to it. So that is about it.

Moderator: The next question is from the line of Ashok Shah from Eklavya Invesco Family Office.

Ashok Shah: Sir, can you just inform or say about does this occupancy level can be increased at the Thane Hospital still by 10% to 15% or 20%?

Ankit Thakker: So Thane hospital occupancies are 72% currently. Anything in mid -70% is, in our view, peak occupancy because we measure it at midnight. So there isn't too much scope to increase occupancy in Thane at current levels.

Ashok Shah: Okay. So are there any plan to open a tertiary center near Pune area or near Thane area towards central suburb, towards Dadar area till, so we can get more and more elite patient for our hospital?

Ankit Thakker: Yes. So as I said, towards Dadar areas is great. Any health care there, the challenge remains land availability and cost of land. So if something sensible presents itself, we are open to doing things in Mumbai. And near Thane is Dombivli where we are doing a hospital. We are also doing a second hospital in Pune. So I hope that answers your question.

Ashok Shah: Okay. So any tertiary plan or something like, only OPD opening and then all the other requirement can be done at Thane Hospital, is it possible or not feasible?

Ankit Thakker: So only OPD, we don't do. We are into full -service tertiary care only. So wherever we are present, it is tertiary care. And that is the model that we are following. We are not doing only OPD kind of satellite base expansion .

Jupiter Life Line Hospitals Limited
May 12, 2025

Page 10 of 10

Ashok Shah: So my question was, since the availability of land is very difficult, so that's why this can be a second view or something similar thing happens in the eye care hospital?

Ankit Thakker: Certainly, that can be one view and a lot of people have very successfully done hub and spoke and things like that. A

nd like you alluded, single specialty eye care has done it very successfully. Our model is more hub focused. So it does not suit our model.

Ashok Shah: Sir, second point is we are doing a major expansion at 2, 3 places over next 2 to 3 years. Sir, with the expansion at full capacity, what could be our total turnover after 3 years?

Ankit Thakker: I wouldn't want to get into financial specific guidance after 3 years. But on the occupancy front, I can give you some color to understand that next financial year, Dombivli should get inaugurated and that should add about 250 -odd beds to our capacity in Phase 1, which can be ramped up over the next couple of years as the capacity expands and then Pune a couple of years after that. So within 3 years, we should have 2 more functional hospitals, which should add to available bed capacity, and it should start contributing on the financial front as well.

Ashok Shah: So Dombivli will be 250 and how much will be Pune beds?

Ankit Thakker: So both are 500 beds. Phase 1 will be around 250, and then we will ramp it up as per need.

Ashok Shah: So in First Phase, total 500 beds will be opening. And Second Phase, we can have a look at the next 500 beds?

Ankit Thakker: Correct.

Moderator: Thank you. As there are no further questions from the participants, I now hand the conference over to the management for closing comments.

Ankit Thakker: So thank you, everyone, for joining on Monday morning. I hope the questions were answered satisfactorily. However, if there are any further questions or you need any more clarifications, please reach out to us through SGA, and we'll be happy to take them up. Thank you and best wishes.

Moderator: Thank you. On behalf of Jupiter Life Line Hospitals Limited, that concludes this conference. Thank you for joining us, and you may now disconnect your lines.

Call: Aug 2025

August 06 , 2025

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P. J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980

Subject : Q1 FY26 Earnings Conference Call – Transcript

Reference: Intimation of Earnings Conference Call dated July 25 , 2025 and Audio Link of Analyst/ Investor Conference Call dated August 4 , 2025 .

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q 1 FY26 Results Conference Call held on Monday , August 4 , 2025 at 11.00 A M (IST) for the quarter ended on June 30, 2025.

The same will be available on the website of the Company at www.jupiterhospital.com .

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking you.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 8

"Jupiter Life Line Hospitals Limited
Q1FY26 Earnings Conference Call"
August 04, 2025

"E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges and the Company website on 4th August 2025 will prevail."

MANAGEMENT : DR. ANKIT THAKKER – JOINT MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER
MR. ANAND APTE – CHIEF OF BUSINESS AND
STRATEGY
MR. SIVASIS SEN – CHIEF FINANCIAL OFFICER
MR. NITIN PATODI – HEAD OF FINANCE
MRS. SUMA UPPARATTI – COMPANY SECRETARY AND
COMPLIANCE OFFICER
SGA - INVESTOR RELATIONS ADVISORS

Jupiter Life Line Hospitals Limited
August 04, 2025

Moderator: Ladies and gentlemen, good day, and welcome to the Jupiter Life Line Hospitals Q1 FY '26 Earnings Conference Call. A brief disclaimer. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participate lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star then zero on a touchtone phone. Please note that this call is being recorded.

With this, I now hand the conference over to Dr. Ankit Thakker, Joint Managing Director and CEO. Thank you, and over to you, sir.

Ankit Thakker: Good morning, everyone. Thank you for joining us on our earnings call to discuss the business and financial performance of Q1 FY '26. I hope you all had a chance to view our financial results and investor presentation, which were uploaded on the company website and the stock exchanges. I'm accompanied by our Chief of Business and Strategy, Mr. Anand Apte; our Chief Financial Officer, Mr. Sivasis Sen; Head of Finance, Mr. Nitin Patodi; our Company Secretary and Compliance Officer, Mrs. Suma Upparatti; and SGA, our IR Advisors on this call.

It's been a rather uneventful quarter with no significant updates to give this time. Progress of all 3 greenfield units, that is Dombivli, the second Pune Hospital and Mira Road are as per the previous updates. In this quarter, we have commissioned a new solar power plant of 1.2 megawatts in Madhya Pradesh. This adds to our existing wind energy capacity, taking our total renewable energy installed base to 6.8 megawatts. These investments will not only offset our carbon footprint, but will also help us in reduction of power costs for the company over the life cycle.

Before beginning with the financial results for the quarter, I wanted to inform everyone about the increased depreciation and finance costs that we have reported in Q1 of this year compared to last year. The same trend is expected to continue for the rest of the financial year as well. The capex incurred in the last year, including the 100 new Census Beds, the new OTs, biomedical enhancements that is the surgical robots, MRIs, cath labs, et cetera, have increased the depreciation load by over INR10 crores this quarter compared to the same period last year. The new debt has also contributed to some higher finance costs that we have reported in our P&L. Therefore, for the entire financial year, while the EBITDA margins are likely to be preserved, but the gap between PAT and EBITDA is expected to widen, leading to lower PAT margins this year. The total income stood at INR347.6 crores in Q1 of this year, that is a 20.5% increase year-on-year. This includes the INR6.6 crores consolidated net income from the Jupiter Pharmacy as well.

Jupiter Life Line Hospitals Limited
August 04, 2025

The EBITDA stood at INR78.1 crores this quarter, increased by 19.6% year-on-year. The margin represented is 22.5%. The PAT was 43.9% in this quarter, a decrease of 1.6% year-on-year. The PAT margin is 12.6%. The ARPOB was INR67,300 in this quarter and the ALOS was 3.78 days. The average occupancy rate was 60.1% in Q1FY26 compared to 63.9% last year. This is a dilution in relative terms because of the increase in Census Beds last year. In absolute terms, the occupancy is higher by about 5% compared to last year. The overall patient volume also has increased by 11.7% to 2.6 -odd lakhs in the first quarter of this year. The payor mix is 56.3% for insurance, 4

2.3% for self-payers and government schemes around 1.4%. So this is the update from me, and I'm happy to take questions. Thank you.

Moderator: Thank you very much. The first question comes from the line of Nancy Yadav from Allegro. Please go ahead.

Nancy Yadav: I just want to confirm net debt number?

Ankit Thakker: Net debt is still 0. The total debt is INR325 crores, and the cash position is something in the zip code of INR600 crores. So, we are net cash of INR275 crores.

Nancy Yadav: All right. Understood. And sir, also, could you give some background about the locations in which you're setting up the new hospitals? Like if you could explain a little bit more about Bibvewadi and the logic behind that? Because in Bibvewadi, we also have Sahyadri. So how does the competitive landscape look? And what's the rationale behind the geographies?

Ankit Thakker: So we are doing 3 greenfield hospitals, 2 in the Mumbai metropolitan region, that is Dombivli and Mira-Bhayandar and the third in Bibvewadi, as you said. All 3 in our view are very dense residential locations, and they are underserved in terms of quality and tertiary health care on the ground, so Dombivli in that 30, 40-minute driving distance of Dombivli, Kalyan, Ulhasnagar,

Badlapur, Ambernath has about 2 million people .

And it does not have quality healthcare today. Similarly, Mira -Bhayandar, Dahisar, Vasai -Virar don't have quality healthcare today. And Bibvewadi , Ganga Dham going up to that entire South Pune belt, we believe, is also underserved and densely populated. So that is the thesis for investment.

Moderator: The next question comes from the line of Amey Chalke from JM Financial.

Amey Chalke: Congrats on a good set of numbers. I have first question on the hospital -wise performance, if it's possible to elaborate a bit on, let's say, Thane and Pune Hospital, the occupancy, revenue growth, et cetera, that would be helpful, during the quarter.

Ankit Thakker: Yes. So on the occupancy side, I think as I have said last time that Thane already has near mature occupancy in the mid -70%, so it is continuing in that zip code. Pune and Indore both -- which had increased capacity that we added last year, both of those places have, in absolute terms, started utilizing some of those increased capacities. And in the absolute occupancy of both Pune

Jupiter Life Line Hospitals Limited

August 04, 2025

Page 4 of 8

and Indore has increased compared to last year. The total occupancy, as I said, in this quarter was 60%, which is in absolute terms 5 -odd -- 5.4-odd percent higher than last year.

Amey Chalke: So is it fair to say the large part of volume growth is led by Pune and Indore for the quarter?

Ankit Thakker: Yes. Yes. That is fair, absolutely fair.

Amey Chalke: Okay. And in terms of ARPOB growth, it also looks very healthy, around more than 10%. Is it possible to like to give some factors to what is driving this ARPOB growth at present? Is this totally led by the case mix or there isn't any factor of price hikes, et cetera ?

Ankit Thakker: So as I said, price hike is a factor in all the 3 locations. That is inflation -linked price hike that will be a factor for all 3 locations. For Indore, especially, there will be a factor of case mix optimization as well because it is still not a matured hospital. So, it is a little bit of combination of case mix and price hike both, which are giving rise to ARPOB. So, everything higher than inflation that you see is case mix.

Amey Chalke: Sure. And in terms of the Pune hospital, particularly, considering Thane is already matured, but Pune is also equivalently big hospital. So the case mix, how is the case share, the CONGO mix share in that Pune hospital? How much scope you have to improve that CONGO share?

Ankit Thakker: How much scope do I have to improve what?

Amey Chalke: The CONGO therapies, the cardiac, oncology, gastro, et cetera, the

top 5 therapies, how much

do we have scope to increase the therapy share there?

Ankit Thakker: Good. I learned a new abbreviation today. I had not heard of that before, CONGO, okay. So I don't know. We don't track occupancy -- I mean, specialty -wise revenue too much. But I think Pune, though on occupancy level, I can't say that it is at its peak or full maturity, but on the complexity -wise, I think it is pretty much there. I don't think there is too much of scope to optimize case mix to a great extent in Pune, maybe small here or there. Indore definitely has scope to improve case mix, but not so much from Pune.

Amey Chalke: Sure. Just last question. So, going ahead, this remainder of the year because our Dombivli Hospital is coming in next year, how these remaining 3 quarters we should see from the ARPOB and the volume growth perspective? Any qualitative guidance would also help.

Ankit Thakker: Yes. So similar in line with what is happening. I think general organic growth that is a part of life cycle, you will see in both Pune and Indore with new beds, new time going past. So you should see some growth for Pune and Indore more than for Thane in this financial year before Dombivli starts contributing from next year.

Moderator: The next question comes from the line of Parth Singhal from SwingMaster Private Limited.

Jupiter Life Line Hospitals Limited

August 04, 2025

Page 5 of 8

Parth Singhal: My question is we are coming with the hospital in Dombivli next year. But going forward, how much expenditure we have incurred? And how much expenditure we must incur for starting the Dombivli hospital?

Ankit Thakker: Parth, so I think we have incurred something around INR200 crores of expenditure already till now, and we should incur similar, around INR200 crores more, in the next few months till we start the Dombivli hospital. So, we are halfway there in terms of capex .

Parth Singhal: Are we expecting our margins to decline?

Ankit Thakker: So -- yes, so first year, Dombivli should have negative EBITDA to some number. I don't know what that number will be. So while on the individual strength, the 3 hospitals will continue performing in line as they should. But on a consolidated level next year, there should be some

compression on account of Dombivli.

Moderator: The next question comes from the line of Abdulkader from ICICI Securities.

Abdulkader: First question is with regards to certain beds you were planning to add at your Thane hospital by adding one more floor, so, sir, where are we on that now?

Ankit Thakker: No. So there is no update on that front. The government is still not accepting applications in the Environment Committee from Thane. There is some Supreme Court story going on because of which new environmental permissions in Thane are not being granted to anybody. So, because it is subjudice and we are not competent to comment on that, I don't have any timelines today.

But as soon as I have a development, I'll definitely be happy to talk about it.

Abdulkader: Okay, sir. And secondly, on your change in the payor mix, so this quarter and I think last quarter as well, we saw slight increase in the insurance-based payers slightly. So would it be fair to assume it would be because of Indore Hospital ramp-up, which has happened now?

Ankit Thakker: Yes. So I think this is going to be a kind of lasting trend nationwide. The insurance penetration is increasing year-on-year everywhere in the country, and that will get reflected in the patients who walk into our facilities also. So, I think this trend -- I don't know at what speed, but I think this trend is likely to continue.

Abdulkader: Understood. And sir, lastly, on your expansion plan, so INR200 crores, you just mentioned that you had spent at Dombivli. And then from Q3 onwards, the new hospital at Pune is also starting to begin. So, for FY '26 and '27, what is the kind of capex outflow

we should see? And at the

same time, what would be the sustainable debt levels for the next 1 - or 2-year perspective?

Ankit Thakker: Yes. So capex, as I said earlier, Dombivli will need about INR200 crores more of capex, which should get consumed by Q1 of next year. And Pune in the initial phases will not need too much money. So if broadly, just as a thumb rule for everybody's benefit, if you break down the project implementation into 3 years, we spend around 15% in the first year, 25% in the second year and Jupiter Life Line Hospitals Limited

August 04, 2025

Page 6 of 8

60% in the third year. So that is a broad thumb rule calculation. So Pune will just spend about 15% in the next year.

Moderator: The next question comes from the line of Neil from Purnartha Investment Advisors Private Limited.

Neil: So in the Pune-Baner facility, there were another 11 beds that were available for expansion. So do we have any updates on that?

Ankit Thakker: So that is a new ICU, which is pretty much ready and commissioned. Based on occupancy, we will just hire some local manpower there and start using it. But currently, it is not commissioned.

Maybe sometime towards the end of the year, it should get commissioned.

Neil: Okay. So it's ready, but it will be commissioned towards the end of the year, right?

Ankit Thakker: Yes. Based on occupancy, correct.

Neil: Okay. And what do you expect the peak occupancies in Pune and Indore to be?

Ankit Thakker: So all the hospitals, we expect the peak to be in the mid-70% range.

Moderator: The next question comes from the line of Karan Mehra from Mehta Investments.

Karan Mehra: Sir, what is the estimated cost benefit or energy savings from the installation of the solar plant at the Indore facility? And also, do we have any plans to replicate this model at other hospitals as well?

Ankit Thakker: So Indore solar plant has cost us about roughly INR5 crores in capex. It is expected to result in around INR1 crore in opex savings annually with a lifetime of 20, 25 years. So that is the project outlay of solar in Madhya Pradesh. In Maharashtra also, we are thinking of adding some more renewable capacity in view of new hospitals coming in. So we are in advanced stages of discussing 3 megawatts of solar in Maharashtra as well.

Moderator: The next question comes from the line of Anjana Shah from Shah Investments.

Anjana Shah: I just would want to understand that we operationalized 78 beds at the beginning of Q4 FY '25. How has the market responded so far? Also, has the occupancy for these beds crossed about -- like above 50%?

Ankit Thakker: Yes. So in absolute terms, Indore occupancy has increased, and the new beds are getting operationalized. In specific unit-wise occupancies, currently, we have not disclosed. But qualitatively, I can tell you that the occupancy on the Indore side has increased.

Anjana Shah: Sir, additionally, if you could just help me understand how has the patient volume growth been on a Y-o-Y and a Q-o-Q basis? A ballpark update would also be helpful.

Jupiter Life Line Hospitals Limited

August 04, 2025

Page 7 of 8

Ankit Thakker:

Anjana Shah:

Ankit Thakker:

Moderator:

Dheeresh :

Ankit Thakker:

Dheeresh :

Ankit Thakker:

Dheeresh :

Ankit Thakker:

Dheeresh :

Ankit Thakker:

Dheeresh :

Ankit Thakker: Yes. So patient volume, as I said, was around 12% on a Y-o-Y basis. Q-o-Q numbers, currently, I don't have, but Y-o-Y was 12%.

Right. Sir, another question would be that a peer hospital has recently commenced operations in Thane. Have we observed any impact on patient volumes or any churn in key consultants or doctors because of that?

No. So, there is no impact on volumes or HR because of that. And our occupancies and performance are tracking in line as they were before.

The next question comes from the line of Dheeresh from WhiteOakCapital.

Sir, the phase out

of the capex that you gave, 15%, 25%, 60%, that is excluding land, right, because land is bought earlier.

Yes. That's right, Dheeresh.

Understood. Sir, on Dombivli, given that you said underserved dense population area, so how quickly do you expect to do the Phase 2?

So current thought is by year 2, we should breakeven on an EBITDA level. And after that, depending on how fast the occupancy ramps up, we will think of Phase 2. The thumb rule that we work by is once we have reached 60-odd percent occupancy at the installed base, we start commissioning work on Phase 2.

And Phase 2 would need only -- the civil is also done, right, along with Phase 1? It would only need...

Yes. Civil is correct. Yes, civil will be fully done for the whole hospital, small interior and biomedical-related capex for those beds. So the cost will be much lower for the additional beds.

So out of the INR500 crores project cost, how much would be left for Phase 2?

So we would spend a little over INR400 crores for Phase 1 and less than INR100 crores for Phase 2. Phase 2 may be broken into Phase 2 and 3 and things like that because no point adding everything together, but we'll keep adding those things.

Makes sense. Makes sense. And sir, like what we are seeing is a lot of hospitals are creating new benchmark in terms of breaking even on greenfield assets. So when you think about year 2 breakeven, you're thinking early part of year 2 or the late part of year 2?

I don't know. I'm not an astrologist. Let us see. There is too much of advanced crystal ball gazing.

I don't think it would be my expertise. But we will obviously put in efforts to make sure that we ramp up well and the clinical facilities are also in line with expectations.

Jupiter Life Line Hospitals Limited

August 04, 2025

Page 8 of 8

Dheeresh : Sure. And lastly, sir, is there an estimate of the 2 -year EBITDA burn that you have shared or you would like to share for Dombivli?

Ankit Thakker: So in the past, the experience has been around INR2 crores, INR2.5 crores a month. I think we should be similar. But once we have operated for about 1 or 2 quarters, maybe we'll understand better.

Moderator: The next question comes from the line of Saurabh Bhole, an individual investor.

Saurabh Bhole: First of all, very good, congratulations on a good set of numbers. Sir, I just want to highlight one thing that ALOS in days that you presented in your presentation, so sir, what is your expectation for this year's quarter -on-quarter growth of this ALOS?

Ankit Thakker: So ALOS for this year -- this quarter is 3.78 days. We think that ALOS is a reflection of the complexity of the cases that we do. For example, liver transplant will stay for 15 days, gallbladder for 2 days, because it is not possible for us to really predict what kind of patients will walk into our door. We can't make predictions on how the ALOS will pan out. But the trend has been over the last several quarters to be in this zip code of around 4 days.

Saurabh Bhole: And sir, my second question is for this financial year, what do you expect about your PAT? Because it's burning that you've shown here for depreciation and finance cost hike. So any expectation or anticipation that you have for this financial year?

Ankit Thakker: Yes. As I said, the gap between EBITDA and PBT or EBITDA and PAT should be a little wider

on account of depreciation and finance cost. In absolute terms, I don't have an expectation currently.

Moderator: As there are no further questions, I would now like to hand the conference over to Dr. Ankit Thakker for closing comments.

Ankit Thakker: Thank you, everyone. I hope the questions were answered satisfactorily. However, if there are any more questions or you need further clarifications, please feel free to contact SGA, and they'll connect you to us. Thank you.

Moderator: On behalf of Jupiter Life Line Hospi

tal s Limited, that concludes this conference. Thank you all for joining us, and you may now disconnect your lines.

Call: Nov 2025

November 13, 2025

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: - Q2 & H1 FY26 Earnings Conference Call – Transcript
Reference: Intimation of Earnings Conference Call dated November 01, 2025 and Audio Link of Analyst/ Investor Conference Call dated November 10, 2025.

Dear Sir/Madam,
Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q2 & H1 FY26 Results Conference Call held on Monday, November 10, 2025 at 1.30 PM (IST) for the quarter and half year ended on September 30, 2025.

The same will be available on the Company's Website at www.jupiterhospital.com

You are requested to kindly take the afore-mentioned on record and oblige.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 10

"Jupiter Life Line Hospitals Limited Q2 and H1 FY'26
Earnings Conference Call"

November 10, 2025

"E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges and the Company website on 10th November 2025 will prevail."

MANAGEMENT : DR. ANKIT THAKKER – JOINT MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER
MR. SIVASIS SEN – CHIEF FINANCIAL OFFICER

MR. ANAND APTE – CHIEF OF BUSINESS AND STRATEGY
MS. SUMA UPPARATTI – COMPANY SECRETARY & COMPLIANCE OFFICER
SGA - INVESTOR RELATIONS ADVISORS

Jupiter Life Line Hospitals Limited
November 10, 2025

Page 2 of 10

Moderator : Ladies and gentlemen, good day and welcome to Jupiter Life Line Hospitals Limited Q2 and H1 FY'26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen-only mode, and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements are not guarantees of future performance and involve risks and uncertainties that are difficult to predict.

I now hand the conference over to Dr. Ankit Thakker, Joint Managing Director and CEO. Thank you and over to you, sir.

Dr. Ankit Thakker: Good afternoon, everyone. I thank you for joining us on our Earnings Call to discuss the Business and Financial Performance for the 2nd Quarter and First Half of FY26. I hope you had a chance to go through our Financial Results and Investor Presentations, which are available on our website and on the Stock Exchanges.

I am joined today by Mr. Anand Apte – our Chief of Business and Strategy, Mr. Sivasis Sen – our CFO, Ms. Suma Upparatti – our Company Secretary and Compliance Officer and our Investor Relations Advisors from SGA.

This quarter, we have completed two years since our listing. We are pleased to report that our journey over the last two years has been fully in line with our plans leading up to the IPO. Our Dombivali Hospital is nearing completion and is set to begin operations on schedule without any foreseeable delay. The South Pune Hospital construction has commenced in this 3rd Quarter of the financial year, and the Mira Road Hospital is now on the architectural drawing board. All our three existing operating hospitals have also grown in line with expectations over the last couple of years. Our focus continues to be on expanding our presence in Western India by building high-quality hospital infrastructure with top-end technology and growing through clinical excellence, patient trust and community support. We remain committed to our ethos of care to keep on enhancing the patient experience and creating long-term value for all our stakeholders.

Now, coming to our financial performance for this period :

Q2 of this year versus last. The total operating income stood at Rs. 374.4 crores, an increase of 11.7% year-on-year. EBITDA for the quarter was Rs. 85.4 crores, a 9.3% increase year-on-year, Jupiter Life Line Hospitals Limited

November 10, 2025

Page 3 of 10

representing a 22.8% margin. The PAT for the quarter is Rs. 57.4 crores, that is 11% year-on-year growth, and the margin is 15.3%.

The H1FY26 numbers, the total operating income, stood at Rs. 727.4 crores, a 14.9% increase year-on-year, whereas EBITDA was Rs. 163.8 crores, a 13.7% increase year-on-year, with a margin of 22.5%. The PAT is Rs. 101.4 crores, with a growth of 5.3% year-on-year, representing a margin of 13.9%. The ARPOB for H1FY26 is Rs. 66,100. The AL OS is 3.84 days and the average occupancy rate is 62.2% on our expanded bed capacity. The payer mix for H1FY26 remains largely unchanged, with insurance accounting for 55.5% of the revenue, self-payers 43.2% and government schemes just 1.3%.

With this, I open the floor for questions and answers. Thank you.

Moderator: Thank you, sir. We will now begin the question-and-answer session. The first question is from the line of Mr. Rahul Jain from Jain Investment. Please go ahead, sir.

Rahul Jain: Yes, thank you so much for taking my question. I have two questions. First, how much CAPEX has been incurred in H1 FY'26, and

what is the current status of our Dombivali Hospital? When are we planning to start the same, and about their hiring process?

Dr. Ankit Thakker: Hi, Rahul. So, thank you for the question. The Dombivali Hospital, as I said, is on track. It is forecasted to start in Q1 of the next financial year. So, we are not anticipating any delays there. We have just started discussions on the recruitment front, which are expected to gain some

momentum in the coming few months. On the CAPEX side, this year so far, we have spent about Rs. 110 odd crores of CAPEX for the new project.

Rahul Jain: Thank you, sir. That is so much helpful. My second question is regarding our current EBITDA margin, stood at around 23%. So, how should we view the median term margin trajectory, especially with the Dombivali Hospital coming on stream? Is there any possibility of margin temporarily moderating towards 20%? Is that so?

Dr. Ankit Thakker: So, on an individual hospital level, mature margins are expected to be in the mid -20%. As the new hospitals come up, they are expected to be EBITDA negative in the first year and break even in the second year. That is our general observation and expectation. So, as Dombivali comes up, it should lead to a little dilution in consolidated margin in the first year. And then in the next year, while it stops dragging the EBITDA, then the next hospitals will come up.

Rahul Jain: Thank you, sir. That is very helpful. I will join back in the queue.

Dr. Ankit Thakker: Thank you.

Moderator: Thank you. The next question is from Mr. Deven from Marcellus Investment Managers. Please go ahead, sir.

Jupiter Life Line Hospitals Limited
November 10, 2025

Page 4 of 10

Deven: Yes. So, I just wanted to understand the difference between reported revenue and the one that we have given. Can you explain?

Moderator: Sorry to interrupt, Mr. Deven, you are breaking up.

Dr. Ankit Thakker: Yes, no, I understand what he's asking. I will paraphrase the question for everybody's benefit. So, from this period onwards, we have started reporting what is known as unbilled revenue, which we understand is in line with industry practice. So, previously, what we did is we only recognized revenue once the patients got discharged, which means at the end of every reporting period, all those patients who were still admitted in the hospital and had received partial services but had not been discharged, we did not report the partial revenue that was accrued on that account. This period onwards, we have also started reporting this unbilled revenue. The revenue for this quarter is Rs. 19.2 crores. So, this is the difference between your presentation number and the P&L reported number that you were referring to. This is a one-time delta which you are seeing in this quarter. Next quarter onwards, they are broadly expected to cancel each other out because in the beginning of the quarter, you will see a reversal entry of previous quarter unbilled revenue and towards the end of the quarter, you will see the unbilled accrued for that quarter, and in a steady state business, they should broadly cancel each other out. So, going forward from next quarter onwards, you should not see any significant impact of this.

Deven: Understood.

Moderator: Thank you.

Moderator: The next question is from the line of Mr. Amit from HG Hawa and Company. Please go ahead. Mr. Amit, can you please unmute yourself and ask your question?

Amit: Yes. Thank you for the opportunity. Sir, my question was connected to the debt. What is the strategic plan for debt reduction, and what is the company's policy, and what is the average cost of funding that the company spends?

Dr. Ankit Thakker: Hi, Amit. So, the current consolidated debt stands at Rs. 325 crores. Against that, we have liquid investments and deposits of over Rs. 500 crores, around Rs. 550 crores if I am not wrong at the

end of September. This is essentially a building up of coffers. The three projects that we have underway, that is Dombivali, South Pune and Mira Road, we expect that we should probably be able to complete with our resources and future internal accrual. This debt might kick in if we need or if we get an opportunity to do one more project. And if we do that project, then this debt will kind of continue for a few years.

Amit: And sir, what will be the blended cost of interest on this Rs. 325 crores?

Dr. Ankit Thakker: So, it is between 7% and 8%, I think, currently. But the carrying cost is pretty much around 1% because, as I said, we are holding.

Jupiter Life Line Hospitals Limited
November 10, 2025

Page 5 of 10

Amit: Doctor retention and wage inflation policy, because we want an expansion model, so doctor tabs and all these things will be like, what is the policy of the company?

Dr. Ankit Thakker: So, most of our doctors are on minimum guarantees with flexible paid systems. So, no one is really employed with a fixed cap. So, essentially, there is no wage as such. By and large, they are compensated commensurate with the efforts that they put in. So, we do not really have to discuss wage inflation with doctors on an ongoing basis for the most part.

Amit: I appreciate you answering my questions, sir. Thank you for the answers and all the best for the future.

Dr. Ankit Thakker: Thank you.

Moderator: The next question is from the line of Mr. Rishabh Sisodia, an individual investor. Please go ahead.

Rishabh Sisodia: All right. Thank you. So, my first question is on the stated pipeline of the new hospitals that we have. So, as you have mentioned, operationalization of the first phase of the hospitals, what is the expected timeline for the second phase? Like since the operation, would it be like one year down the line or maybe two years down the line when we add up the second phase of those individual hospitals?

Dr. Ankit Thakker: So, second phase will depend on how the occupancy ramps up. I do not really want to play astrologer and get into when that will happen. But once, our general philosophy is that once we cross 60% occupancy of the installed base, we think about expansion. So, as and when we manage to reach 60% of phase one, then gradually we will keep on adding beds.

Rishabh Sisodia: Okay. Second, do we have anything on the inorganic side on the cards or we plan to move out of the Maharashtra market? Because we are predominantly dominated over here, barring Indore.

So, do we have any plans to get out of this market, maybe like nearby Gujarat, MP or Karnataka, maybe?

Dr. Ankit Thakker: Yes. So, first inorganic, there is nothing live that we are chasing. But if and when there is something which we are discussing or that materializes, I will be happy to talk about it. On the focus areas, we are open to any sensible opportunity in Western India. We are open to Gujarat and Madhya Pradesh as well, not necessarily Maharashtra. But currently, we have seen more promising opportunities coming out of Maharashtra. Hence, those are the ones we have tapped.

Rishabh Sisodia: Okay, understood. So, last would be on the specialty mix. If you can break them down for first half, what is our specialty mix and what is our targeted mix, if at all we have anything in mind?

Dr. Ankit Thakker: We do not track specialty mix and we do not even have any target. We are a full service community hospital. So, whichever our specialty mix pretty much mimics the disease burden of Jupiter Life Line Hospitals Limited

November 10, 2025

Page 6 of 10

the communities that we serve. So, whoever walks into our doors, we are bound to serve them for their needs.

Rishabh Sisodia: Okay, that is helpful. So, one last question from my side. So, as in when you operationalize this new hospital, what is the expected timeline of getting the insurance

companies getting tied up for those hospitals?

Dr. Ankit Thakker: I do not know. It is a dynamic process. So, I really cannot predict when it will start. But yes, we understand that cashless services are important for patients in today's time. So, we will make our best efforts to make sure that the patients are not inconvenienced.

Rishabh Sisodia: Okay, that is all from my side. Thank you. All the best to you for the future.

Dr. Ankit Thakker: Thank you.

Moderator: Thank you. The next question is from the line of Mr. Dheeresh from White Oak Capital Asset Management Limited. Please go ahead, sir.

Dheeresh: Yes, thank you for the opportunity. Sir, the IP volumes for the quarter have declined, right? Low single digit. When you look at them year over year. So, if you can give some further insights into the hospitals that have seen more decline versus less decline.

Dr. Ankit Thakker: So, the IP volumes are pretty much flat. Last year, I think it was 27,200. This time it is 27,400. So, largely flat.

Dheeresh: You are talking about 1H, sir. I am talking about Q2.

Dr. Ankit Thakker: Q2. I have 1H numbers, but okay. Let me look up Q2. But basically, Q2 generally sees the occupancy peak due to infection burden, which superimposes over the average occupancy that we see throughout the year. So, generally, based on the infection outbreak, there is a year-to-year variation of a couple of percentage points in occupancy in Q2. This year, Pune had a lower outbreak than usual, which is on the community health side a good sign. But yes, so Pune did not see as high a peak occupancy as it did in the last year. So, I am guessing that could be contributing to the Q2 numbers. I do not have them right now, but that could be an explanation.

Dheeresh: All right. And for the adjustments that you have done, the adjusted growth is what you have shown in the PPT, right?

Dr. Ankit Thakker: So, the PPT shows as if there is no adjustment. It only shows operating numbers, operating revenue and operating EBITDA, because that is what we have been doing all along. So, I did not want confusion there. The annexure, P&L table shows the adjusted numbers.

Jupiter Life Line Hospitals Limited

November 10, 2025

Page 7 of 10

Dheeresh: Okay. So, Q2 is like 12% top-line growth and 9% EBITDA growth. So, this is lower than the 1Q growth run rate. So, the EBITDA, some of it I understand, is lower IP because of seasonality

this quarter. But is there some drag on the Dombivali in the cost that is reflective of lower EBITDA or is it just operating leverage?

Dr. Ankit Thakker: I think it is operating. I do not think Dombivali should be having a drag now. But I think towards the last quarter, we should start seeing some Dombivali effects coming in.

Dheeresh: All right. Thank you, sir.

Moderator: Thank you. The next question is from the line of Ms. Anjana Shah from Shah Investments. Please go ahead, ma'am.

Anjana Shah: Thank you for this opportunity, sir. So, I have a question. ARPOB improved from Rs. 57,700 to close to Rs. 66,100 during the first half of FY '26. So, if you could highlight what are the key factors are driving this almost close to around 15% increase?

Dr. Ankit Thakker: So, it is a combination of some case mix optimization in Indore. Some of the ongoing rate revisions happen with insurance companies on a periodic basis as and when the contracts expire and general inflation-based pricing.

Anjana Shah: Sure, sir. Thank you. That is it from my end.

Moderator: The next question is from the line of Mr. Ankeet Pandya from Baroda BNP Paribas Mutual Fund. Please go ahead, sir.

Ankeet Pandya: Yes. Hi. Good afternoon, and thank you for the opportunity. So, two, three questions from my side. So, firstly, just if you can take us through the occupancy level at all the three hospitals for the current quarter?

Dr. Ankit Thakker: So, I think

we do not have reporting of unit-wise occupancy. Consol level, I can tell you for this quarter, we were at 64.5%. This is on the expanded base of occupancy on, I mean, expanded base of beds that we added last year. So, last year, this quarter, we were at 70%. And this year, we are at 64.5%, but we added 150 odd beds this year.

Ankeet Pandya: Fair, sir, but just directionally or qualitatively, if you can just like a highlight, if possible

Dr. Ankit Thakker: Sure. Yes, qualitatively, I am happy to do that. So, Thane, mid-70s, it is largely plateaued as I have said earlier. Pune, going towards 70-ish and getting matured in the next couple of years. And Indore, we have added all those 80, 90 beds. So, they are showing an increase in occupancy and revenue both in the last couple of quarters.

Jupiter Life Line Hospitals Limited

November 10, 2025

Page 8 of 10

Ankeet Pandya: Thank you, sir. Sir, secondly, just for the quarter, on the professional expenses, professional fee side, there is almost 31%, 32% year-on-year growth and even sequentially almost 24% growth. So, anything one-off or this should be sort of like the way that one can work with?

Dr. Ankit Thakker: No, sir, this is completely one-off. I have put a short note, and this is pretty much in with the new accounting treatment that we have done. There is a Rs. 12 crore one-time provision in the professional fee component based on this new policy, which we have done. So, if you read the financials without this one-off Rs. 12 crores, Rs. 12.5 crore impact, then you will see the continual and ongoing operating cost.

Ankeet Pandya: Okay, fair enough. And just lastly, so once the Dombivali Hospitals gets commissioned, given that there is no other organized player in that market, how should one look at the occupancy and ARPOB for the first year onwards and in general, then ramping up? But for the first year, how should one look at it?

Dr. Ankit Thakker: Yes, there is no large provider in that region. We believe that in the long run, the demand should be very good in that region for the simple reason of lack of supply, like you said, and dense population. What will specifically happen in year one or year two, I do not want to hazard that guess. But in the long run, we see a very strong demand in that region.

Ankeet Pandya: But sir, can we assume that by at least by the second year or third year, we can go upwards of 50%, 55% of occupancy?

Dr. Ankit Thakker: Yes, so year two, I think we definitely think that a bit of break-even should happen. And that happens upwards of 40%, 45% occupancy, as we have seen. So, yes, I think that could be a decent assumption.

Ankeet Pandya: Okay. And just on the ARPOB also, can we assume that it will be significantly lower or like around 15%, 20% lower than the current company level ARPOB?

Dr. Ankit Thakker: A lot of the ARPOB is dependent on two main things, the pricing and the case mix optimization. In the early years of a hospital, you generally do more, you know, primary and secondary work and less of tertiary work. And second is pricing, which should be pretty much similar to the MMR pricing that we are getting now. But because of early years, we should definitely expect a slightly lower ARPOB in Dombivali than we see in matured hospitals.

Ankeet Pandya: Okay, that is it from my side. Thank you.

Moderator: The next question is from the line of Mr. Dheeresh from White Oak Capital Asset Management Limited. Please go ahead.

Dheeresh: Yes, thank you again. Just so that I properly understand the accounting, it says there was unbilled revenue, which is Rs. 19.2 crores. And the cost related to those unbilled revenues is Rs. 12.3

Page 9 of 10

crores, which has been included in professional fees. So, I am just curious to understand, it should have been across all the three lin

e items, right? Like there will be professional fees, there will be cost of goods, and there will be employee cost. Why is it all in professional fees? The cost of servicing that incremental unbilled revenue of Rs. 19 crores?

Dr. Ankit Thakker: Yes, so this Rs. 12 crores is two components. One component is a cost pertaining to this Rs. 19 crores that is the professional fee for this Rs. 19 crores, which is Rs. 3 crores. The other component is another Rs. 9.3 crores. That is, while we did this policy of unbilled revenue, we also thought that we should be a little more conservative in our provisions for the professional fee. So, so far, the understanding with the doctors is that the professional fee variable component gets paid only if the bills get settled by the insurance companies and when the money gets realized. So, for all outstanding bills, that is the money, that was not yet received by the company, there was no provision for the doctor's share. We thought that we should be a little more conservative and provide for the entire doctor's cost, assuming the money would come. So, the Rs. 9.3 crores is towards these unsettled and outstanding bills and the Rs. 3 crores is towards this unbilled revenue. So, the doctor's component includes these two things. As far as the cost of goods and other things are concerned, those were already getting captured on a PO-to-PO basis and it is very hard to define cost of goods based on specific IPs. So, even though we were not recognizing the unbilled revenue, but the material cost was getting reflected on an ongoing operations basis.

Dheeresh: Understood. And just to understand, this Rs. 9.3 crores you have recognized, but in reality, the way you pay out is if it is collected only then you pay out or the liability arises even if you do not recognize?

Dr. Ankit Thakker: Only if we realize the money we pay out. So, if there are any bad debts or anything, then to that extent, the provisions will get reversed.

Dheeresh: Okay. This is industry practice, or this is your policy? Because the doctor should not have the risk of, let us say, an insurance company not paying.

Dr. Ankit Thakker: This is an industry practice.

Dheeresh: It is an industry practice. Okay. All right. Thank you.

Moderator: Thank you. The next question is from the line of Mr. Shashank Sharma from HEM Securities. Please go ahead.

Shashank Sharma: Good afternoon. Thank you for taking my question. My only question is, sir, you were planning the seventh hospital, which was in the discussion stage last time. So, is there something concrete on this as to the location you are targeting on any land acquisition plan or something of timeline as such?

Jupiter Life Line Hospitals Limited
November 10, 2025

Page 10 of 10

Dr. Ankit Thakker: No, nothing concrete yet. We are in few discussions, but because it is organic and with all the uncertainties associated with land, currently I have nothing to report on that.

Shashank Sharma: Okay, sir.

Moderator: Thank you. Ladies and gentlemen, that was the last question for this session. I would now like to hand over the conference to the management for closing comments. Please go ahead, sir.

Dr. Ankit Thakker: Thank you, everyone, for joining. I hope the questions were answered satisfactorily. However, if you need any more clarifications or want to know anything else in particular, please do not hesitate to reach out to our IR team with SGA. Thank you.

Moderator: Thank you, sir. On behalf of Jupiter Life Line Hospitals Limited, that concludes this conference. Thank you for joining us and you may now disconnect your lines.

Call: Feb 2026

February 05, 2026

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P.J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980
Subject: - Q3 & 9M FY26 Earnings Conference Call – Transcript
Reference: Intimation of Earnings Conference Call dated January 23, 2026 and Audio Link of
Analyst/ Investor Conference Call dated February 02, 2026 .

Dear Sir/Madam,
Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find enclosed the transcript of the Q 3 & 9M FY26 Results Conference Call held on Monday , February 02, 202 6 at 10:00 AM (IST) for the quarte r and nine months ended on December 31, 2025 .

The same will be available on the Company's Website at www.jupiterhospital.com

You are requested to kindly take the afore -mentioned on record and oblige.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 13

"Jupiter Life Line Hospitals Limited
Q3 and 9M FY '26 Earnings Conference Call"
February 02 , 202 6

"E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges and the Company website on 2nd February 2026 will prevail."

MANAGEMENT : DR. ANKIT THAKKER – JOINT MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER
MR. SIVASIS SEN – CHIEF FINANCIAL OFFICER
MRS. SUMA UPPARATTI – COMPANY SECRETARY AND
COMPLIANCE OFFICER
SGA - INVESTOR RELATIONS ADVISORS

Moderator: Ladies and gentlemen, good day, and welcome to Jupiter Life Line Hospitals Limited Q3 and 9M FY '26 Earnings Conference Call. This conference call may contain forward-looking statements about the company which are based on the beliefs, opinions and expectations of the company as on date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. Ankit Thakker, Joint Managing Director and CEO. Thank you, and over to you, Mr. Thakker.

Ankit Thakker: Good morning, everyone. Thank you for joining us on our earnings call to discuss the business and financial performance for Q3 and 9 months of FY '26. I hope you had an opportunity to review our results and investor presentation, which have been uploaded on the website and the stock exchanges. I'm joined today by Mr. Sivasen, our CFO; Mrs. Suma Upparatti, our Company Secretary and Compliance Officer; and our Investor Relations Advisors, SGA.

I'm delighted to kick off this Monday morning call with good news. Our hospital in Dombivli is ready to be launched. The 750,000 square foot 500-bed structure has been completed before time and on budget, ahead of its planned launch in Q1 of FY '27. This was a phenomenal project execution effort wherein we were able to complete the entire construction for the 500-bed, fit-outs for over 300 beds and all biomedical equipment installation in just over 24 months and at a capex of roughly INR425 crores.

I want to take a moment here to thank all our partners in this project, beginning from the consultants, contractors, vendors, our own project team and also the regulatory authorities, all of whom went above and beyond to enable us to deliver this project.

We also welcome all the doctors and the team members who have already come on board to join us in our mission to deliver the kind of health care to the region around Dombivli that the community has come to expect from Jupiter over the last 2 decades.

The hospital is slated to be inaugurated on February 15 and will commence full clinical operations thereafter. In the interest of operational efficiency, we will only begin operating 200 beds in Phase 1 and then ramp up capacity in a phased manner as the occupancy increases. A heads up for everyone on this call, though, that beginning next quarter, you will see a much higher depreciation load than you have seen so far and should also expect an EBITDA drag on the consolidated numbers for around the next 2 years before the new hospital can start contributing financially.

Jupiter Life Line Hospitals Limited
February 02, 2026

The Pune South project construction has kicked off and is progressing well, and the Mira Road project is under regulatory approval process. Alongside this expansion, our existing hospitals continue to demonstrate steady operating momentum with no surprises or specific highlights to report other than the impact of the new Labor Code. Our PBT in this quarter is, therefore, impacted to the tune of INR6.4 crores due to this exceptional onetime provision.

I will give you the highlights of our consolidated financial performance now. The numbers for Q3 of this financial year. Total income stood at INR365.3 crores in this quarter, an increase of 9.8% year-on-year. EBITDA stood at INR83.4 crores, an increase of 9.2% year-on-year. The EBITDA margin is 22.8% in this quarter. The PAT stood

at INR42.5 crores, representing a decrease of 18.7% year-on-year and the margin for the quarter is 11.6%.

The 9-month numbers. Total income is INR1,111.9 crores, an increase of 15.1% year-on-year. The EBITDA is INR254 crores, an increase of 15.2% year-on-year. The EBITDA margin is 22.8% for the 9-month period. The PAT is INR143.9 crores for the 9-month period, a decrease of 3.1% year-on-year. As highlighted earlier, the PAT is impacted due to the new Labor Code changes and the margin is 12.9%.

The ARPOB for the 9-month period is 66,800. The ALOS is 3.85 days and the average occupancy for 9 months is 61.9%. The payer mix, insurance represents 55.7%; self-payers 43.2% and government schemes at just around 1.1%.

With this, I conclude my opening remarks and open the floor for questions and answers. Thank you.

Moderator: Thank you. We will now begin the question -and-answer section. The first question comes from the line of Manav Jain from Jain Investments .

Manav Jain : Congratulations on the opening of the new hospital. My question is regarding the Pune Bibvewadi project. Could you please share an update on the current stage of the construction and the total capex incurred to date for the hospital?

Ankit Thakker: Thank you. So the Bibvewadi project is, as I said, construction has begun. We have just started the basement work. So it is in the early stages of construction. The excavation is now over. As we have said , we are slated to begin sometime in calendar year '28, and we are reasonably sure to achieve that target. Capex so far, as you can imagine, because it's only excavation and a little bit of basement , which is not very high , but maybe less than INR50 crores so far.

Manav Jain : Okay, sir. And could you please share the total capex incurred during 9 months FY '26 and also provide hospital -wise capex for Dombivli and Pune facilities?

Ankit Thakker: So, Dombivli, as I said, INR425 crores is the total that we have spent for the project. Pune, I can see a number of around INR45 crores so far. Mira Road, we have not started.

Moderator: Next question comes from the line of Ashutosh Nemani with JM Financial Family Office .

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 4 of 13

Ashutosh Nemani : So my first question is regarding the ramp -up plan of Dombivli Hospital. So how do you anticipate occupancy ramp -up, if you can guide, let's say, for the next 3 to 4 quarters? And how would be the specialty mix? And what is the expected ARPOB of the hospital?

Ankit Thakker: So as I've said, you have to look at it in a longer term. The first 3 -year period, we consider as a stabilization, foundational or maturity kind of period. And the next 3 -year period, we consider as a rapid growth period. So these are long -term infrastructure kind of community assets, and we don't really look at it quarter -to-quarter for the first couple of years. How exactly it will ramp up remains to be seen. But typically, for all the projects that we have done, we consider 2 to 3 years as an establishment period.

The specialty mix will be identical to all other hospitals. We will offer all services here beginning from child birth to transplantation, as I have said. The revenues should be in line with the Mumbai region, Thane hospital that we have.

The ARPOB, of course, is also representative of the case mix. So initially, when a hospital starts , it has a relatively lesser load of complex services or tertiary quaternary work and more load of primary secondary work. So, as the hospital gains maturity, the ARPOB should converge. But in the earlier periods, ARPOBs will look smaller than the Thane hospital.

Ashutosh Nemani : Okay. And what will be the timeline for breakeven of this hospital and EBITDA drag for the first 2 to 3 years?

Ankit Thakker: By end of year 2, we expect to be EBITDA breakeven.

Ashutosh Nemani : And the third question is,

could you tell us the ARPOB of Q3 '26 on a consol basis?

Ankit Thakker: ARPOB of Q3 '26 on consol basis is 68,000.

Ashutosh Nemani : And occupancy?

Ankit Thakker: Occupancy is 61.4%.

Moderator: The next question comes from the line of Kritika Damani with Prospera Financial Solutions .

Kritika Damani : Congratulations on a strong quarter. I know despite the sustained EBITDA margins, depreciation and finance costs have been rising due to recent and ongoing capex . How should we think about the margin trajectory over the next few quarters as the new assets start contributing, but the costs are already in the P&L?

Ankit Thakker: So Kritika , on the 3 operating hospitals, 2 are near maturity, so they should have similar margins. Indore is in its growth phase. We should expect slightly incremental margins from Indore over the next couple of years. And Dombivli, as I just said a short while back, should lose money for the first year or 2. So there will be a little bit of drag on the consol numbers on account of Dombivli for the next couple of years.

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 5 of 13

Kritika Damani : And my second question is with the multiple greenfield hospital under development alongside the mature hospital, I mean, what internal metrics do you monitor most closely to ensure growth does not begin to dilute operational control or clinical outcomes?

Ankit Thakker: So both are completely independent teams. The project team has very little or nothing to do with the operations and the clinical operating teams, the medical teams have absolutely nothing to do with the new projects going on. So both of them have their separate goals cut out very clearly and new greenfields coming up have zero impact on our clinical performance or operational

performance.

Moderator: Next question comes from the line of Himanshu Binani with Anand Rathi.

Himanshu Binani: So sir, I have one question basically now the European trade deal is like largely done. So what we are seeing is that there has been like an absolute decrease in the import duties of medical equipment. So how one should actually look into the capex numbers going forward as in what is your initial understanding in terms of the capex, which can be reduced or there is like any sort of like saving into the capex numbers for you as well as for the industry?

Ankit Thakker: I have two understandings. One is that headlines are different and fine prints and implementation is different. So I would wait to see how it gets implemented and what is in the fine print. And the other thing, just to set context also for everybody's benefit is that even though a lot of companies are headquartered today in Europe and the U.S., a lot of them have manufacturing facilities in China. So the country of export becomes China and the deal with Europe, I don't know how much impact it would have in machines being shipped out of China.

Himanshu Binani: Okay. Got it. And sir, second question is on the Dombivli project. So what would be the initial like the EBITDA drag from that hospital for the first 2 years?

Ankit Thakker: So remains to be same. In the past, as I have said, our previous hospital experience is first year, something between INR 2 crores to INR 3 crores a month should be the average for the first year. But let us see how this year.

Moderator: Next question comes from the line of Kaustav Bubna from BMSPL Capital.

Kaustav Bubna: So I basically wanted to understand for all the hospitals what's your strategy when you find land to create a new hospital? Are these hospitals in areas where you have no competition around you? And then if that's the case, how do you get good doctors and good faculty if these hospitals are in common areas where accessible to easily accessible to good faculty and doctors?

Ankit Thakker: Yes. So Kaustav, the metric for selection of locations is multifold

. A, we are currently only

looking at large cities in Western India. Typically in, when you are in a large city, it is also a proxy for HR and availability of manpower, including doctors. But within the large cities, we do look for those specific micro markets where the resident population is very high. And in the near vicinity, there would be a relatively low supply of high-end tertiary care services. This does

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 6 of 13

not mean that there are no other hospitals because all large cities have covered, but it means that they are predominantly supplied by Tier 2 hospitals.

The other thing that I must highlight is that the Tier 2 hospital is Tier 2 because of infrastructure and not necessarily because of manpower. A lot of these doctors in big cities, they are all trained similarly to, for example, doctors in Dombivli and doctors in South Bombay, they would be classmates in the same medical school. For example, they may be from JJ or KEM or whatever. But just based on where they live and work, the quality of infrastructure and technology available to them may not be commensurate with their qualifications, and they make do with what they have.

So when we enter these micro markets, it becomes a natural choice for a lot of these skilled medical practitioners to want to affiliate and associate with us, and it becomes a win-win symbiotic kind of association. So I hope that gives you some color and understanding on what we are doing and how.

Kaustav Bubna: So is this the same strategy for your Pune hospitals, too, the one that's coming up? Is it the same strategy across every single hospital?

Ankit Thakker: Yes.

Kaustav Bubna: Okay. And I wanted to understand 3 years or 5 years down the line, what is our ARPOB target?

I mean is there some level of offerings which have higher average revenue per bed which hospital currently does not undertake? I mean just could you strategically give some indication on will ARPOBs remain at similar levels? Or are we trying to increase higher ARPOB offerings?

Ankit Thakker: So I'll answer the second question first. There is no service that we don't offer currently. We practice all branches of medicine and offer critical tertiary, quaternary level services in all branches of medicine. We do not have any ARPOB target either now or for the future. We think that this ARPOB number is more of a byproduct of what you do. We are not looking at or chasing any specific ARPOB number.

We will continue to provide all services in all the hospitals because that aligns with our operating and clinical philosophy. We don't want to have narrowly focused hospitals. We want to have broad-spectrum full-service hospitals. And irrespective of which branch generates how much ARPOB. Honestly, we don't track it internally also. So yes, whatever happens.

Generally, in the initial phases of operations, as I was saying earlier, the ARPOBs are lower because you do more of primary and secondary work. But as the hospital matures, the ARPOB also reaches maturity. After which, it only grows in line with inflation. So that would be my

broad guidance to you that the mature asset ARPOBs should grow in line with inflation and the newer hospitals should grow a little faster in the earlier phases. And once they mature, they should grow in line with inflation. But no, there is no target.

Moderator: Next question comes from the line of Amit Thawani with Clear Blue Capital .

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 7 of 13

Amit Thawani : I think this is the first quarter we've reported single digit top line growth. Can you explain what has happened this quarter?

Ankit Thakker: Nothing has happened. Thank you for bringing it to my notice. I have not noticed that. But , a couple of questions back said that health care cannot be monitored on quarter -to-quarter. Also, each

hospital has finite capacity . Very soon, I would hope that all the mature hospitals are able to report zero growth because they are already at maturity.

So the growth eventually comes from new and upcoming hospitals where you start from zero.

Then e now does not really have too much ability to generate more occupancy growth. Pune, the opportunity is narrowing. Indore still definitely has opportunity for higher growth, and we hope to see that play out over the next couple of years.

Amit Thawani : But any issue on the payer side? I mean we had some problem with insurance companies as an industry. I'm not sure how severe that problem was with Jupiter?

Ankit Thakker: No, we have not had any serious problem with the payer companies. We have had largely uninterrupted services for almost all payers. We also have not had any unusual friction with the payer community. So yes, I would not attribute anything to friction with the payers.

Amit Thawani : Do you see any takeaways for us from the new EU -India trade deal on medical tourism?

Ankit Thakker: I have not caught if there is any impact on medical tourism per se for the EU deal. I'm not sure if there is, but I will look it up. On the import duty side, as the question some time back, if it does play out the way it is publicized, especially that equipment being shipped out of Europe, hopefully should get cheaper, and that should, to some extent, at least help us counter the depreciating rupee. So that might be welcome.

Amit Thawani : Got it. My last question, sorry, I think I might have missed the answer for this, can you tell me what the Y-o-Y growth in ARPOB is? And yes, if you can just break up the revenue growth into ALOS, ARPOB?

Ankit Thakker: ALOS, I don't have -- what I have is last year, ALOS was for 9 months, 3.88. This point in time, it is 3.85. So largely similar. ARPOB was 59,000 plus something, and this time, it is 66,000.

Amit Thawani : So almost entire 10% growth this quarter is from ARPOB?

Ankit Thakker: Yes.

Moderator: Next question comes from the line of with Aryamaan with Prudent IM.

Aryamaan : Just one question from my side. Sir, what could be in respect to the CGHS price hike? So what could be the benefit from this? And what's the time line you see it will benefit at all?

Ankit Thakker: So your voice is muffled, but what I heard is you were asking about CGHS. Is that correct?

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 8 of 13

Aryamaan : Yes, CGHS price hikes.

Ankit Thakker: Yes. So we currently don't have any CGHS exposure on our P&L. So as we stand today, it does not impact us. But I understand that the CGHS rates are revised after the period of 10 years, and there is a substantial hike that the government has offered this time. But as far as Jupiter is concerned, you will not see any impact on that at all.

Moderator: Next question comes from the line of Jai Jain with J J Investments .

Jai Jain : Sir, just one question from my side. Last year, we added 78 beds at Indore hospital. How is the hospitals overall performance since commissioning of those beds, particularly in terms of occupancy ramp -up and ARPOB growth, if you can just highlight us?

Ankit Thakker: Yes, Ja i, you are right. We added about 78 beds last year. Of course, because of a larger base, the percentage occupancy is lower in Indore. But on an absolute term, I'm happy to confirm to you that the occupancy this year is higher than what it was last year and that the 78 beds have started getting utilized and are being put to use.

The ARPOB side, we should have had a growth of, let me see, maybe 15 -odd percent, I think, on account of both inflation and this maturity phase. As I was saying, you will see a higher ARPOB growth in the first 6 -odd years and until it reaches maturity. After which, it only grows in line with inflation. So Indore ARPOB for the next couple of years should grow a

little faster

than inflation logically, and that is what we are also seeing play out.

Moderator: Next question comes from the line of Rishi Kapoor with FIL.

Rishi Kapoor : My question is regarding the demand supply situation. So I just want to have the general view, you brought a view regarding this. So how is the demand -supply situation evolving across the Thane, Pune and Indore, given the rising occupancies and ongoing capacity additions?

Ankit Thakker: So in the current 3 locations, it is pretty much status quo. The population keeps increasing. The insurance penetration keeps increasing in large Indian cities. So on account of both those factors, the demand is still very high. I don't think the supply situation is enough in either of the 3 markets at a high quality. And I think that high -quality supply will very easily get absorbed in Thane, Pune and Indore, even if more were to come from where we stand today.

Rishi Kapoor: All right. And regarding specifically about the ARPOB growth that you talked about the new hospital that is coming. But on a consolidated basis, any view on the ARPOB growth outlook for the next 1 to 3 years?

Ankit Thakker: I don't know how it will really play out because there will be some drag from Dombivli and some positive from the other 3 on account of inflation and higher growth of Indore. So on a blended level, how it will play out, I have not really done too much modeling around it. But Dombivli will be diluting. Indore will be a little higher than inflation. Thane Pune will be inflation -linked growth.

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 9 of 13

Moderator: Next question comes from the line of Ashutosh Nemani with JM Financial Family Office.

Ashutosh Nemani : First question is , with so much bed addition for the next 2 years , there have been some concerns highlighted by the peers regarding the availability of star doctors and specifically nurses also.

So how do you foresee that? Do you see doctor costs rising substantially when all this capacity comes up in the industry?

Ankit Thakker: So nurses is a national challenge, and there is nothing new. It has been a challenge for a long time, not specific to one hospital or one location. There is no magic answer to it. I think it will continue to remain a challenge. On the doctor side, my view is that if you are in large cities, then you don't have too much of a problem.

As you start going into smaller and smaller Tier 3 and those kind of locations, then the availability of doctors is much lower. And interestingly, the doctor cost in smaller cities is much higher than doctor cost in larger cities. But in the locations that we operate, it is not hard to find good doctors.

Ashutosh Nemani: Could you just like quantitatively , tell us what is doctor cost as a percentage of revenue? And how has it evolved in the past 2 to 3 years?

Ankit Thakker: So I don't have quantitative numbers. But typically, the doctor cost varies between 20% and 25% of the top line and we have generally been in that range.

Ashutosh Nemani: One of the peers has also added a hospital in that region. So any impact on doctor attrition you are seeing occupancy more or less stable?

Ankit Thakker: No impact on doctor attrition. We have had one full -time doctor move, so I would call it zero attrition.

Moderator: Next question comes from the line of Kaustav Bubna with BMSPL Capital .

Kaustav Bubna : I just had a few more questions. So in previous interviews, it was mentioned that you may think about adding a seventh hospital. So do you have any updates on that in terms of how you were thinking of where where do you think is a good location for your seventh hospital? How are you thinking about that, if so? And I think I have a broader vision question also along with that, I mean, what is your strategy really in terms of capex -led growth over the next decade, taking into si

tuation, obviously, taking into understanding your perception of demand supply over the next decade? So if you could answer those questions.

Ankit Thakker: So I'm happy to get a question about a decade and not a quarter. I like longer -term thinking than quarterly thinking, especially for health care. As the demand supply stands today, even now as we speak, there is a huge mismatch at a national level. Even at prominent high Tier 1 metro city levels, I think we still have a lot more supply to create than what we need.

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 10 of 13

We have to replace some of the old secondary care nursing homes with respectable health care operators because nowhere else in the world is health care provided in 15, 20, 30-bed nursing homes as it is provided in Mumbai and other cities of India.

So on the supply side, I'm absolutely convinced that there is more needed than what the whole

industry collectively will be able to deliver, the players today will be collectively able to deliver over the next decade. So I think the growth story on Indian health care is probably longer than a decade. There is just too much to be done.

And along with that, as time goes on, more and more cities progress, they become larger and they become centers which will be able to absorb large -scale hospitals because of rising population, rising income levels and higher insurance penetration. So at an industry level, I think it is a multi -decade story. It is not a yearly or certainly not a quarterly story.

As far as Jupiter is concerned, you are right in your observation, we are keen to do the seventh hospital as well. We remain committed to Western India for now. We are in discussions with a few opportunities currently, but I don't have any announcement to make about where that location will be and when it will really mature because we are keen to do a greenfield and land being land with all its uncertainties in India, you can't really go out and make predictions before you deliver.

Kaustav Bubna : So as a company, you over the next decade, you plan to participate in the supply growth. You will not it's not like I want to stay at 8 hospitals, 7 hospitals and then let the supply grow as much, we won't grow anymore. You plan to grow with the supply growth. Is that a correct understanding?

Ankit Thakker: There is nothing else I know how to do. So we'll have to keep doing this.

Kaustav Bubna: Okay. And just one last question on the doctor aspect. I know it's an ongoing struggle. It's just part of the industry. But also, please, if you could help my understanding better because this question comes out of a little bit of lack of understanding and knowledge about exactly how this works and how it is currently. But let's take Reliance Hospital, for example, in Lokhandwala in Mumbai.

I know that, that hospital offers now tells doctors that listen, visit us exclusively. And obviously, and they apparently provide a very nice package to doctors, which this incentivizes doctors to go and look for other opportunities to say that we'll compensate you well enough to be with us only. So are other hospitals also doing that apart from Reliance? And how does we do that? And if we don't have only Reliance is that and other hospitals are doing that, how does that affect retaining doctor talent?

Ankit Thakker: So I don't have specific case studies on different hospitals, but I'll tell you my thought process.

My thought process is that in the long term, it is not sustainable to compensate people significantly above or significantly below industry standards. You have to be in line with the Jupiter Life Line Hospitals Limited
February 02, 2026

Page 11 of 13

industry because significantly above naturally is inefficient. Significantly below you risk continual attrition and in unstable teams. So you have to be fair in your compensation models.

As far as we are concerned, we have a mix of some doctors who choose to exclusively practice with us, and we are also okay if -- who we consider as respectable professionals, some of them would want to come on a visiting basis as well.

But what we do endeavor and we have achieved is that all branches of medicine will have someone exclusively working with the hospital so that if a patient shows up at our door at no point in time, we are in a position to not deliver on the care. So we will have exclusive practitioners in all branches, but we will not insist on only having exclusive practitioners.

Kaustav Bubna: Okay. Great. And if I could just squeeze in one more. I mean you see hospitals in South Bombay and all these known hospitals, not just across Bombay. There is a big selling point to get customers is to have the best known doctors perform surgeries and do consulting in their hospitals.

So I mean since our locations are not the status quo patients in terms of accessibility from where those doctors live, like obviously, Dombivli would be a little further away from, let's say, where a very popular doctor lives who is known to be the best in the field, how does the company get those doctors compensate those doctors or what provide attractive opportunities to those type of doctors to come and consult and operate at, let's say, Thane or Dombivli hospital of Jupiter?

Ankit Thakker: So, Kaustav, the definition of popular is known amongst the people. So the question is which people. The doctors who you are defining as popular in South Bombay are unknown names in Coimbatore. So medicine is a local and hyperlocal service. You definitely need known and trustworthy doctors who are available in the region where you can seek care.

There are extremely good doctors in MD Anderson and in Cleveland Clinic. But unfortunately, South Bombay cannot access them. It does not mean South Bombay does not go to some other people who they consider popular. So that is similar for all locations in the world, that each micro market will have people who are more popular, more qualified. And based on those locations, people will choose where they want to seek care.

Moderator: Mr. Bubna, please rejoin the queue for more questions. Next question comes from the line of Shashi Ranjan with Anandan Capital.

Shashi Ranjan : Just for my enlightenment, can you help me understand, do we cater to P MJAY or Ayushman Bharat cases or extremely weaker section patients who come to us? And in case we cater to them, are we getting paid by the government on time?

Ankit Thakker: So as you must have seen from our report, we have just 1% of the revenue coming in from the government scheme. So in our view, that is our social contribution. We only have government schemes for 2 branches, radiation treatment for cancer and congenital heart diseases for children, pediatric heart problems. So these are the only 2 problems where we have government schemes.

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 12 of 13

So it is not a meaningful impact on our P&L. And in any case, in our mind, it is a social contribution. So yes, they don't really impact us too much.

Moderator: Next question comes from the line of Amit Thawani with Clear Blue Capital.

Amit Thawani : I wanted to pick your brain on what you believe is the long-term ARPOB growth.

Ankit Thakker: So I've just answered that at length earlier. ARPOB will grow in line with inflation for mature hospitals and faster in the first few years. Once it reaches maturity, it will be in line with inflation.

Amit Thawani : when we renew with our insurers, what is the insurance renewal that we are taking from them?

Ankit Thakker: Inflation linked.

Amit Thawani : is it quantified in our contracts?

Ankit Thakker: It can't be quantified in advance, but at the time of each renewal, it is negotiated.

Amit Thawani : Okay. At the time of renewal

we are negotiated based on what the prevailing inflation is?

Ankit Thakker: Correct.

Amit Thawani : Okay. And because I believe our costs could go higher than inflation linked. I mean cost of nurses and doctors could be higher than inflation. and our ARPOB is also going to be linked to inflation. So how is that going to impact our margins?

Ankit Thakker: This is going to be a long answer, Amit. I think we should catch up separately.

Amit Thawani : If you can give it in a couple of lines I'll appreciate it. I mean I just want to know if there is a long-term impact on our margins.

Ankit Thakker: I don't know. Let us see whatever happens to the industry will happen, how the macroeconomics will play out, how the wages will happen over the next several years, what will be the impact of trade and capex and consumables and GST over the next multiple years, and how will taxation play out. Too much of astrology is difficult to do. I'm sorry, but I really don't have too many answers about this.

Amit Thawani : But last, there is something called a common empanelment program. We don't plan to join that, right?

Ankit Thakker: No. Currently, there is nothing under discussion with Jupiter on common empanelment.

Moderator: Ladies and gentlemen, that was the last question for today. We have reached the end of the question-and-answer session. I would now like to hand the conference over to the management for closing comments.

Jupiter Life Line Hospitals Limited

February 02, 2026

Page 13 of 13

Ankit Thakker: Thank you, everyone. I hope that I have answered the questions satisfactorily. If anyone has more questions, please feel free to reach out to SGA, and they'll put you in touch with us. Thank you.

Moderator: Thank you. On behalf of Jupiter Life Line Hospital s Limited, that concludes this conference. Thank you for joining us. You may now disconnect your lines.

Call: May 2026

May 21, 2026

To,
National Stock Exchange of India Ltd.
Exchange Plaza,
Bandra -Kurla Complex,
Bandra (East), Mumbai -400 051
Symbol: JLHL To,
BSE Limited
P. J. Towers,
25th Floor, Dalal Street, Fort
Mumbai 400 001
Code: 543980

Subject : Q4 & FY 2 6 Earnings Conference Call – Transcript

Reference: Intimation of Earnings Conference Call dated May 11, 202 6 and Audio Link of Analyst/ Investor Conference Call dated May 1 8, 202 6

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find e nclosed the transcript of the Q4 & FY 2 6 Results Conference Call held on Monday , May 1 8, 202 6 at 10.30 A M (IST) for the quarter and Financial year ended on March 31, 202 6.

The same will be available on the website of the Company at www.jupiterhospital.com .

You are requested to kindly take the afore -mentioned on record and oblige.

Thanking you.

For JUPITER LIFE LINE HOSPITALS LIMITED

Suma Upparatti
Company Secretary & Compliance Officer

Page 1 of 9

“Jupiter Life Line Hospitals Limited
Q4 & FY26 Earnings Conference Call”
May 18, 202 6

“E&OE - This transcript is edited for factual errors. In case of discrepancy, the audio recordings uploaded on the stock exchanges and the Company website on 18th May 2026 will prevail.”

MANAGEMENT : DR. ANKIT THAKKER – JOINT MANAGING DIRECTOR
AND CHIEF EXECUTIVE OFFICER
MR. SIVASIS SEN – CHIEF FINANCIAL OFFICER
MRS. SUMA UPPARATTI – COMPANY SECRETARY AND
COMPLIANCE OFFICER
MR. ADITYA GUPTA – SENIOR VICE PRESIDENT
CORPORATE AFFAIRS
SGA – INVESTOR RELATIONS ADVISORS

Jupiter Life Line Hospitals Limited
May 18, 202 6

Moderator: Ladies and gentlemen, good day, and welcome to the Jupiter Life Line Hospitals Limited Q4 and FY26 Earnings Call. This conference call may contain forward-looking statements about the company, which are based on the beliefs, opinions and expectations of the company as on the date of this call. These statements are not the guarantees of future performance and involve risks and uncertainties that are difficult to predict.

As a reminder, all participant line will be in listen-only mode and there will be an opportunity for you to ask question after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing star then zero on your touch-tone phone. Please note that this conference is being recorded.

I now hand the conference over to Dr. Ankit Thakker, Joint Managing Director and CEO. Thank you, and over to you, sir.

Ankit Thakker: Good morning, everyone. Thank you for joining us this Monday morning, for Jupiter Hospitals earnings call for the '25-'26 financial year. With me today are Sivasish Sen, our CFO; Suma Upparatti, our Company Secretary and Compliance Officer; and Aditya Gupta, our Senior Vice President of Corporate Affairs. I hope you had a chance to go through the investor presentation and financials we uploaded on the exchanges and our website.

Before I get into the numbers, I'd like to take you through the year, what we accomplished and where we are headed. FY26 was in many ways, a momentous year for us. The inauguration of Dombivli in February marked the fulfillment of a commitment. It was the first greenfield hospital we have launched since listing.

We had set ourselves a target of commissioning it by Q1 FY27, and I'm very pleased that we beat that and opened ahead of schedule. To give you a sense of the scale, we built 750,000 square feet, the entire superstructure for 500 beds and completed fit outs for approximately 300 beds, all in just over 24 months. And we did it without any cost escalations on time, on budget, no surprises.

The early response from doctors and the local community has been very encouraging.

Accreditation and insurance empanelment processes are underway. And we remain confident about achieving the year 2 breakeven that we guided to.

Even as Dombivli ramps up, our other projects are moving along well. Pune South is on track to open in calendar year '28, as we had committed. Mira Road has completed its architectural drawings and is undergoing regulatory clearances. We expect excavation to begin by the end of the year. Dombivli, Pune South and Mira Road were all commitments we had made explicitly. BKC, however, is something a little different. BKC is one of the most prominent addresses in the country. Putting up a 400-bed quaternary care hospital on over 1 lakh square feet of land there, is not just another hospital. It's a declaration that Jupiter is ready to compete at the very highest levels in India's most consequential health care market. But we will build it, the way we build everything else, patient-centric, clinically efficient and with responsible capex.

Jupiter Life Line Hospitals Limited

May 18, 2026

The goal from day 1 will be to earn the trust of patients, who will have every option in the world available to them. With BKC in the pipeline, we are looking at approximately 1,700 new beds in this expansion cycle, taking our long-term capacity to nearly 3,000 beds. That's a network we believe will be genuinely differentiated and will cement our position as the leading hospital group, in Western India.

We are not trying to just grow fast. We are trying to grow right. Every project in our pipeline is greenfield. Each one is designed as a full-scale hub, not a spoke, not a satellite, but a flagship facility in its own right. Every one of them is in a densely

populated urban geography in Mumbai

and Pune. And we pretty much own all the real estate.

This is an asset-heavy model by deliberate choice. We believe owned infrastructure gives us the stability of operations for the long-term value of what we are building. Across the 4 hospitals in this cycle, from Dombivli all the way to BKC, the 1,700 beds will come up at an average cost of around INR1.5 crores per bed.

A quick word on the balance sheet before moving to the P&L numbers, we deployed around INR500 crores in capex this year, Dombivli, BKC land payment, and ongoing Pune and Mira Road projects. And yet on a net basis, we are still cash positive. The gross debt stood at around INR500 crores, while the cash on hand was around INR545 crores.

This has been possible because of the strong operating cash flows coming in from our hospitals in Thane, Pune and Indore. Our current view is that the internal accruals over the next 4-odd years, combined with debt at under 3 times EBITDA, should be sufficient to fund everything

that we have announced.

Now to the numbers. Total income for FY26 is INR1,499.8 crores, up 15.2% year -on-year. EBITDA came in at INR343.3 crores, up 14.4% with margins at 22.9%. PAT for the full year was INR194.2 crores.

For the quarter, the revenue was INR387.8 crores, up 15.1%. EBITDA INR89.2 crores, representing a 23% margin and the PAT was INR50.2 crores, up 11.5%. ARPOB grew 11.7% to INR67,700, ALOS was 3.87 days and the bed occupancy for the 3 pre -existing hospitals stood at 61.2%.

Overall volumes were up 9.9% to 10.8 lakhs with both OP and IP numbers reporting growth.

The insurance revenues now make up 55.4% of our revenue mix, while government stands at just 1%. Jupiter had a strong year. We remain focused on where we are going and excited about what lies ahead.

Thank you. And I'll now be happy to take questions.

Moderator: Thank you very much. We will now begin the question -and-answer session. The first question is from the line of Kurpa Shah from AS Capital. Please go ahead.

Jupiter Life Line Hospitals Limited

May 18, 202 6

Page 4 of 9

Kurpa Shah: Hello sir. Thank you for the opportunity. First question is, sir, could you just help us understand how the recently commissioned hospital is ramping up in terms of occupancy and revenue contribution? And, by when we can expect the major insurance tie -up to be in the place?

Ankit Thakker: Good morning. So, it's still very early days to talk about occupancy and numbers. We are just about 2 months into operation. So, it is the initial response, as I said, from the patients and the doctors quite eager to join the team has been quite good. The occupancy status, we could probably meaningfully discuss in a quarter or 2, when we have had some time under the belt. The insurance tie -ups typically get linked to the accreditation, which needs at least six months of operational data before you can proceed with that. So, we have started with the process of engaging both the accreditation NABH committees as well as the insurance, but it is likely to happen between 6 to 12 months.

Kurpa Shah: Okay, sir. Got it. Sir, my second question as the overall bed capacity increases from around 1,200 beds currently to nearly 2,900 beds over the next few years, how are we planning to manage the overall capex and funding requirement over the next 5 years?

Ankit Thakker: So, our internal accruals that we'll generate over the next 5 years, along with some debt, as I said, with a currently self -imposed ceiling of 3 times of EBITDA, we believe that a combination of these 2 should take us over there.

Kurpa Shah: Okay, sir. Thank you, sir.

Ankit Thakker: Thank you.

Moderator: Thank you. We will take our next question from the line of Amey Chalke from JM Financial.

Please go ahead.

Amey Chalke: Yes. Thanks for taking my question and congrats on the good numbers. So, I have first question, Ankit

, on the unit -wise occupancy and ARPOB for full year at least can be provided, that would be helpful.

Ankit Thakker: For Thane, the occupancy has been around 75%; Pune, close to 65% and Indore is between 45% and 50%, because of the added capacity that we did last year.

Amey Chalke: Sure. And going ahead, apart from Dombivli for our base business, how should we look at it?

Like, Thane would be able to grow at 10%, driven by a little bit of mix as well as some bit of volumes? Or you think it would be lower than 10%.

Ankit Thakker: So, Thane essentially should have inflation -linked growth at the current capacity, whatever the inflation number comes to. Pune, as I said, still has headroom to grow in terms of capacity utilization as well. So, Pune will have occupancy and inflation as a driver besides, of course, Indore, which will have all the drivers.

Amey Chalke: Right. So, for the base business, at least more than 10% growth should be possible, right, for next 2 years?

Jupiter Life Line Hospitals Limited

May 18, 202 6

Page 5 of 9

Ankit Thakker: I mean, in the current war situation, I don't want to take a guess of how inflation will play out. But qualitatively, that is what I want to tell you that Thane inflation, Pune inflation and occupancy, Indore inflation, occupancy and case mix.

Amey Chalke: Sure. And on the cost side, what are the metrics should we watch out? How the doctor cost, etcetera, is evolving in each of these regions? Are you facing any competitive pressure in any of

these regions, which we should keep in mind?

Ankit Thakker: No. On the cost side, the only thing that I am watching out for very keenly is the depreciating rupee and the cost of capex, so that is kind of a national reality. So that is something if the situation does not come under good control, returns to really ba lloon.

But apart from that, nothing really to say in terms of doctor costs, some consumable costs, again, linked to global supply chain disruptions, not meaningfully impacted yet, but those are some things on the cost side that we are watching out for.

Amey Chalke: Sure. The last question I have is on the Mumbai asset. Like have we finalized the plan for how the capex and all would be funded, etcetera?

Ankit Thakker: So qualitatively, what I just said earlier, back of the envelope three times of EBITDA and internal accruals over the next 4, 5 years should take us through the line of everything, including BKC and the other 2 projects. Detailed calculations, of course, we will do subsequently.

Currently, the land it has been allotted to us. We have paid some initial money, but it is still in the regulatory clearance, and prepossession formalities are underway with MMRDA. So once all of that happens, we'll sit down on the drawing board, figure out what the exact scope is before we can finalize specific capex numbers.

Amey Chalke: So that INR300 -plus crores, which was initial amount that has already been in our capex, right, for this year or something is left for next year?

Ankit Thakker: No, we have paid 25% of the land cost as per the tender condition.

Amey Chalke: Okay. So, remaining would be paid next year or will it also be in a staggered manner?

Ankit Thakker: It should be paid in this financial year. Everything will be paid in this financial year.

Amey Chalke: Sure. Sure. Thank you so much. I will rejoin the queue.

Ankit Thakker: Thank you.

Moderator: Thank you. Next question is from the line of Priya Kulkarni from PN Capital. Please go ahead.

Priya Kulkarni: Yes. Thank you, sir, for this opportunity. I would like to ask a question on the Thane hospital expansion. So, could you please provide an update on the approval process for the additional source? Also, by when do you expect the approvals to come through and the expansion work to begin?

Jupiter Life Line Hospitals Limited

May 18, 202 6

Page 6 of 9

Ankit Thakker: So, the

approval processes are underway. We have received some interim approvals, but not final. That is why I have not specifically pointed it out. I can't really predict regulatory timelines .

But I think sometime in this financial year, we should have the approvals with us. And once I have them, I will be happy to inform everyone about the exact scope and scale of expansion.

Priya Kulkarni: Okay Sir. I understood. That answers my question. Thank you.

Ankit Thakker: Thank you.

Moderator: Thank you. We will take our next question from the line of Gourav Bhama from JM Financial. Please go ahead.

Gourav Bhama: Hi sir. Good morning. Am I audible?

Ankit Thakker: Yes, Gourav.

Gourav Bhama: Yes, sir. Thank you for the opportunity. I had a small bookkeeping question actually. The INR9.4 crores from the Dombivli unit that is completely baked in, in the quarter 4 results, right?

Ankit Thakker: Yes.

Gourav Bhama: Okay. That's all from my end, sir. Thank you.

Moderator: Thank you. We will take our next question from the line of Hetvi Sanghvi from HH Investment. Please go ahead.

Hetvi Sanghvi: Hi. Thank you for the opportunity. So, I had a question regarding the second Phase of Indore expansion, involving the addition of 111 beds. So, by when do we expect this Phase two come operational?

Ankit Thakker: So, Hetvi as I said, the occupancy currently is just under 50% now. Once the occupancy crosses 60% or near 60%, then we will think about the next phase of bed addition.

Hetvi Sanghvi: Okay. Noted. Another question I had was about ARPOB. The ARPOB has increased from around INR60,600 to INR67,600 during the financial year. So, could you help us understand what is driving this strong improvement? Is this largely because of better case mix and specialty contribution or the pricing improvements also play a role?

Ankit Thakker: So, it is a combination of a couple of things. Indore maturing and improving in case mix is definitely adding to the ARPOB. And we have had a couple of renewals from the insurance company, which happened once in 2 years, so, some price increase that has kicked in after a 2 - year period as well, so a combination of some inflationary price growth and case mix.

Hetvi Sanghvi: Okay. Got it. Thank you for answering.

Moderator: Thank you. Next question is from the line of Dikshant Gupta from Geojit PMS. Please go ahead.

Jupiter Life Line Hospitals Limited

Dikshant Gupta: Yes. Good morning, sir. And thank you for the opportunity. So, I just wanted to know how what will be the EBITDA burn for Dombivli this year?

Ankit Thakker: So, we have said between INR2 crores to INR3 crores a month on an average for the whole year.

Dikshant Gupta: Yes. And by Q1 of FY28, can we expect an EBITDA breakeven?

Ankit Thakker: Yes. So, end of 2 years, we should expect an EBITDA breakeven.

Dikshant Gupta: End of 2 years, so that would be Q4 FY28, right?

Ankit Thakker: 2 years of operation, calendar year '26, we have started. So, calendar year end of , '27, '28, you should expect it.

Dikshant Gupta: Okay. Okay. Thank you so much sir.

Moderator: Thank you. We will take our next question from the line of Karan Sharma from Sharma Securities. Please go ahead.

Karan Sharma: Hello. Good morning, sir.

Ankit Thakker: Good morning, Karan.

Karan Sharma: So, sir, I have one question. So, when is, the next insurance renewal is lined up for all our existing hospitals, Thane, Pune, Indore across?

Ankit Thakker: So, this is an ongoing process, and there are government insurances and multiple different private insurances. Each of them have their own cycles. So essentially, every year, there are a few contracts which are up from renewal. So, there is no specific month or season in which all of them get renewed. So, it's on a rolling basis.

Karan Sharma: Yes. So yes, you were saying something?

Ankit Thakker: No, I said, it's on a r

olling basis, there is no specific date.

Karan Sharma: Okay, sir yes. Thank you.

Ankit Thakker: Thank you.

Moderator: Thank you. We will take our next question from the line of Dhvani Shah from DSP. Please go ahead.

Dhvani Shah: Yes. Hi. Sir actually wanted some qualitative direction on the Indore facility. I think in the last year, if I think about your subsidiary revenues, those are mainly from Indore, except for the last quarter. The EBITDA as well seems to have seen a very strong ramp -up, if you can just explain the levers going forward? I understand, you've mentioned case mix occupancy, but a little more detail on the same?

Jupiter Life Line Hospitals Limited

May 18, 2026

Ankit Thakker: So, Dhvani, you managed to catch the important bits. Unfortunately, I don't have too much more to say. Currently, Indore is in its ramp -up phase. The new beds that we added last year have slowly started getting occupied.

And essentially, that is what it is. The occupancy has to improve. And the case mix over the next 1 or 2 years we still believe has a potential to improve a little more. Combined, both these factors should take it to a near maturity state in the next 2 or 3 years.

Dhvani Shah: Understood. And one more thing on the Dombivli asset is all the doctor hiring and the fixed cost now in the place? Or will there be some additional costs coming in on the fixed side going forward?

Ankit Thakker: So, there will be some additional definitely coming in, though a large part of the team we have had in place for the launch. But –the team building is not a onetime event. It will keep on happening for the next couple of years. You'll keep adding more and more doctors, because it's a large facility. So yes, I think you should expect more fixed costs.

Dhvani Shah: Understood. Okay. Thank you so much.

Ankit Thakker: Thank you.

Moderator: Thank you. Next question is from the line of Anuj Kashyap from A3 Capital. Please go ahead.

Anuj Kashyap: Sir, I wanted to know as we are in MMR region, so what is the percentage of revenue we drive from the foreign patients?

Ankit Thakker: Very little. We have very little reliance on international medical tourism. We predominantly, our model is catering to local residential communities and all our hospitals are designed such that the capacity is sufficient to cater to the immediate 45 -minute driving distance community. So yes, we don't have too much of reliance on foreign medical tourism.

Anuj Kashyap: And sir, sir, one more forward -looking question I have is, like, the central government is taking the initiative like, just to, bring in more foreign patients into our ecosystems. And sir, do Jupiter as a hospital chain is focusing on that very thing, like what the government is trying to do?

Ankit Thakker: So personally, from an organizational standpoint, no, we are not very focused on international

medical tourism. But...

Anuj Kashyap: And sir, just to add on to it, sir, like is it margin accretive like the international tourism, the medical tourism?

Ankit Thakker: I mean, it depends on your individual P&L. As far as we are concerned, most of the facilities have enough local demand. And we don't find it very margin accretive or we don't find the need to focus on international. We'd rather focus on domestic and local.

Anuj Kashyap: Thank you, sir. Thank you.

Jupiter Life Line Hospitals Limited

May 18, 2026

Page 9 of 9

Moderator: Thank you. That was the last question for today. I now hand the conference over to the management for closing comments. Thank you.

Ankit Thakker: So, thank you, everyone, for joining us today on this call. If you have any further questions that I was not able to answer, please feel free to get in touch with us through SGA, our Investor Relations Advisers. Thank you.

Moderator: Thank you very much. On behalf of Jupiter Life Line Hospitals Limited, that

concludes this

conference. Thank you all for joining us today. And you may now disconnect your lines.